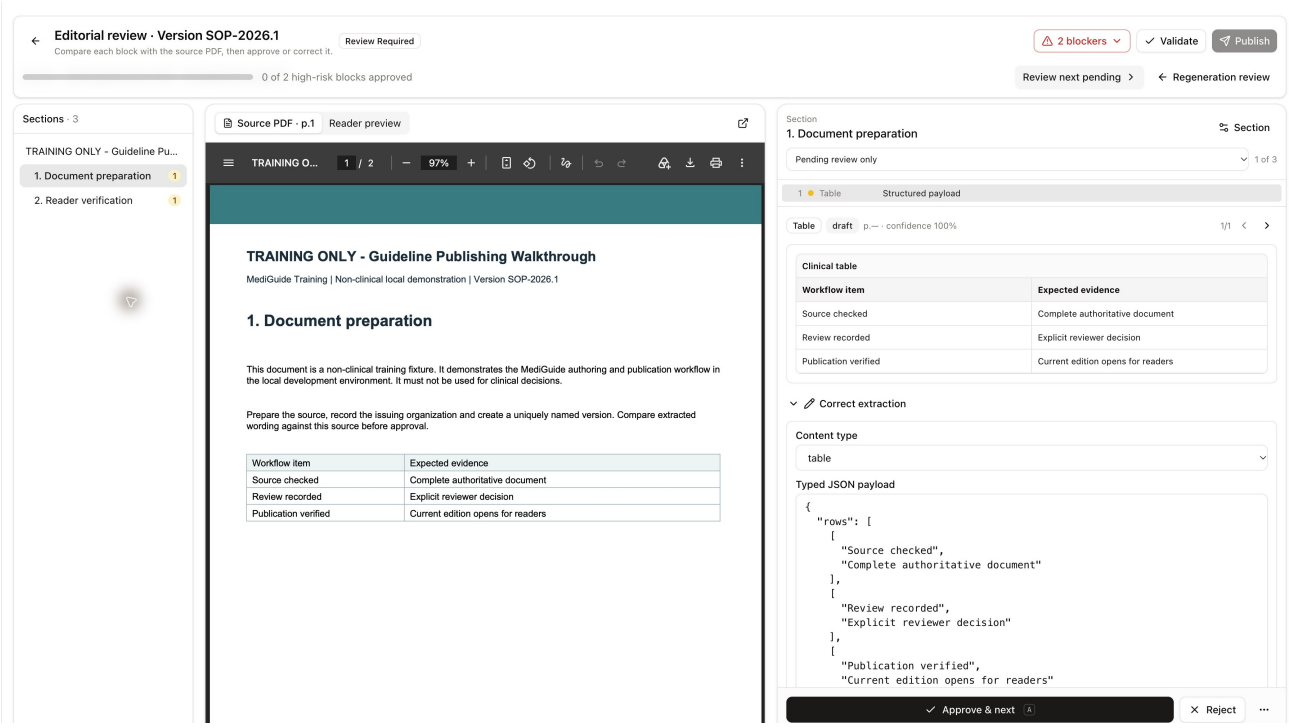
Clinical Guidelines
The complete dashboard workflow

Upload → edit → regenerate → review → accept → publish → verify website and app



What changed in this edition
Real dashboard screenshots replace the earlier reconstructed diagrams. Procedures, operating details, full screen plates and enlarged panels show the actual controls and review states.

For authors, clinical reviewers, publishers and reader testers. Prepared 25 October 2026. Training content is clearly marked and contains no clinical recommendations.

Controlled document: Operational reference pending the organization’s clinical governance approval. Assign a local owner and review after material dashboard changes.

How to use this guide

Follow the procedure, then its screen plates

Each numbered procedure states the starting point, actions, expected result and a watch point. Screen plates immediately afterward show the real interface. Enlarged details preserve small labels. PDF bookmarks provide direct navigation.

Know which environment is shown

The first library image is a read-only production capture. Authoring screens use the locally running dashboard with a non-clinical TRAINING ONLY document, version SOP-2026.1. The public reader and published-state plates use a separate existing local fixture.

What this walkthrough actually reached

The two-page training source was uploaded, corrected, saved through revision 3, regenerated, individually and bulk reviewed, and accepted. Completeness reported 9/9 reviewed blocks and all publication gates passing. The final Publish confirmation was inspected and canceled. No production clinical records were changed.

Understand the mobile evidence

Dashboard mobile previews are labeled as previews. Android app plates come from the repository’s real store screenshot assets and show a different guideline. On-device publication, synchronization and offline checks remain required for each real edition.

Use the expected result as a gate

Stop and investigate a mismatch between source, actual rendered content, report or target edition. Screen counts and layout vary with document size, permissions and software version. Training counts are examples, never a substitute for full review.

Find the stage you need

Create and upload

Procedures 01–07: lifecycle, people and access, document route, metadata, edition, source upload, extraction and indexing.

Author and regenerate

Procedures 08–14: Markdown workspace, headings, images, tables, preparation and saving, collaboration, revision-bound regeneration.

Review and accept

Procedures 15–20: editorial layout, ordinary-content review, individual decisions, corrections, completeness, acceptance.

Publish and verify

Procedures 21–24: validation and publication, public website, mobile/offline verification, optional notifications.

Aftercare and exceptions

Procedures 25–28: update/withdrawal, original-file/link routes, troubleshooting, publication handover.

Navigation tip

Use the PDF bookmarks or search for an exact control label such as Prepare for review, Completeness report or Accept regenerated projection. Every screen plate is named in its related procedure.

01 The complete workflow

Use this map to keep the source, generated content, review decisions and public version aligned.

1. Create and upload

Create a guideline document and a version. Upload the authoritative source and wait for processing. See the corresponding procedure and screen plates.

2. Edit and regenerate

Correct the Markdown, tables and images. Save the intended revision and regenerate its reader content. See the corresponding procedure and screen plates.

3. Review and accept

Review ordinary prose as well as safety-sensitive blocks. Check completeness and accept the exact regenerated revision. See the corresponding procedure and screen plates.

4. Publish and verify

Validate and publish, then verify the website, mobile reader, downloads and search. See the corresponding procedure and screen plates. Notifications, updates and exceptions follow in procedures 24-28.

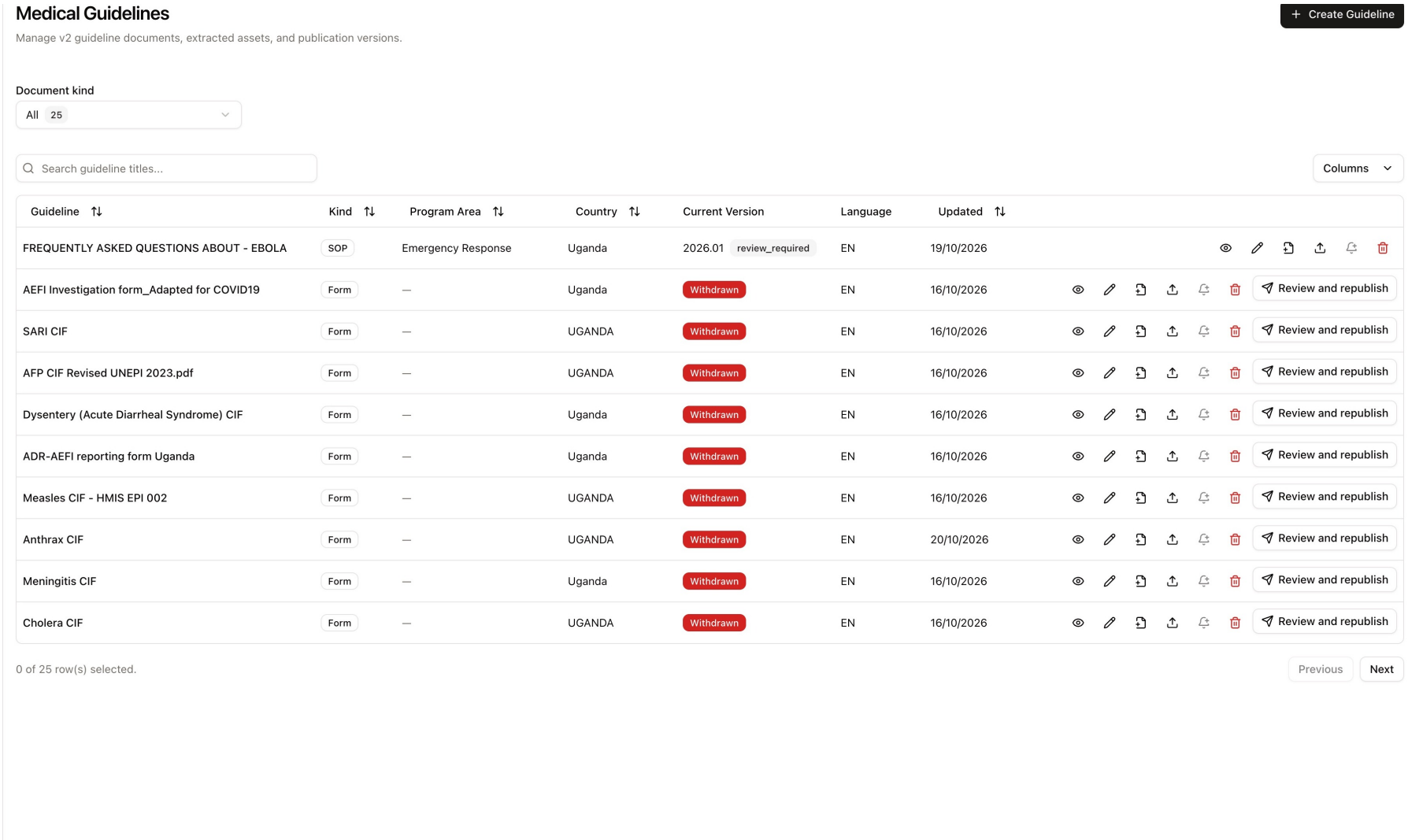
Continue when
Saving, reviewer assignment, generation completion and a private preview do not publish a guideline.

Watch point: Keep the last approved publication available while its replacement is prepared.

Screen plates follow: Guidelines library

Guidelines library

Clinical Guidelines > All Guidelines



Find the intended document before choosing a version. The list is a read-only production capture; all subsequent authoring examples use localhost.

02 People, access and preparation

Assign responsibility before starting. The organization must authorize clinical decisions and publication.

1. Author / editor

Needs document creation access plus guideline.markdown.read, guideline.markdown.edit, guideline.markdown.upload, guideline.asset.manage and guideline.structure.regenerate for the assigned work. Prepare the authoritative PDF or UTF-8 Markdown, images and issuing organization.

2. Reviewer

Uses guideline.review. High-risk clinical decisions and regeneration acceptance also require guideline.high_risk.approve. Compare content with the authoritative source; use an independent reviewer where local governance requires it.

3. Publisher

Needs guideline.publish and access to the document. The server still enforces publication validation. An administrator cannot bypass missing review by changing a dashboard badge.

4. Before uploading

Confirm title, document kind, version label, source organization, country, language, publication/review dates, applicable categories and diseases. Avoid patient-identifiable information in training files or review notes.

Continue when
An active account with the correct role can see the expected controls. If a control is missing, ask an administrator to check permissions.

Watch point: Assignment is coordination, not clinical approval. Never approve content merely to clear a warning.

03 Choose the correct document route

The selected document kind determines whether readers receive structured content, an original file, or a link.

1. Structured clinical guideline

Choose a normal guideline kind for searchable, editable chapters and blocks. Upload PDF, .md or .markdown. Follow the full editing, regeneration, review and publication process in the following procedures.

2. Publish as uploaded

Some kinds, such as forms, keep the PDF or Word (.docx) file exactly as uploaded. Text indexing supports discovery, but there is no Markdown editing or structured-block workflow. See the special-routes procedure.

3. Publish as link

A link kind publishes a complete HTTPS URL. No source file is uploaded and no extraction or Markdown editor runs. Review the destination before publishing. See the special-routes procedure.

4. Choose deliberately

Use categories and diseases for discovery. Legacy program area is optional. If the expected structured workflow is absent, check the document kind instead of uploading the same file repeatedly.

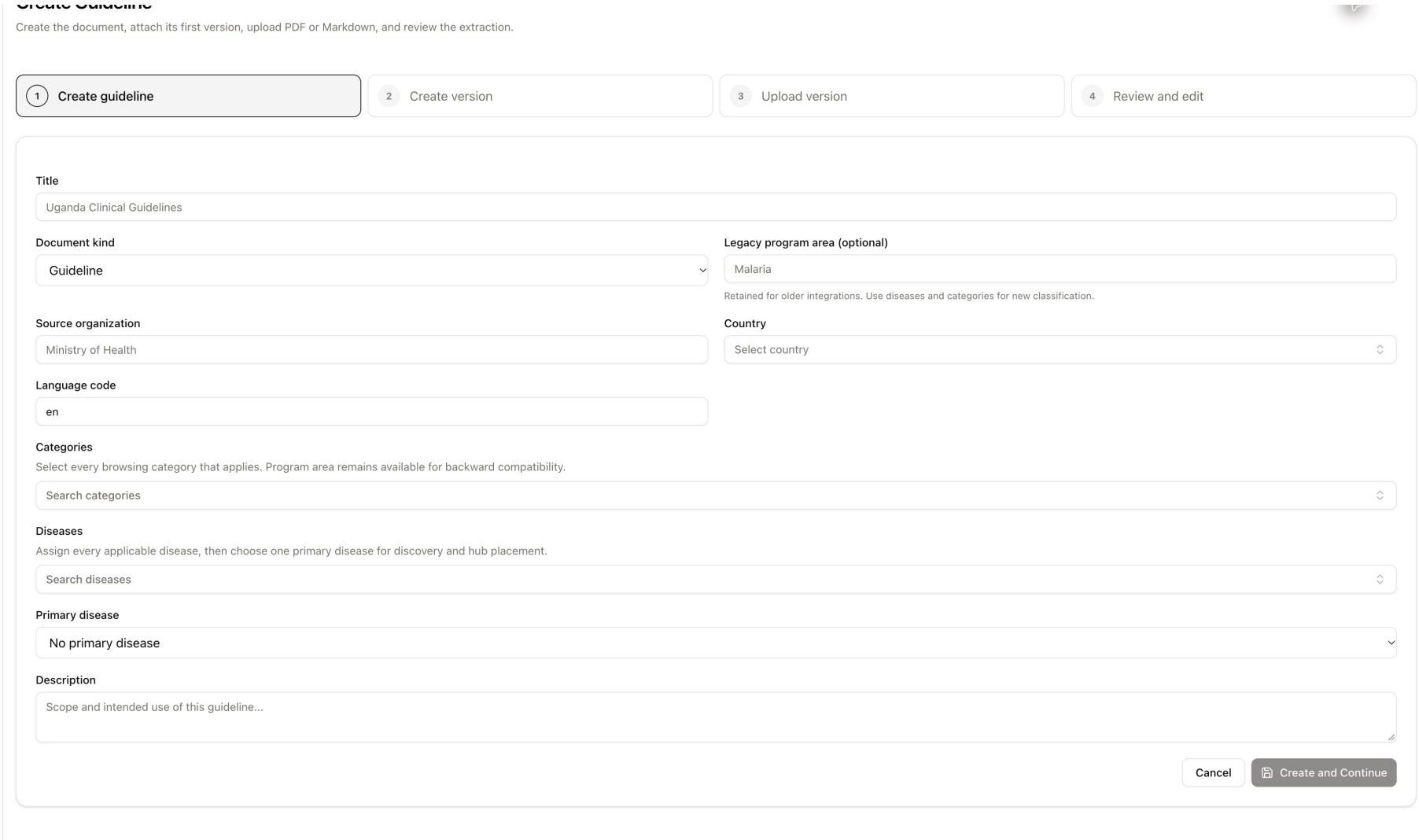
Continue when
For an editable clinical guideline, continue with the structured route.

Watch point: Do not select an original-file or link kind just to avoid clinical review requirements.

Screen plates follow: The four-stage creation wizard

The four-stage creation wizard

Clinical Guidelines > Create Guideline



The stages are Create guideline, Create version, Upload version, and Review and edit. Document kind determines the authoring route.

04 Create the guideline record

Dashboard > Clinical Guidelines > Create Guideline. This record describes the document across all its versions.

1. Open the creation wizard

Select Create Guideline from the guidelines area. The four stages are Create guideline, Create version, Upload version, and Review and edit.

2. Complete the metadata

Enter the official title and select Document kind. Add Source organization, Country and Language code. Add a description that explains the document without changing its clinical meaning.

3. Classify for discovery

Select every relevant Category and Disease, then a Primary disease from the selected diseases. Use active taxonomy entries. Classification supports browsing and hub placement; it is not publication approval.

4. Save the record

Select Create and Continue. Confirm that the next stage names the correct document. To correct document-level metadata later, use Edit Metadata on the document page.

Continue when
The guideline record exists and the wizard advances to Create version.

Watch point: Use the source title consistently; a template title or unrelated Markdown H1 can block publication.

Screen plates follow: Completed document metadata

Field-by-field check

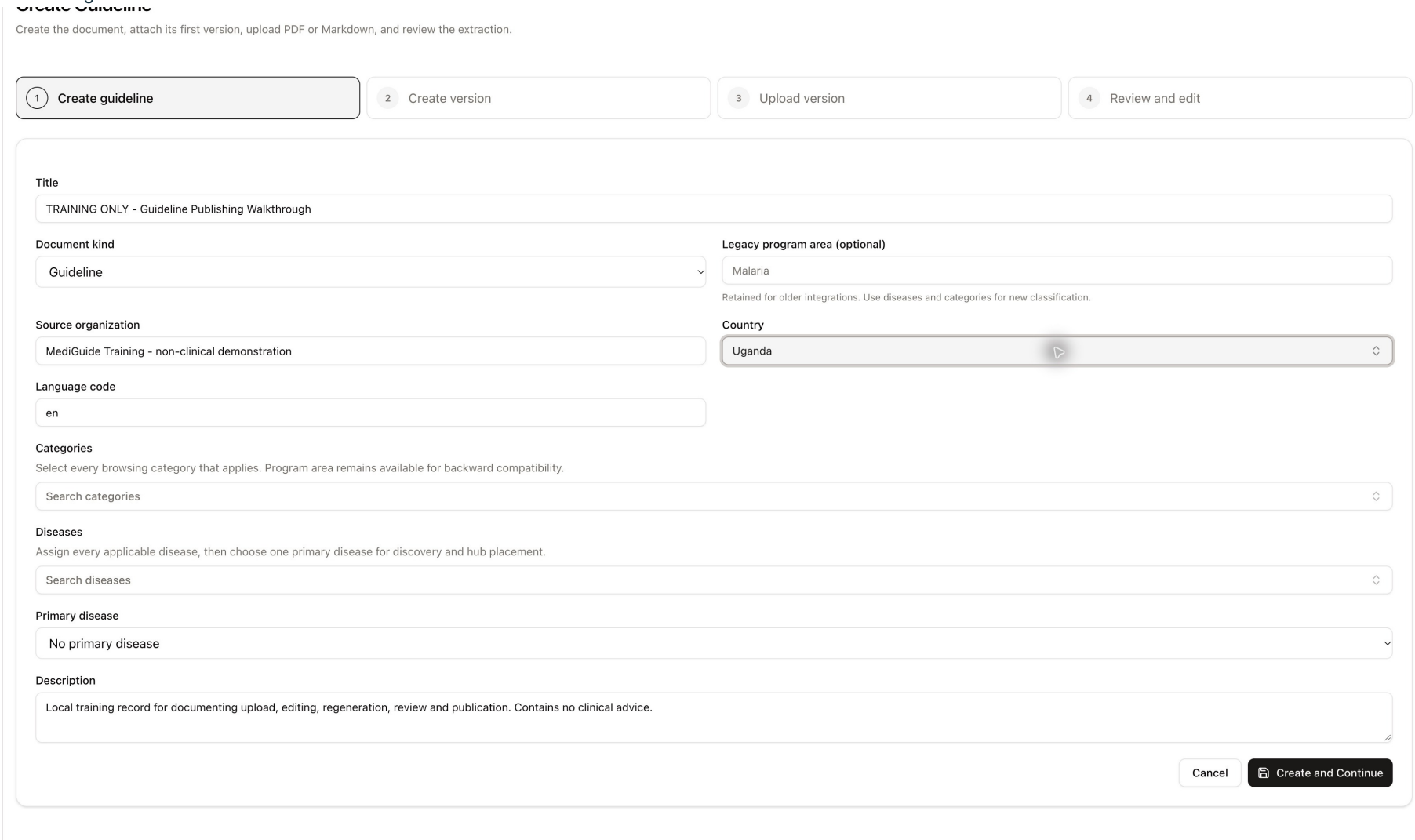
Title: exact official source title. Document kind: choose the intended structured/original/link route. Legacy program area: optional where shown. Source organization: issuing body, not an author signature. Country and language: match the document. Categories/diseases: select relevant taxonomy entries; Primary disease comes from selected diseases. Description: a short accurate summary.

After Create and Continue

The record is saved and Create version becomes active. If validation fails, stay on this form and correct the named field; do not create a second record to retry.

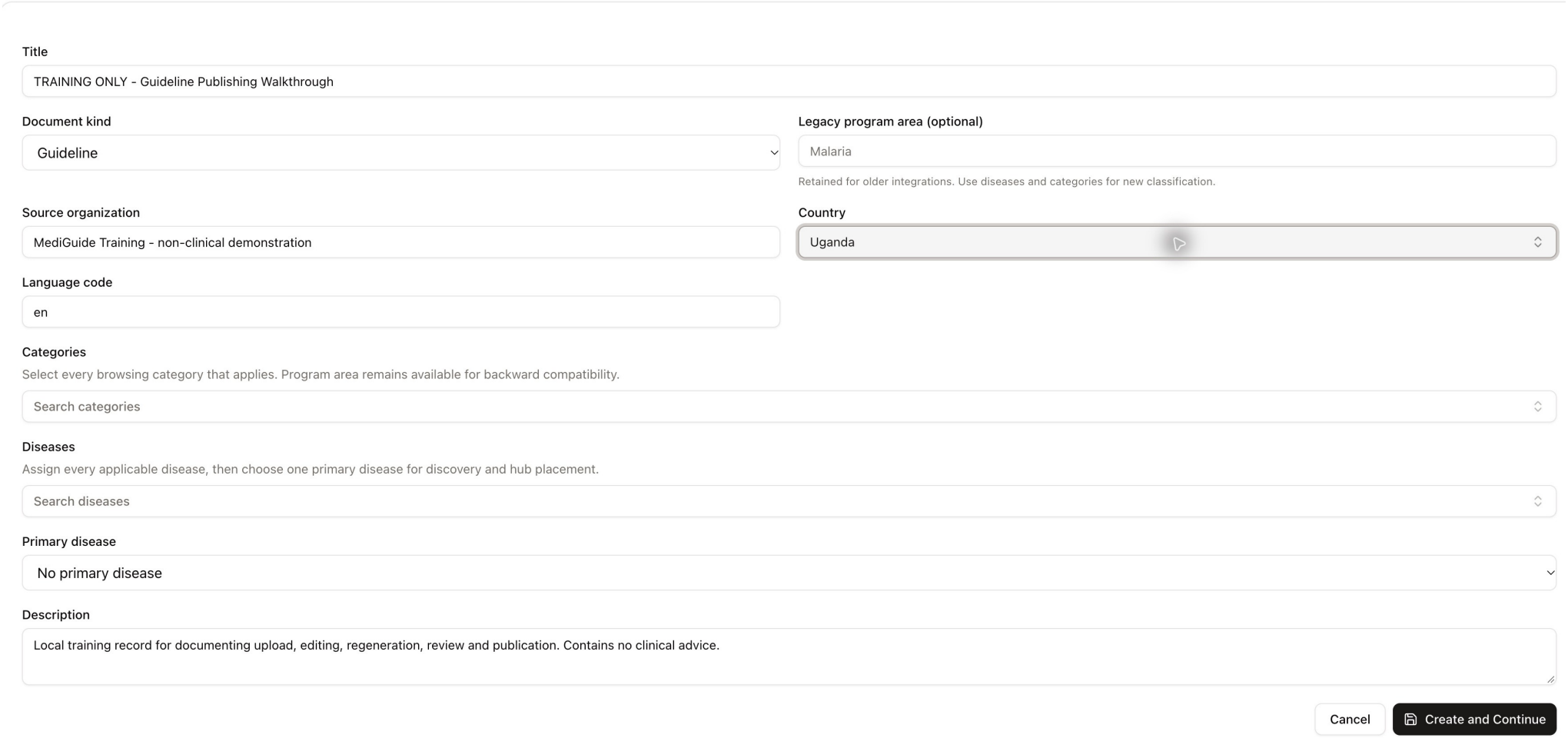
Completed document metadata

Create guideline stage



Read each field before Create and Continue. The TRAINING ONLY title and non-clinical description identify this local example.

Completed document metadata — close-up



Read each field before Create and Continue. The TRAINING ONLY title and non-clinical description identify this local example.

05 Create the first version

Document version labels identify the clinical edition; they are separate from the MediGuide application release.

1. Name the edition

In Create first version, enter the official Version label, such as 2026.1. Choose a label that is unique for this document and follows the organization's naming convention.

2. Record the dates

Enter Publication date and Review date when applicable. These describe the guideline edition; entering a date does not schedule or perform publication.

3. Create the draft

Select Create Version. A saved version is required before uploading source files. Finish Later returns to the document, where New Version can create additional editions.

4. Confirm your target

Before any upload or edit, check the document title, version label and draft state. Published versions are immutable; updates use a new draft rather than an in-place edit.

Continue when
The draft version is listed under Versions and the upload stage is available.

Watch point: Do not reuse an existing edition label for a replacement. Retain older versions for audit.

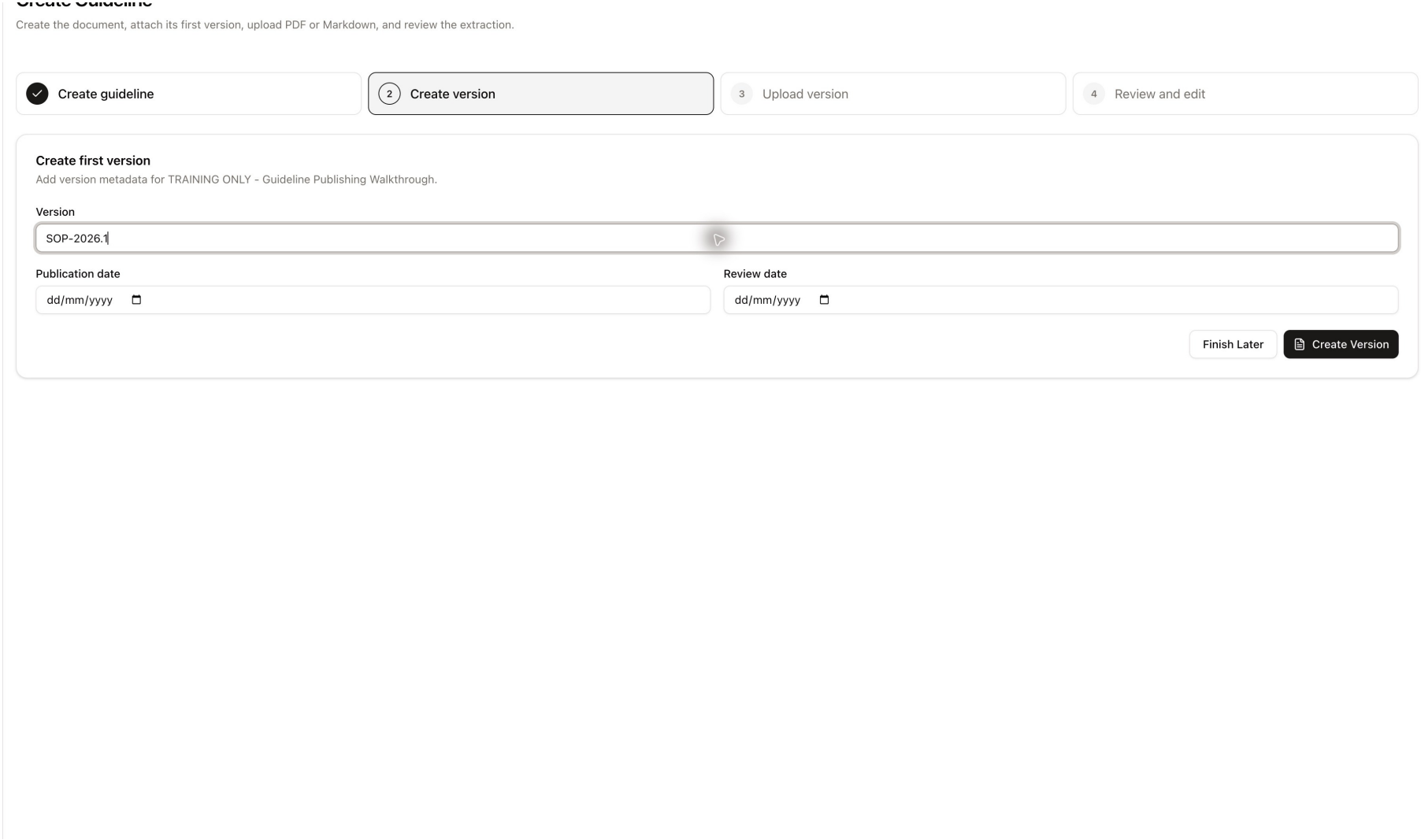
Screen plates follow: Create the edition

Record the edition

Use a unique label such as the issuing body’s edition or year/revision. Confirm dates against the source. The software build number and the guideline edition are different identifiers.

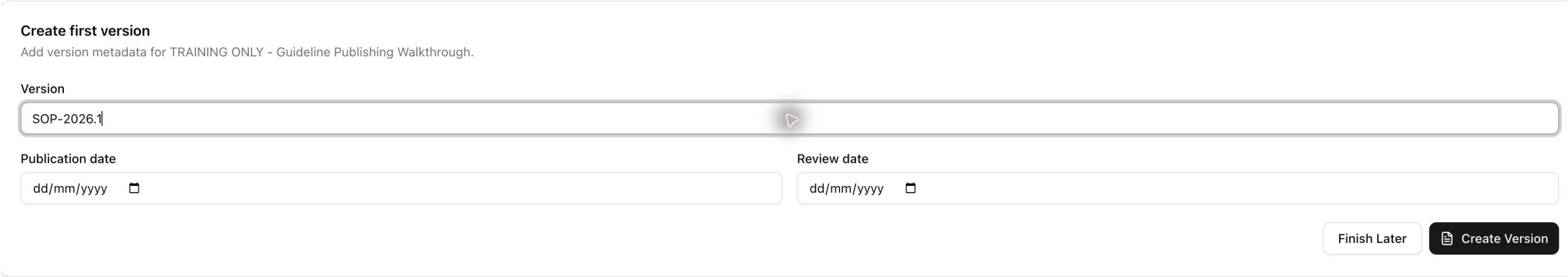
Create the edition

Create version stage



Version label identifies the document edition. Publication date and Review date do not publish or schedule the version.

Create the edition — close-up



Version label identifies the document edition. Publication date and Review date do not publish or schedule the version.

06 Upload the authoritative source

The source is the fidelity reference for every subsequent correction and clinical review decision.

1. Choose a supported file

In Upload guideline source, choose the authoritative PDF or UTF-8 Markdown (.md / .markdown). The creation screen allows up to 100 MB. For an existing draft, use Upload PDF to open the source dialog, which also accepts Markdown.

2. Transfer the source

Select Upload source. The screen checks the file, transfers it, verifies storage and queues processing. Keep the selected version unchanged during the upload.

3. Resume safely

Pause is available during transfer. To resume, choose the same file and select Resume upload; the screen preserves completed upload parts for up to 24 hours. Cancel upload abandons the transfer session.

4. Choose another starting path if needed

Start with blank Markdown or Start from template opens authoring without PDF extraction. Existing Markdown may also be loaded from the editor. Obtain the original images separately for Markdown imports.

Continue when
Storage verification finishes and Source transfer and processing shows a real processing job.

Watch point: An upload reaching 100% is not proof that extraction, indexing, clinical review or publication succeeded.

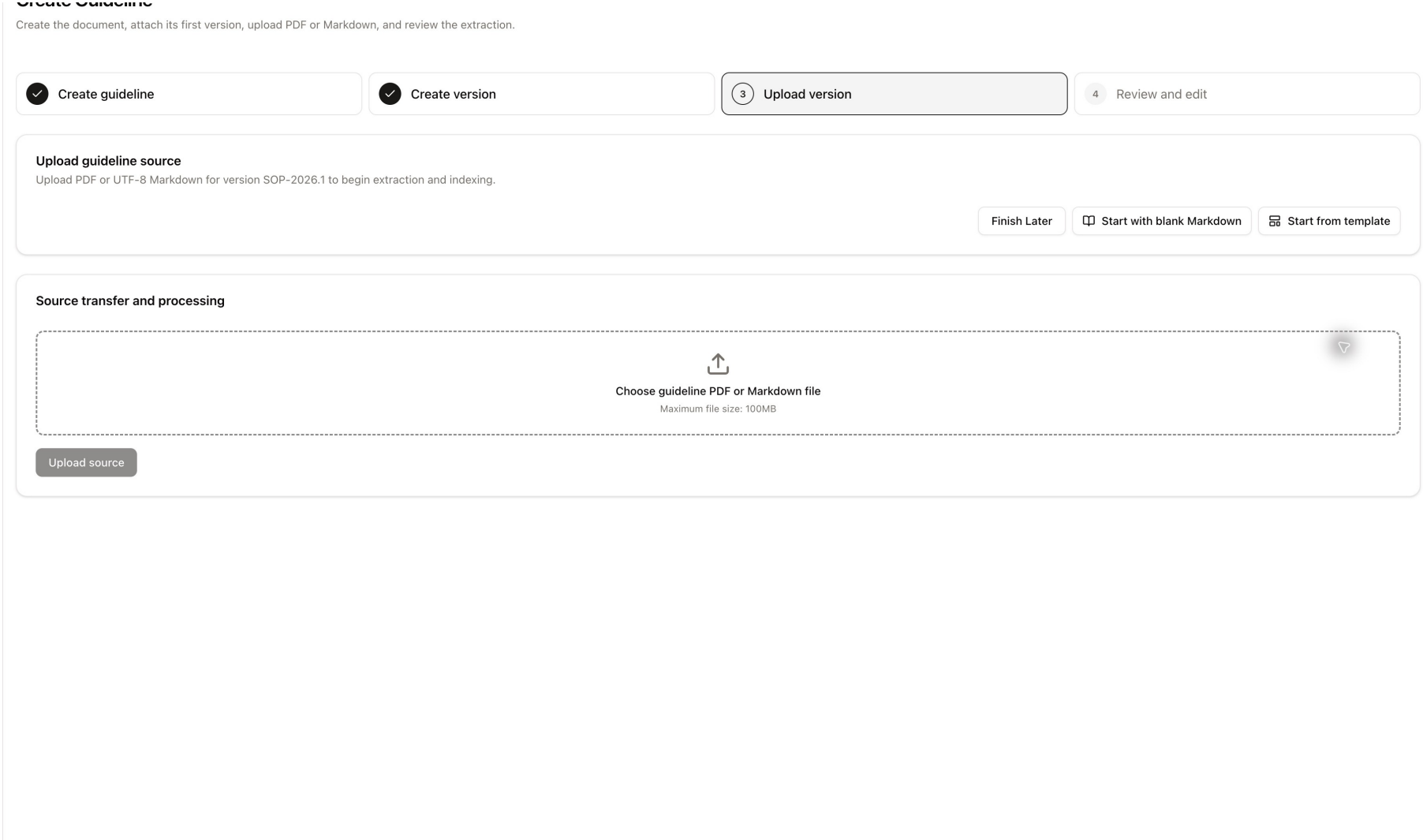
Screen plates follow: Choose the source route; Source selected, before transfer

Before Upload source

Confirm the selected filename appears in the upload panel. Removing or replacing a selected file before transfer is different from replacing an already published edition. Keep the authoritative source in the approved document archive.

Choose the source route

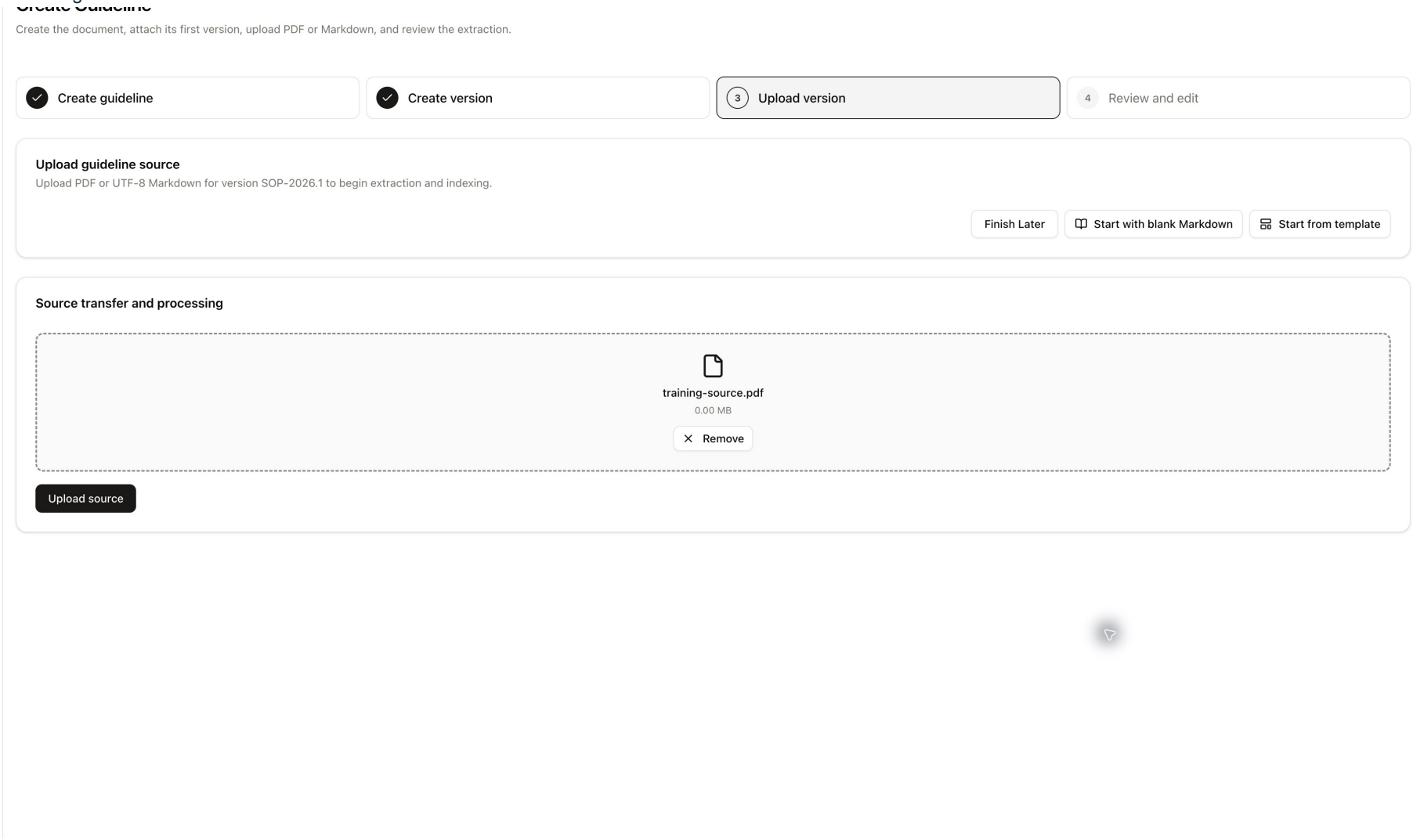
Upload version stage



Choose a PDF or Markdown source; the alternatives are Start with blank Markdown and Start from template.

Source selected, before transfer

Upload version stage



Verify the selected filename, target document and edition, then Upload source. Selecting the file alone does not transfer it.

07 Monitor extraction and indexing

Wait for the real processing result. A large PDF can take time, especially when OCR or table extraction is required.

1. Watch the stage list

Follow Source transfer and processing. It reports the current stage, progress, attempts and any available timings. The wizard refreshes while extracted Markdown is still unavailable.

2. Wait for usable output

When processing completes, open the document and choose Edit Markdown or Editorial Review as appropriate. Preview original keeps the source available for comparison; processing completion alone does not make extracted blocks approved.

3. Handle a failed job

Read the error and failing stage. Retry processing is offered for eligible failed/canceled jobs and reuses completed checkpoints where possible. An automatic retry time may also be shown.

4. Escalate a persistent failure

Record the title, version, job identifier, stage, attempt count, time and error. Ask an administrator to investigate worker/storage/embedding failures. Do not create several duplicate documents to work around one job.

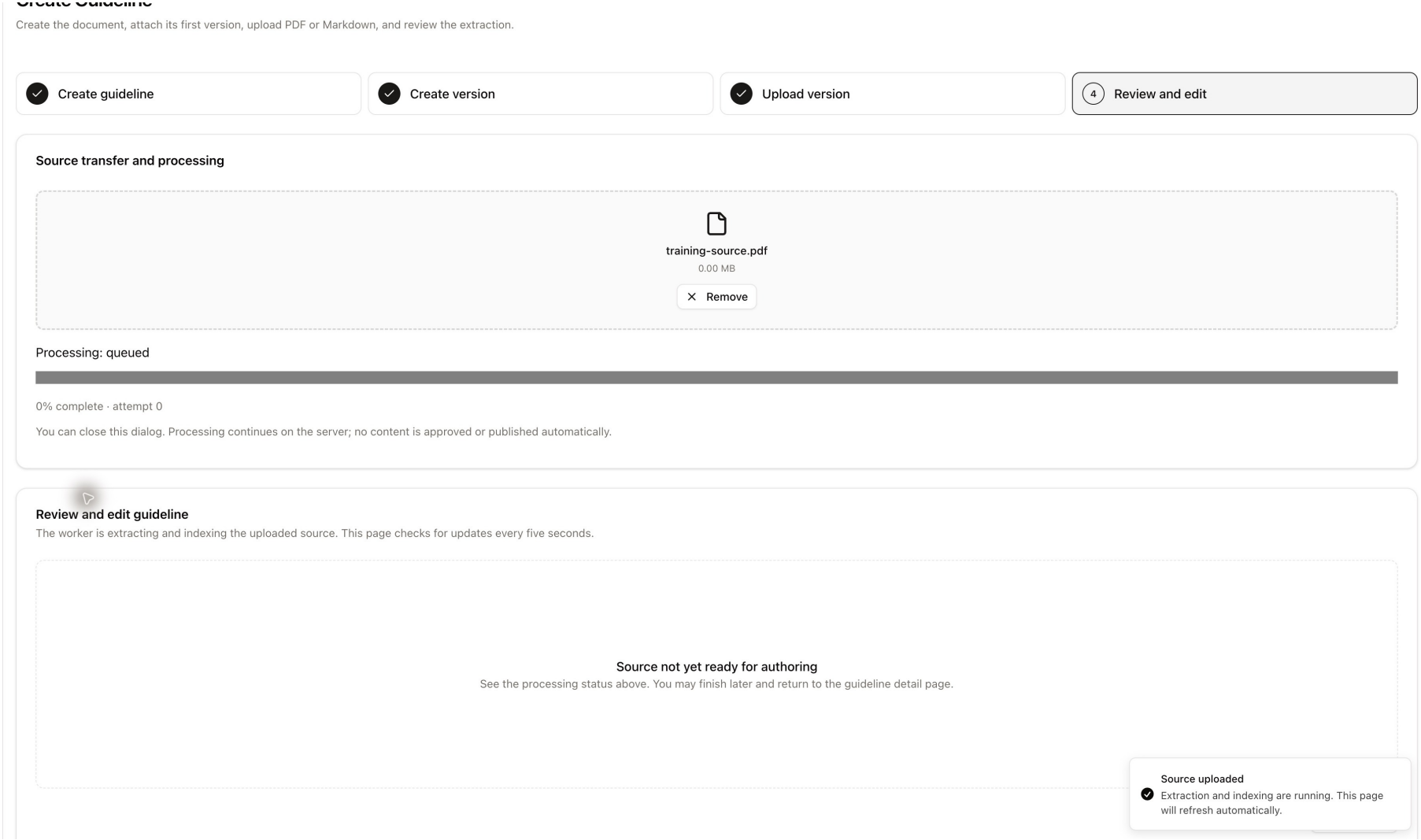
Continue when
The correct draft has its extracted source and is ready for editorial work.

Watch point: Do not assume a fixed completion time or that a quiet progress bar means the job has failed.

Screen plates follow: Transfer complete; processing queued; Completed extraction stages; Ready to author

Transfer complete; processing queued

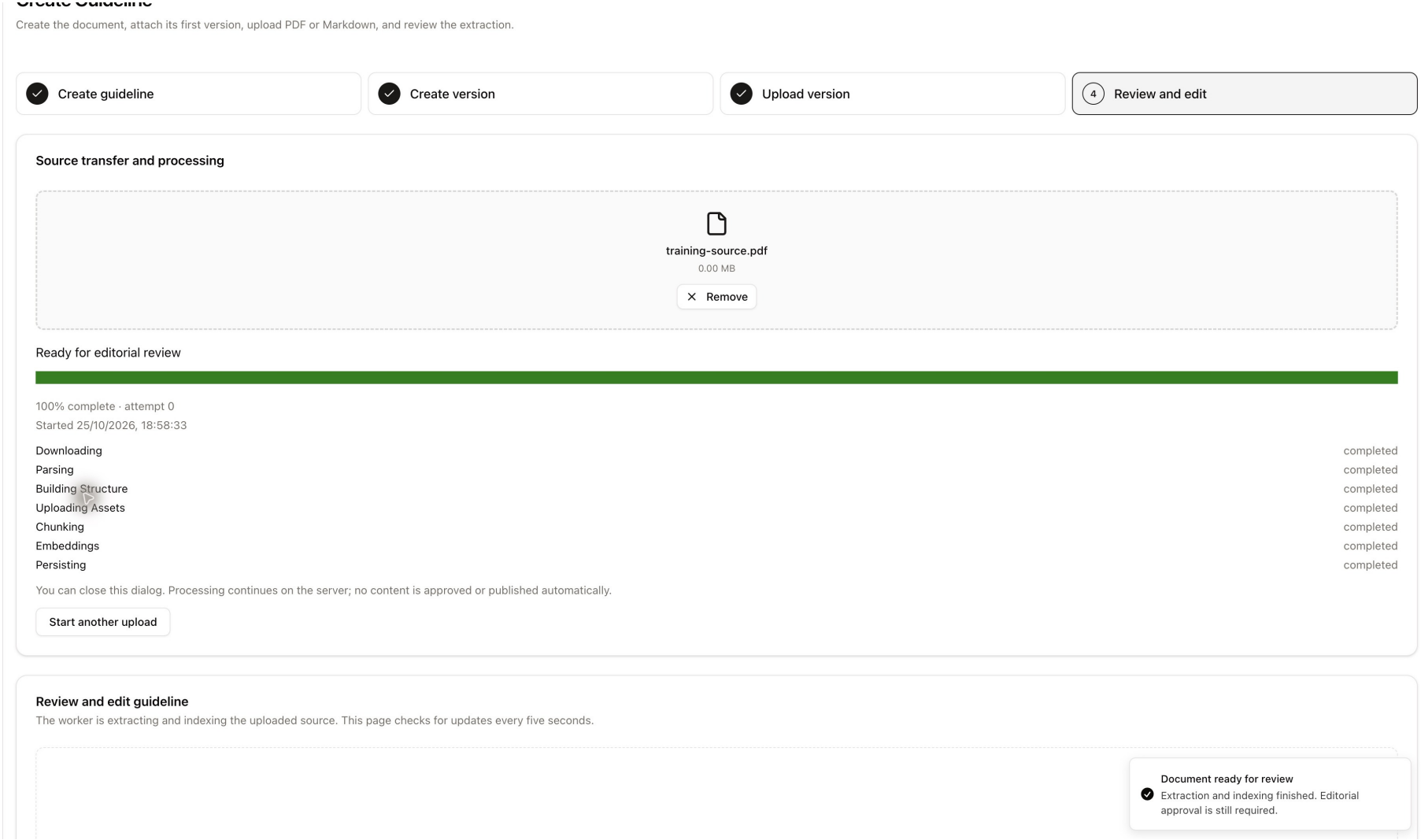
Source transfer and processing



The queued job is a separate step after file transfer. Do not leave with the assumption that the reader content is already reviewed.

Completed extraction stages

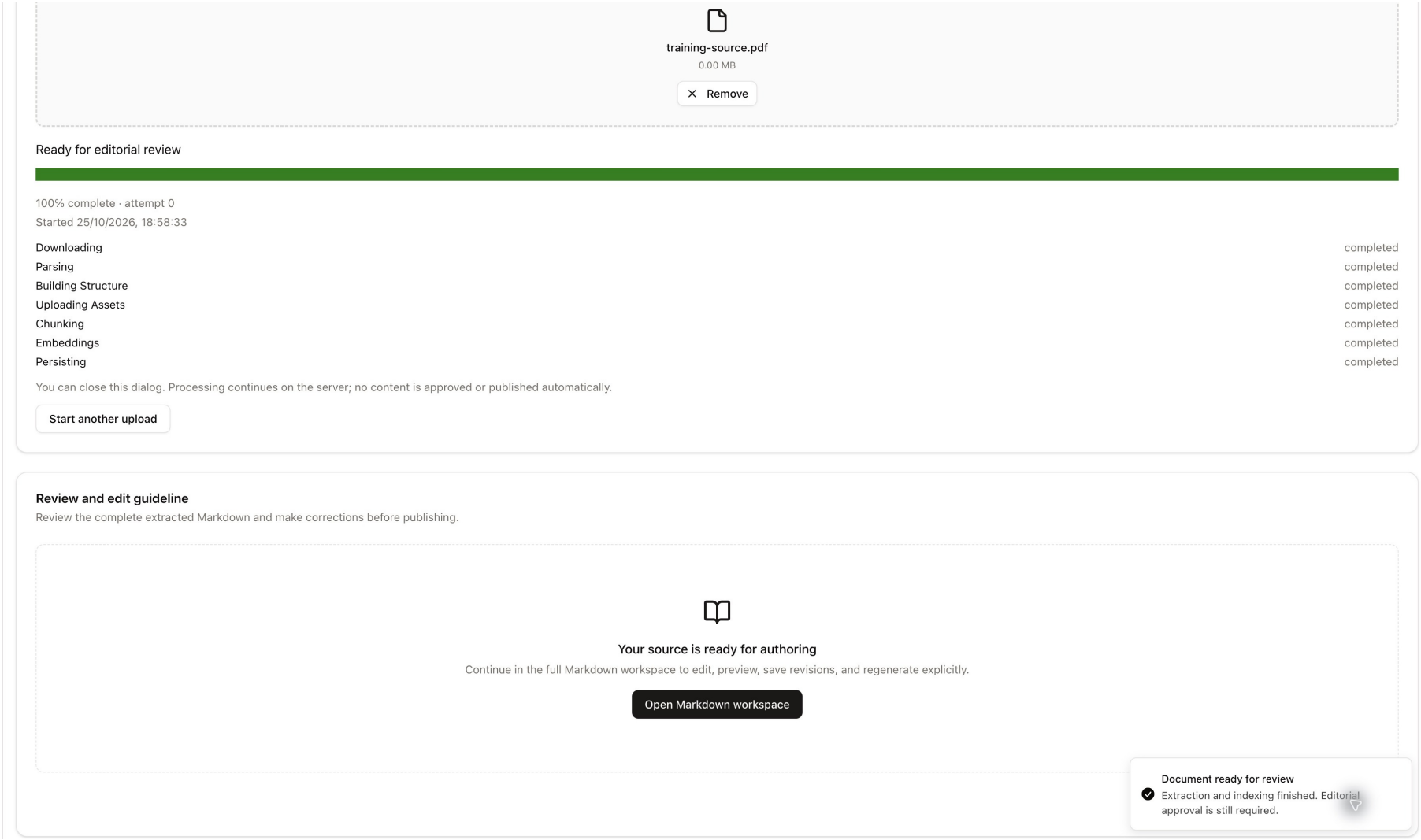
Source transfer and processing



This small training PDF completed quickly. Stage rows show their actual completion states; duration varies for real documents.

Ready to author

Review and edit stage



Use Open Markdown workspace after the source is available. A green processing bar is readiness for editing, not public release.

08 Open the Markdown workspace

Document > draft version > Edit Markdown (or Author Markdown when starting without extracted source).

1. Check the version and revision

Confirm the guideline title and version. The editor shows the saved revision and structured-content status. Work in the draft intended for publication.

2. Choose a working view

Use Edit, Split or Preview. Split makes it easier to compare source syntax and rendered output. Use Original PDF where available to verify fidelity; Markdown-only sources cannot claim PDF page citations.

3. Inspect the whole document

Review the outline, front matter, chapter boundaries, paragraphs, lists, links, tables, figures and footnotes. Check the beginning, middle and end, not only the first chapter.

4. Use private previews carefully

Reader, mobile, search and RAG presentations help expose structural problems. They are previews, not evidence that content has been published or clinically approved.

Continue when
The editor contains the correct complete source for the draft edition.

Watch point: Retain the authoritative source beside the editor. Extraction can lose text or reorder content without an obvious syntax error.

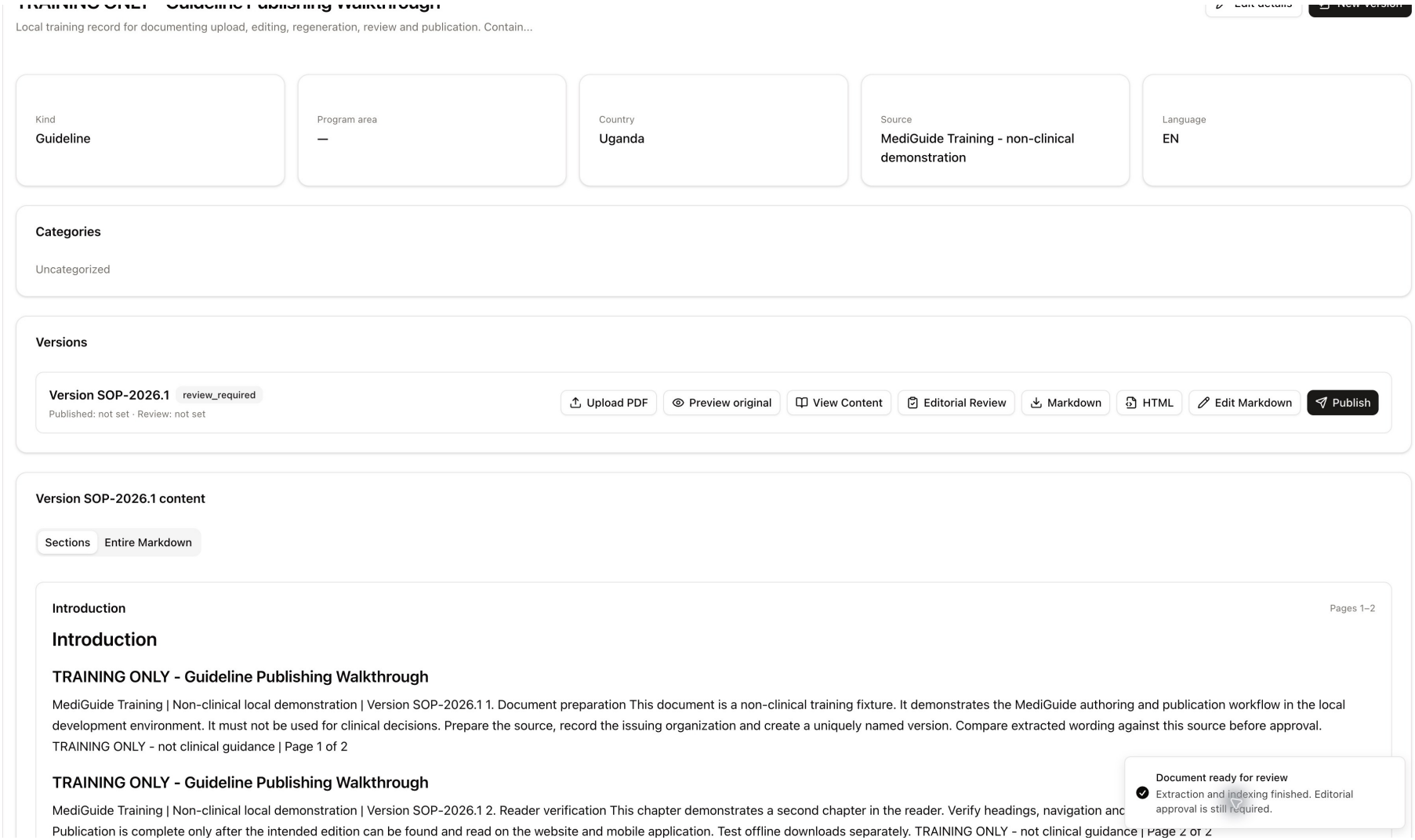
Screen plates follow: Version-level actions; Extraction defects are visible

What each editor control does

Edit changes source; Split shows source and render together; Preview shows output. History compares immutable saved revisions. Assets manages figure files. Review & activity coordinates reviewers. Compare regenerated evaluates a generated projection. None of these controls publishes by itself.

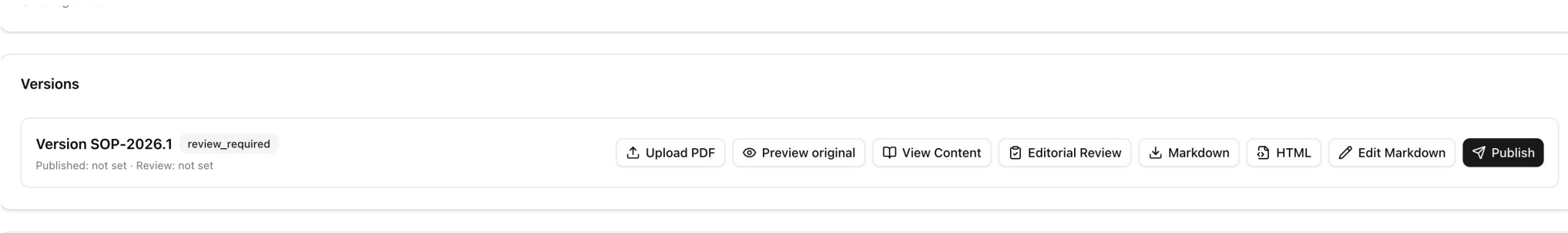
Version-level actions

Document detail > Versions



Check the version row before Upload PDF, Preview original, View Content, Edit Markdown or Editorial Review.

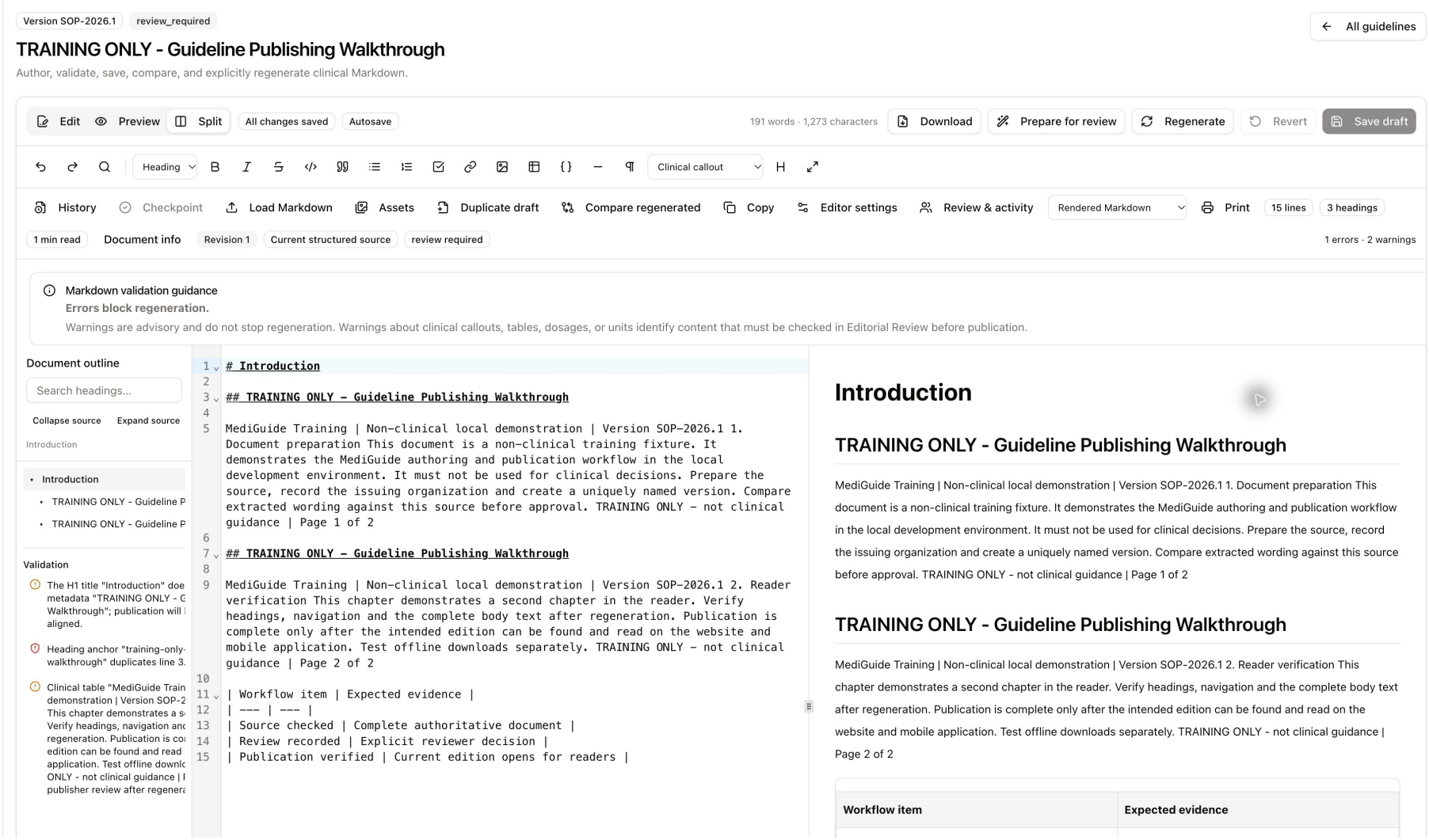
Version-level actions — close-up



Check the version row before Upload PDF, Preview original, View Content, Edit Markdown or Editorial Review.

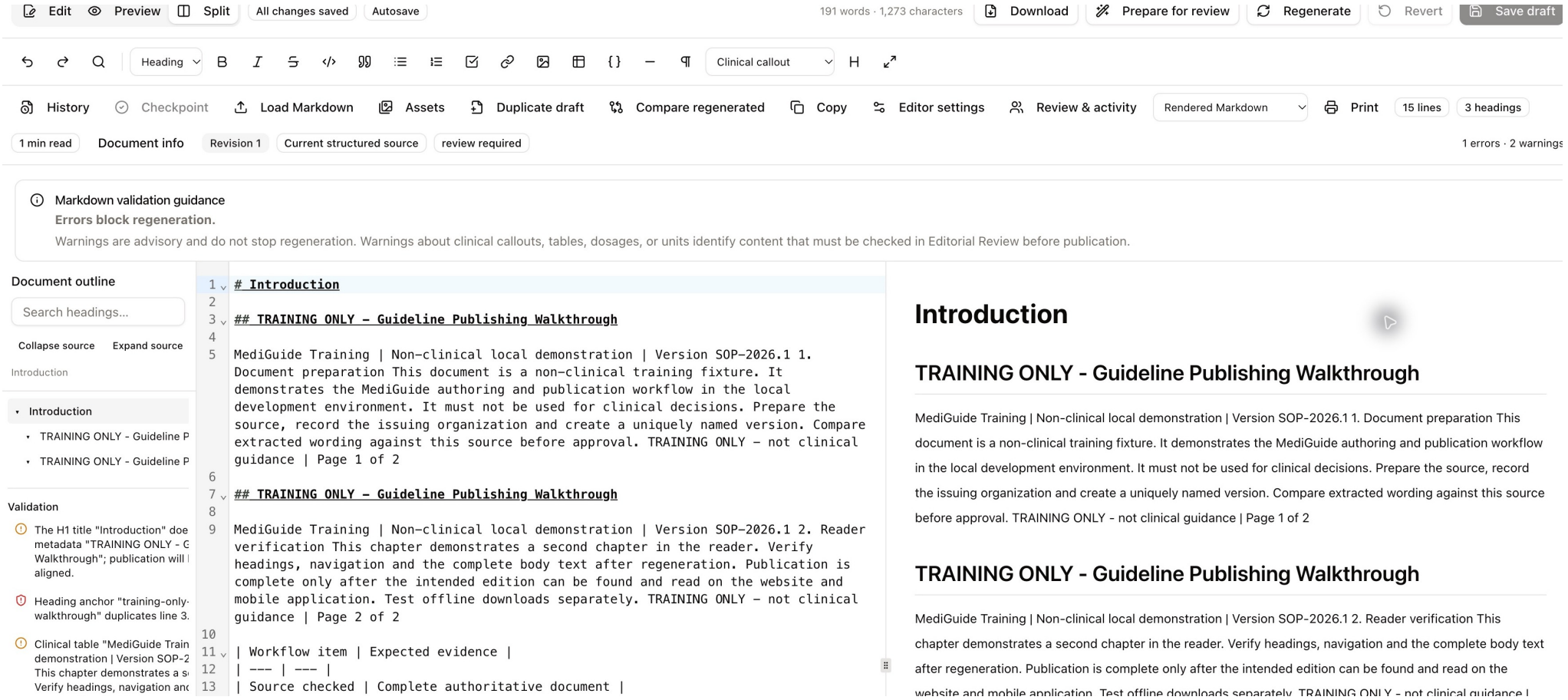
Extraction defects are visible

Edit Markdown > Split



The extracted H1 says Introduction instead of the document title, and headings repeat. Use validation plus the authoritative PDF to correct structure.

Extraction defects are visible — close-up



The extracted H1 says Introduction instead of the document title, and headings repeat. Use validation plus the authoritative PDF to correct structure.

09 Repair headings and reading order

A clear hierarchy becomes the chapter and section navigation on the website and mobile app.

1. Use one document title

Keep one H1 (# Guideline title). Use H2 (##) for chapters or major front matter, H3 for major sections within a chapter and H4 for subsections. Do not skip heading levels.

2. Match the source structure

Preserve the sequence and meaning of chapters. Remove repeated page headers/footers only after checking the source. Keep clinically meaningful notes, references and appendices.

3. Replace training/template content

Replace template titles, unchanged template headings and every _Add reviewed clinical content._ marker. A template provides structure, not clinical wording.

4. Check navigation and anchors

Preview the section outline and internal links. Renamed headings may retain stable anchors from revision metadata; verify that links still lead to the correct clinical section.

Continue when
One coherent title and an unbroken parent-child outline match the source.

Watch point: A normal single H1 acts as the document wrapper; its H2 children become mobile chapter cards. Do not create extra H1 headings just to obtain more cards.

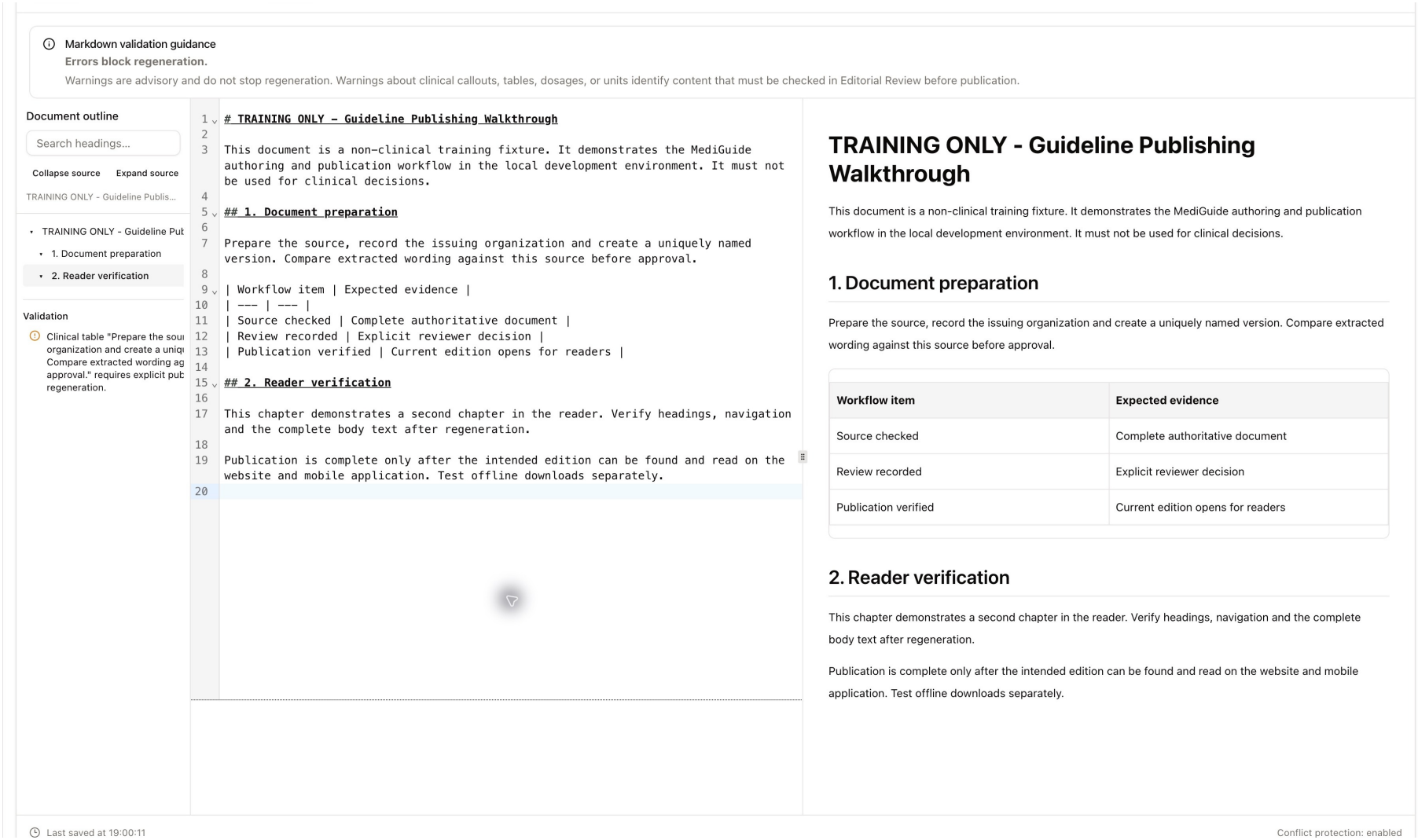
Screen plates follow: Corrected heading hierarchy

Worked correction in the screenshots

Extraction produced “# Introduction” and repeated section labels. The demonstration replaces them with one exact document H1 and two source-matching H2 chapters. The table and the full prose are preserved. Real clinical corrections require source evidence.

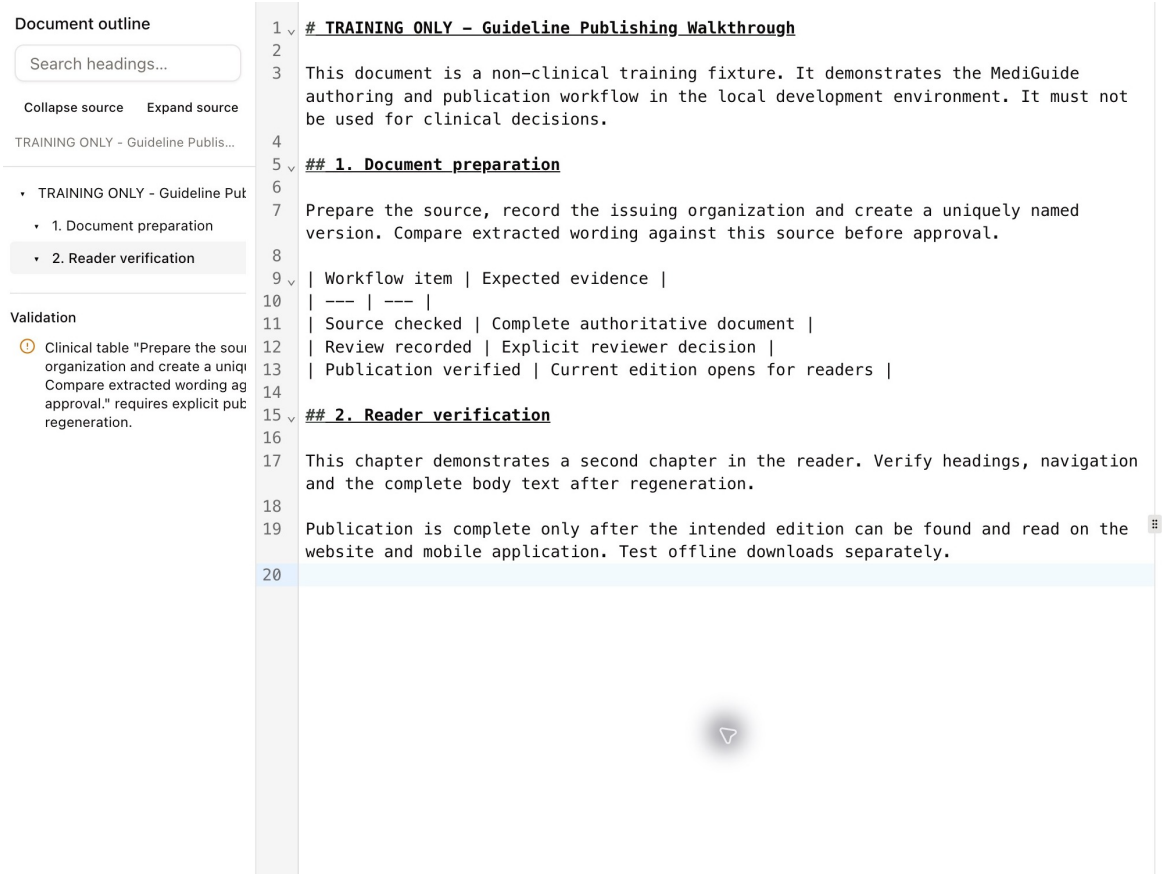
Corrected heading hierarchy

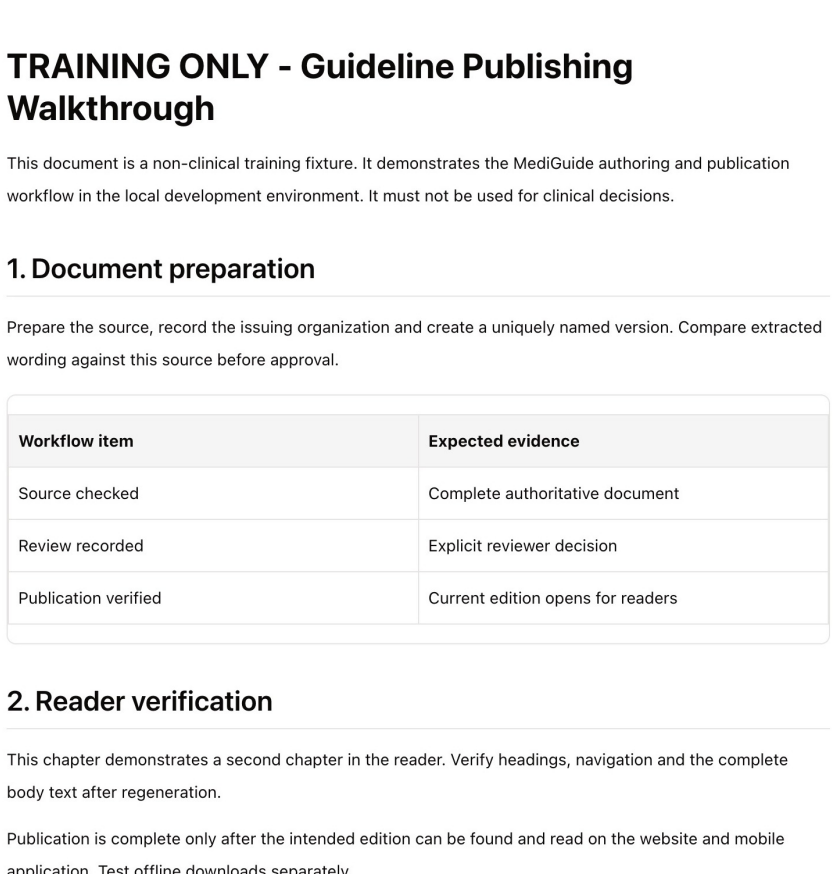
Edit Markdown > Split



One H1 matches the record title; two H2 chapters preserve source order. The warning about a table still requires editorial review.

Corrected heading hierarchy — close-up





TRAINING ONLY - Guideline Publishing Walkthrough

This document is a non-clinical training fixture. It demonstrates the MediGuide authoring and publication workflow in the local development environment. It must not be used for clinical decisions.

1. Document preparation

Prepare the source, record the issuing organization and create a uniquely named version. Compare extracted wording against this source before approval.

Workflow item	Expected evidence
Source checked	Complete authoritative document
Review recorded	Explicit reviewer decision
Publication verified	Current edition opens for readers

2. Reader verification

This chapter demonstrates a second chapter in the reader. Verify headings, navigation and the complete body text after regeneration.

Publication is complete only after the intended edition can be found and read on the website and mobile application. Test offline downloads separately.

One H1 matches the record title; two H2 chapters preserve source order. The warning about a table still requires editorial review.

10 Upload and reconnect images

Use Assets in the editor to manage version-scoped images and their attribution.

1. Gather the exact figures

Obtain the source images or extract them from the authoritative PDF. Check charts, axes, legends and labels. Do not substitute a similar chart when the required original is missing.

2. Complete asset metadata

Open Assets > Guideline asset library. Choose, drop or paste images. Supported files are PNG, JPEG, GIF and WebP, up to 10 MB each. Provide meaningful alternative text; add caption, source, attribution and license as applicable.

3. Insert the correct reference

Upload the image, inspect its preview and insert it where it belongs. Prefer the stable guideline-asset:// reference returned by the library. Resolve Broken references and ambiguous filenames before regeneration.

4. Review clinical figures

Mark clinically sensitive figures appropriately. Review the asset and generated figure against the source. A figure is not ready for publication simply because its image loads.

Continue when
Every intended image resolves to the correct version asset with meaningful alternative text.

Watch point: Replacing a clinical figure requires a fresh source check. Missing or unreviewed referenced assets can block publication.

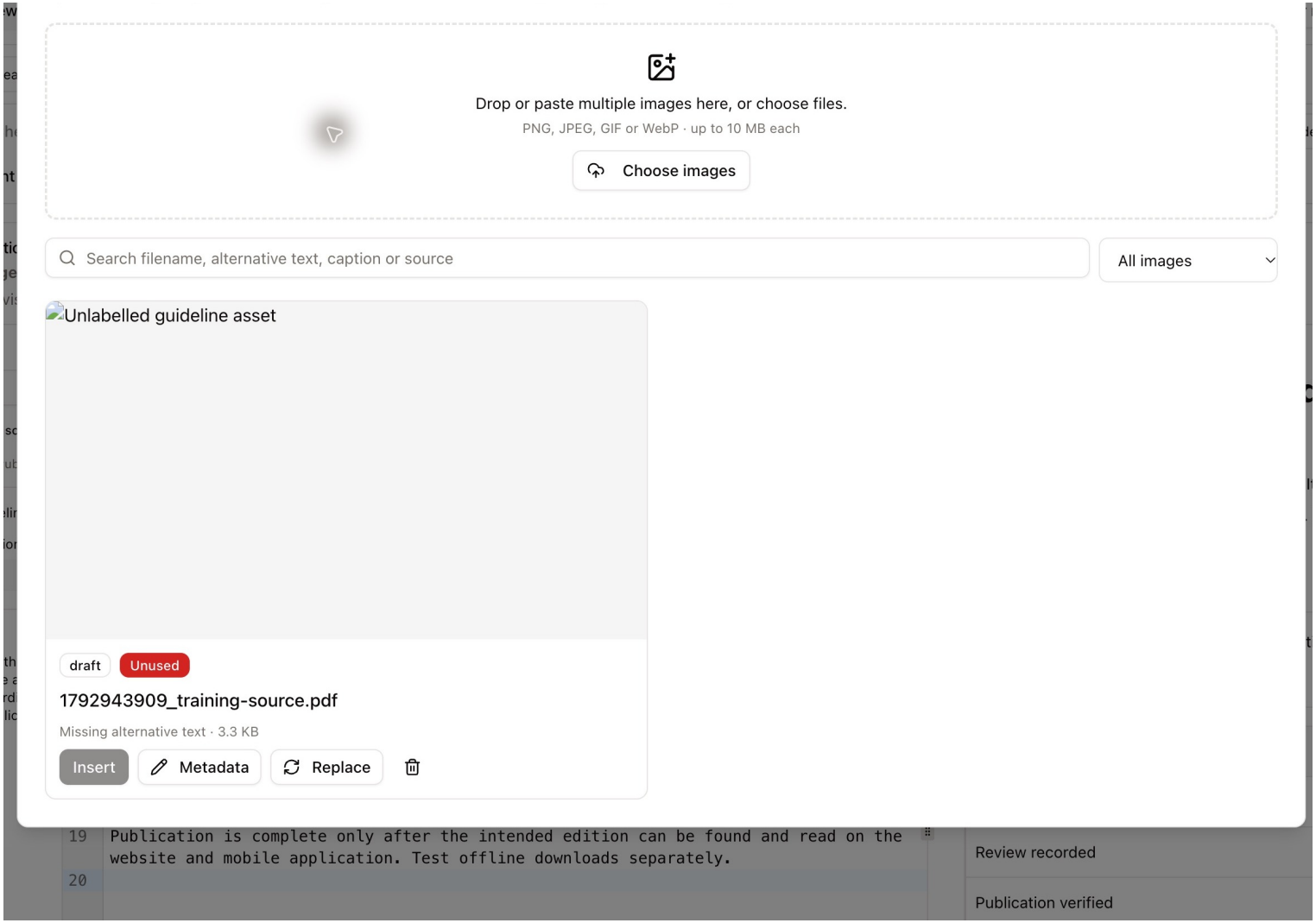
Screen plates follow: Version asset library; Image metadata before upload; Uploaded image ready to insert

Placement and reference check

Put the insertion cursor at the intended location before Insert. Confirm the Markdown contains the intended guideline-asset:// reference and that the rendered preview loads the same figure. Remove accidental duplicate references only after checking context.

Version asset library

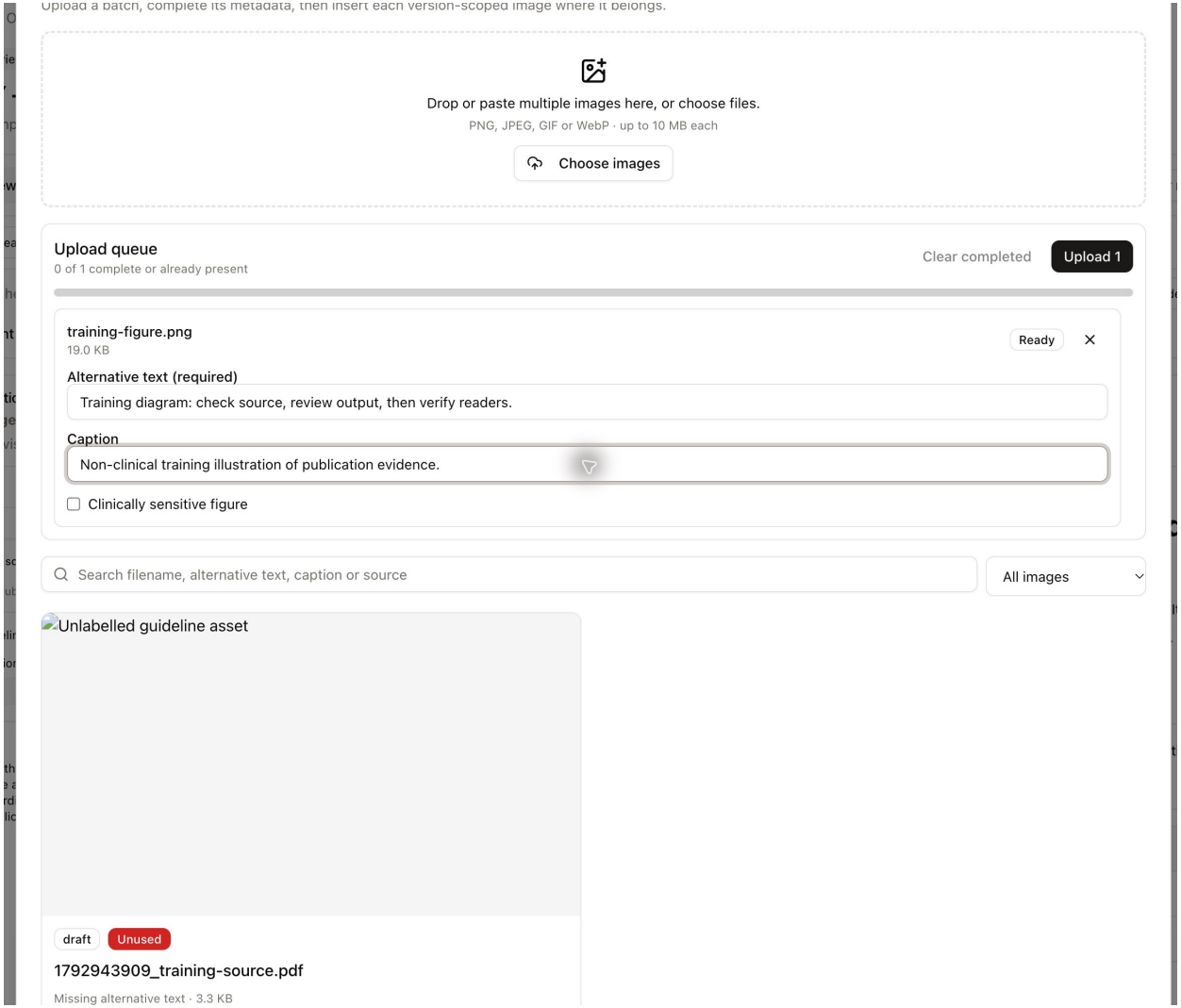
Assets > Guideline asset library



An extracted asset is already present. Inspect it before uploading or inserting any replacement.

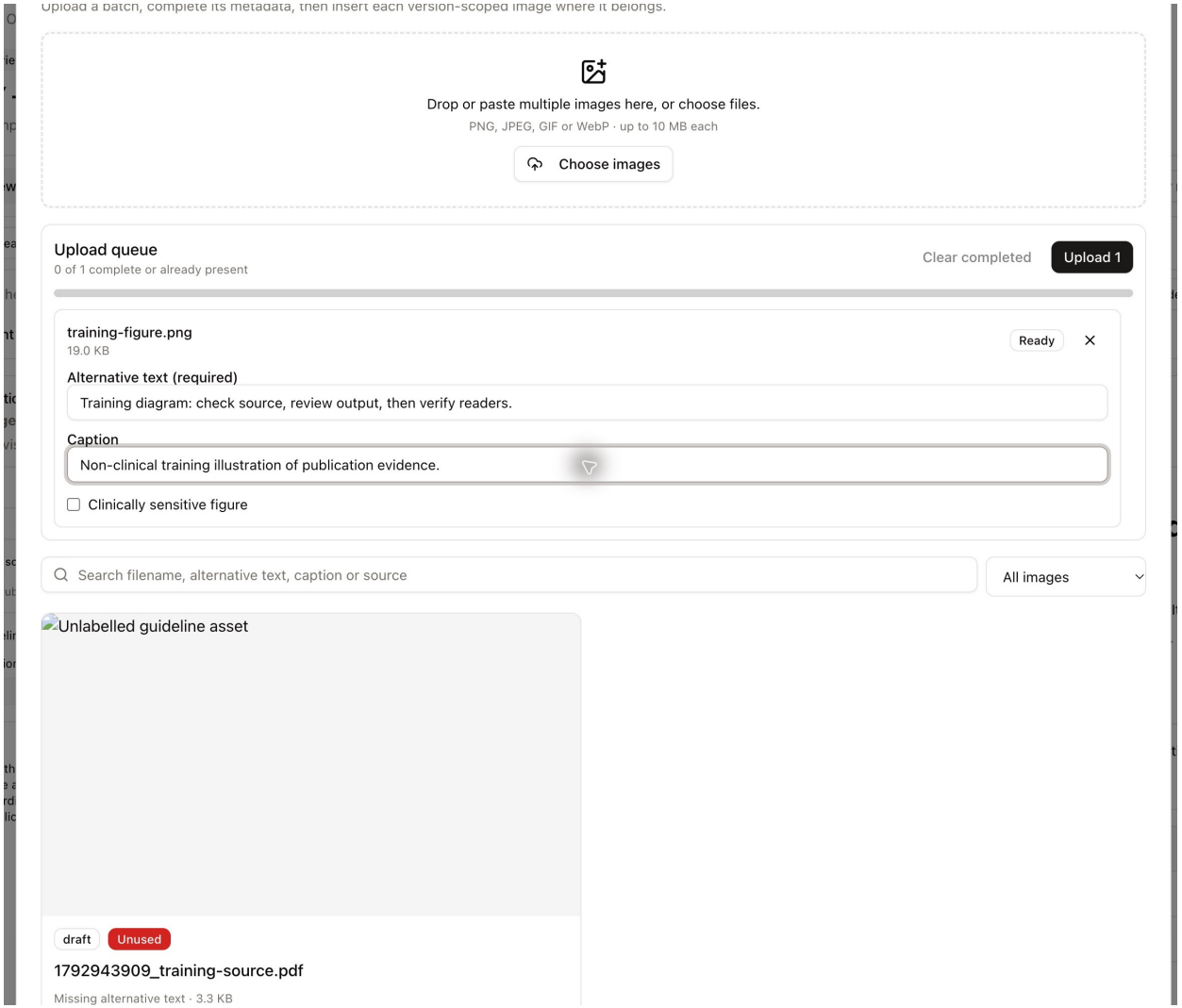
Image metadata before upload

Assets > Upload panel



The figure file, alternative text and caption are filled in. Review source, attribution and license where applicable; flag clinical sensitivity accurately.

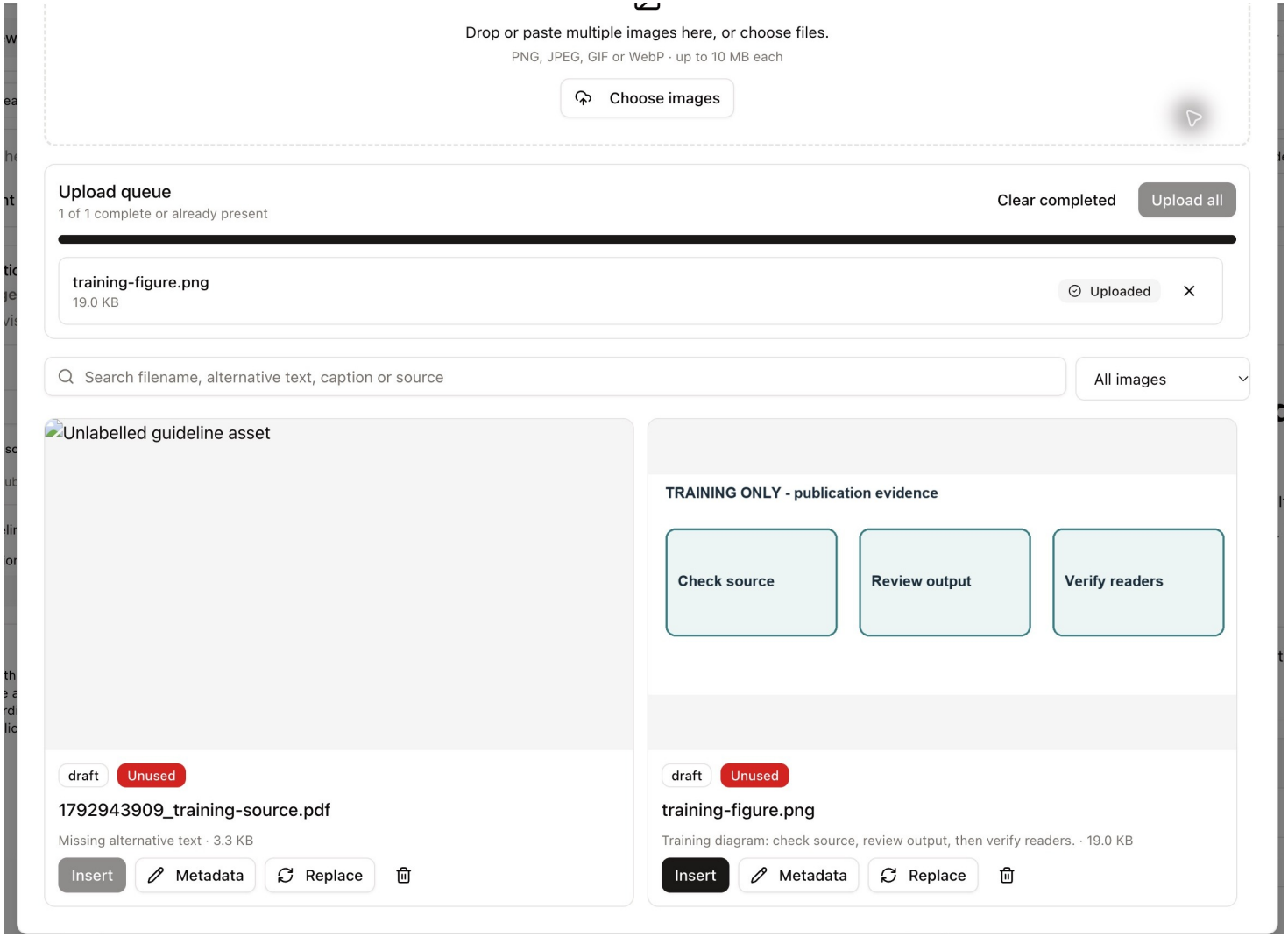
Image metadata before upload — close-up



The figure file, alternative text and caption are filled in. Review source, attribution and license where applicable; flag clinical sensitivity accurately.

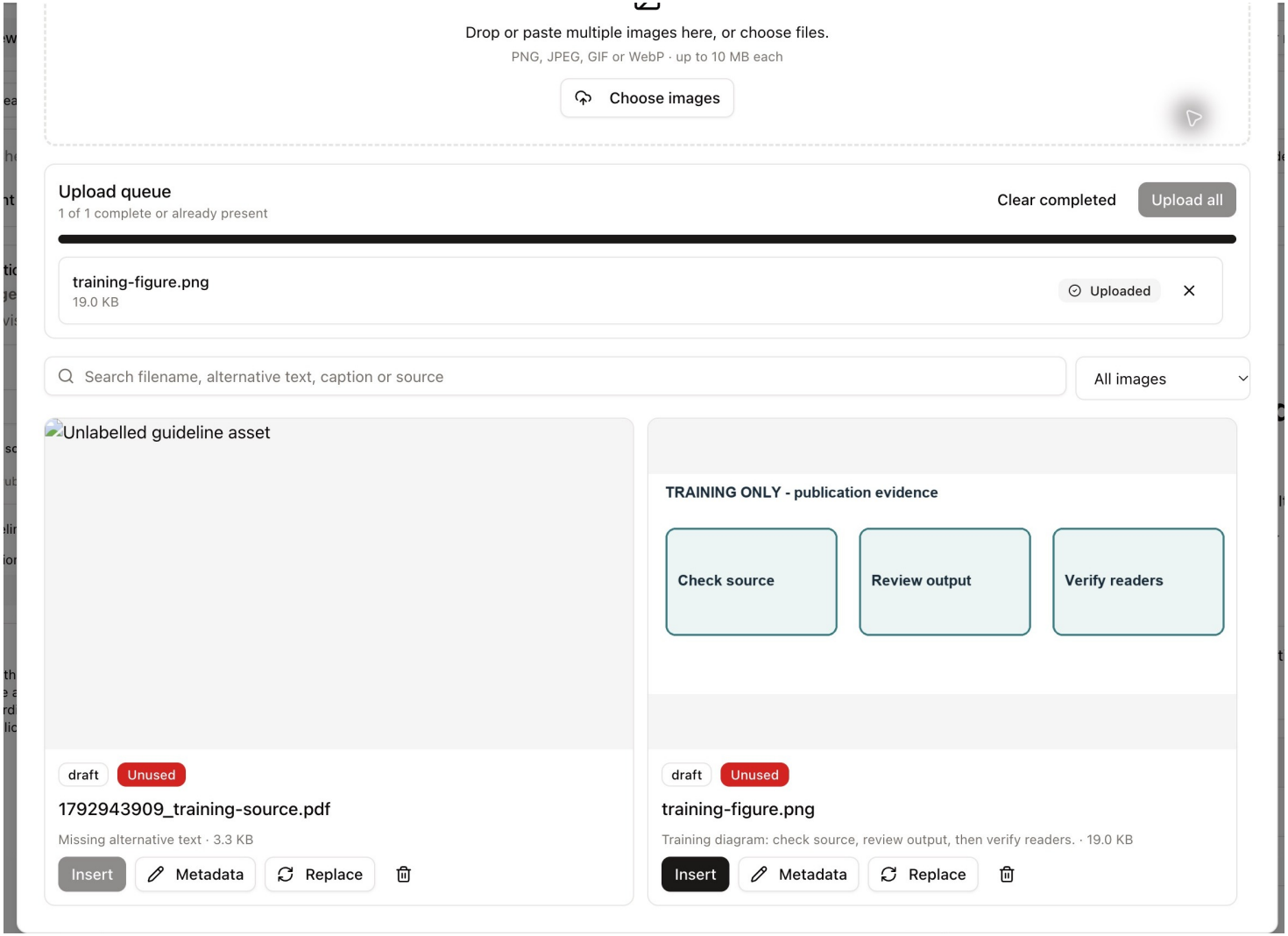
Uploaded image ready to insert

Guideline asset library



Inspect the preview and metadata, then Insert beside the intended asset. The editor inserts a stable guideline-asset reference.

Uploaded image ready to insert — close-up



Inspect the preview and metadata, then Insert beside the intended asset. The editor inserts a stable guideline-asset reference.

11 Repair and verify clinical tables

Table formatting and clinical correctness are separate checks. Both must pass.

1. Preserve the cell grid

Each Markdown table needs a header row, a separator row and an equal number of cells in every row. Keep empty leading or trailing cells and escape literal pipes inside cell text when needed.

2. Use the visual table editor

Inspect the whole grid before adding, deleting or moving rows/columns. Preserve the source headings, merged-cell meaning, notes and reading order. Verify rendered output after each structural edit.

3. Check every clinical detail

Compare headers, row associations, values, decimal points, units, age/weight groups, routes, intervals, footnotes, caption and source page. Never infer a missing dose or unit to make a table look complete.

4. Save and regenerate

A valid table may still show high_risk_table_review_required. Save the source, regenerate, then approve the generated table individually in Editorial Review after source comparison.

Continue when
The rendered grid is faithful to the authoritative table, including intentionally blank cells.

Watch point: Do not remove a row or warning merely to reach zero warnings. A syntactically valid table can still be clinically wrong.

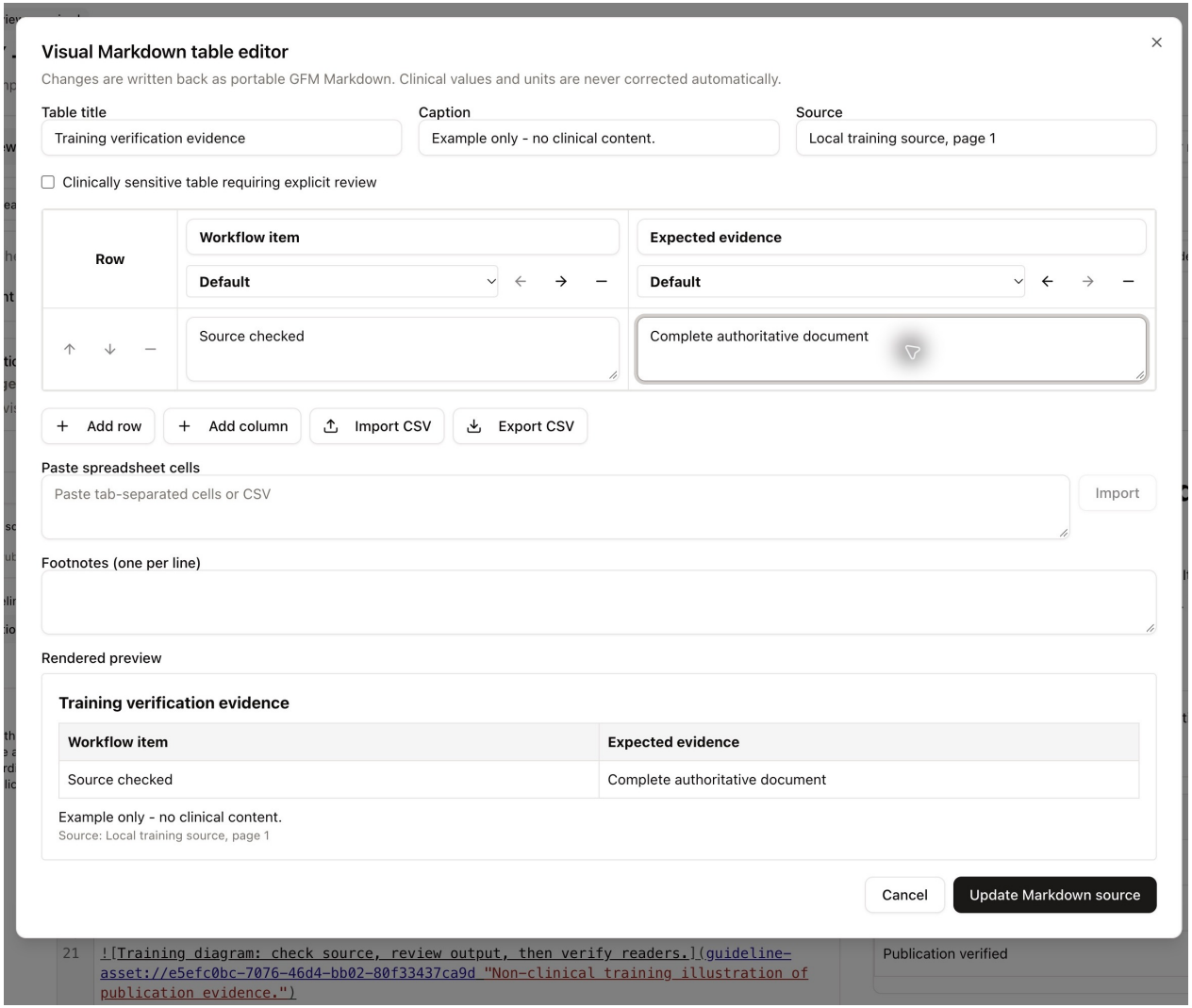
Screen plates follow: Visual Markdown table editor

Table dialog controls

Title describes the table; Caption preserves explanatory context; Source records provenance. Set each header and its alignment, then add or remove rows/columns only as required by the source. CSV controls affect the grid; Footnotes retain qualifying notes. Inspect Preview before Update Markdown table. Cancel abandons the proposed table edit.

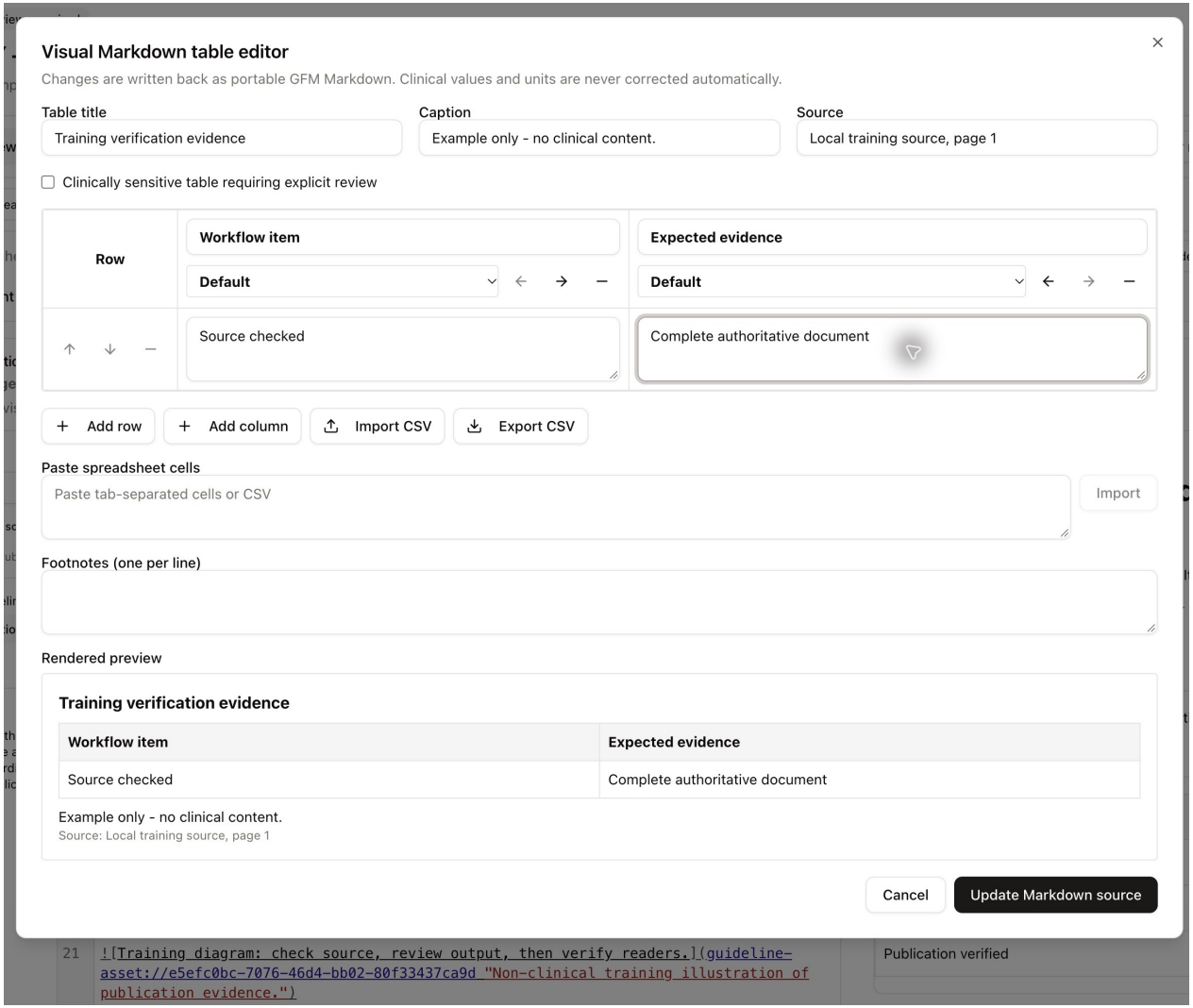
Visual Markdown table editor

Formatting toolbar > Table



This is a newly composed example table: title, caption, source, headers, row and footnote controls. It was canceled rather than applied to the training source.

Visual Markdown table editor — close-up



This is a newly composed example table: title, caption, source, headers, row and footnote controls. It was canceled rather than applied to the training source.

12 Prepare, save and checkpoint

Save the exact source you intend to regenerate. Autosave and checkpoints are not publication actions.

1. Inspect preparation proposals

Select Prepare for review. Review its before/after diff and Automatic, Editor review and Manual review categories. Apply only changes that preserve source meaning; preparation does not approve clinical content.

2. Resolve blocking errors

Use the validation panel and source-line guidance to fix malformed tables, unsafe/unsupported markup, missing alternative text, broken links and invalid hierarchy. Warnings about tables and doses usually remain review obligations.

3. Save the revision

Select Save draft and confirm the revision badge updates. Regenerate is disabled while changes are unsaved. A saved revision is immutable; saving newer source makes the previous structured projection outdated.

4. Create a named milestone

For a substantial change, use Checkpoint and describe the change. If a conflict occurs, preserve/export your local work, load the current server revision and reconcile differences; do not overwrite another author blindly.

Continue when
The intended source is saved on the server and has no regeneration-blocking errors.

Watch point: Local settings and local drafts are not a substitute for a confirmed server save. Do not regenerate from an older revision.

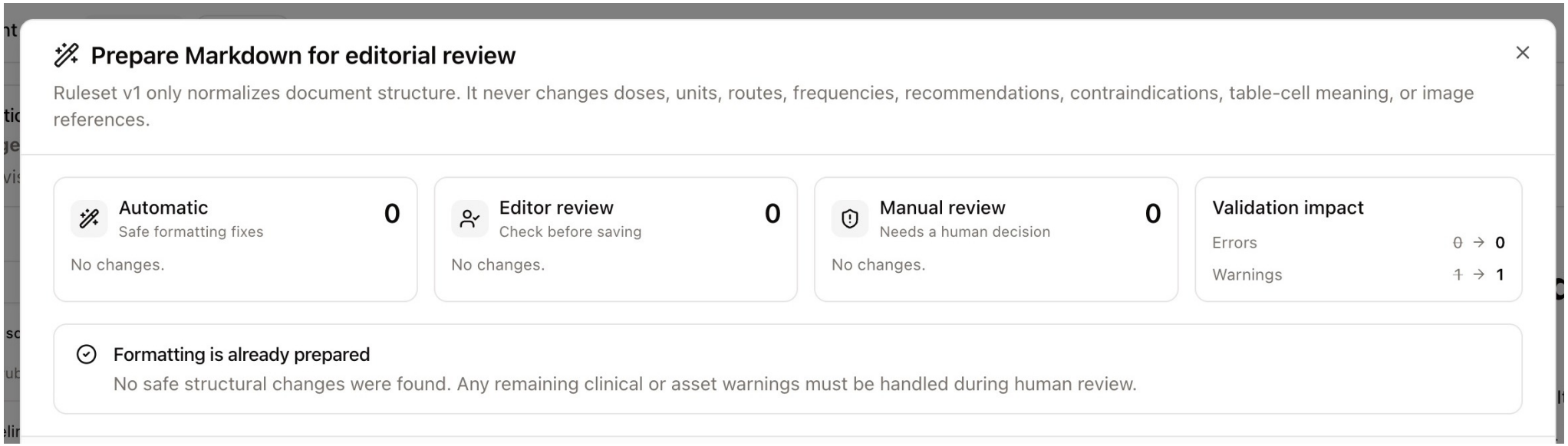
Screen plates follow: Prepare for review is a proposal; Saved source with stable image reference; Immutable revision history; Compare source revisions

Practical save sequence

After typing, wait for All changes saved or use Save draft when enabled. Read the revision number. Create a meaningful Checkpoint before a large change when the control is available. Use History to compare the relevant revisions; restoration creates a new source state that needs fresh regeneration and review.

Prepare for review is a proposal

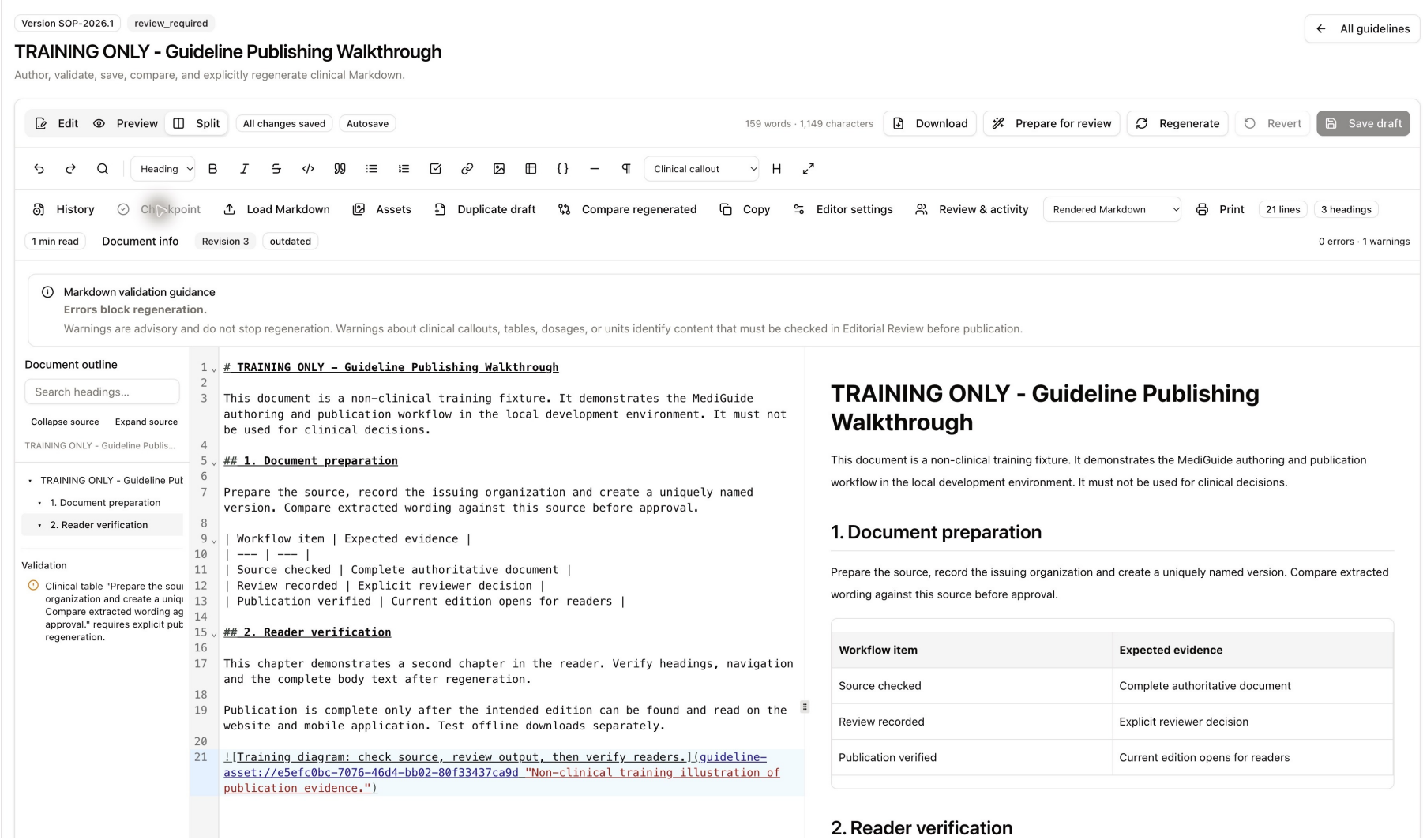
Prepare for review dialog



The dialog has Automatic, Editor review and Manual review categories. This example proposes no changes; no clinical approval is performed.

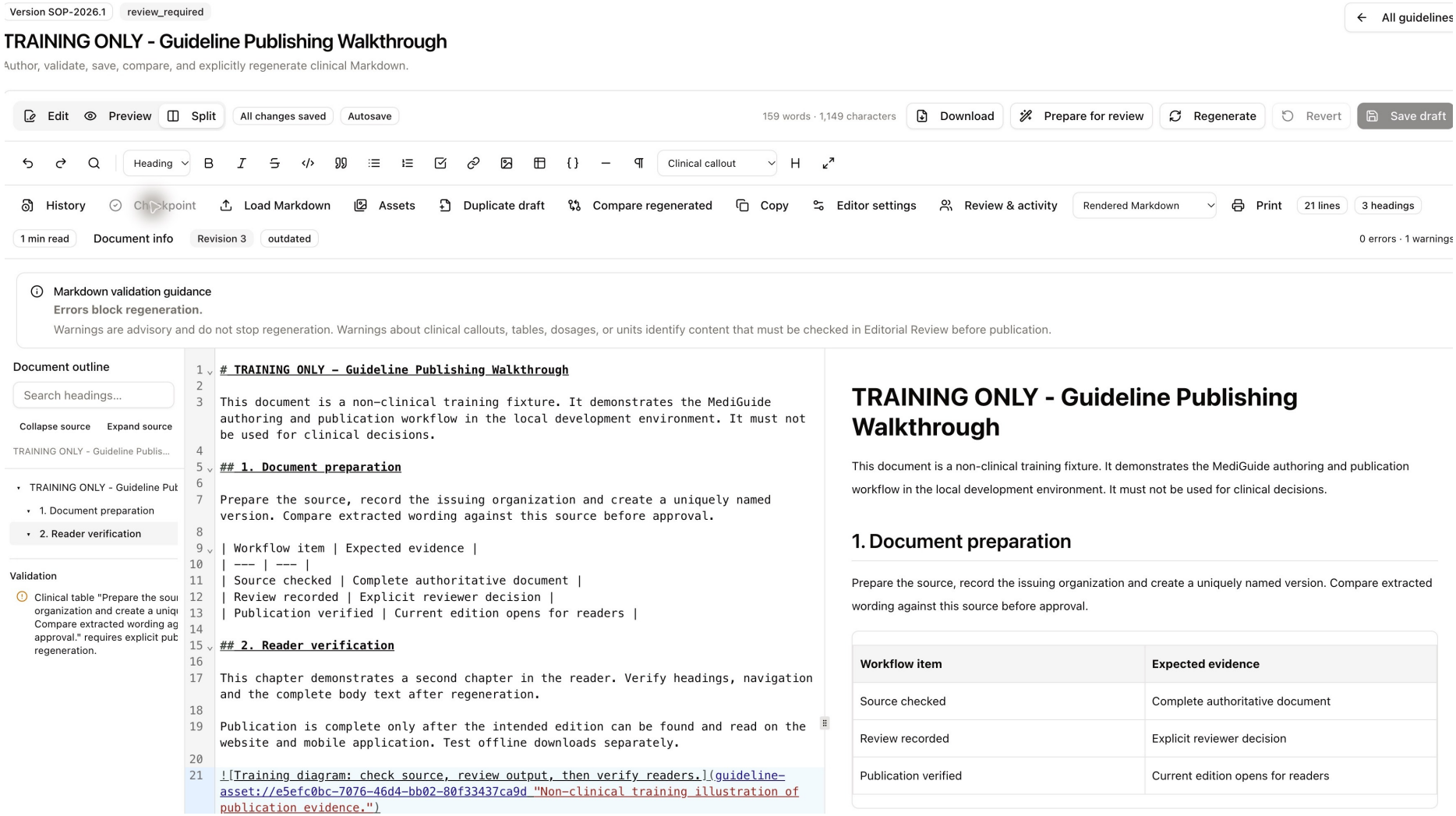
Saved source with stable image reference

Edit Markdown > Split



The new guideline-asset reference is part of the saved source. Saving it does not regenerate or approve the structured figure.

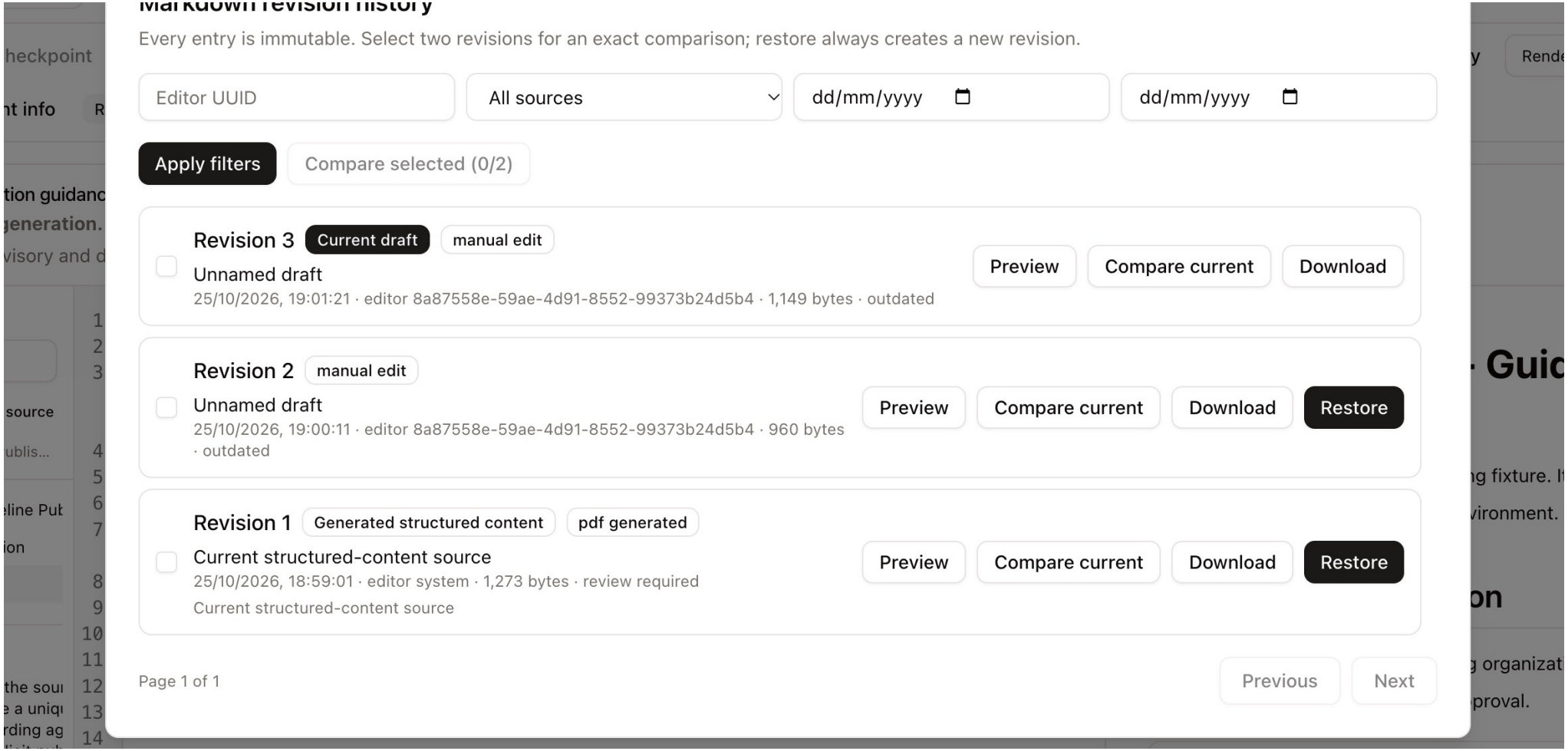
Saved source with stable image reference — close-up



The new guideline-asset reference is part of the saved source. Saving it does not regenerate or approve the structured figure.

Immutable revision history

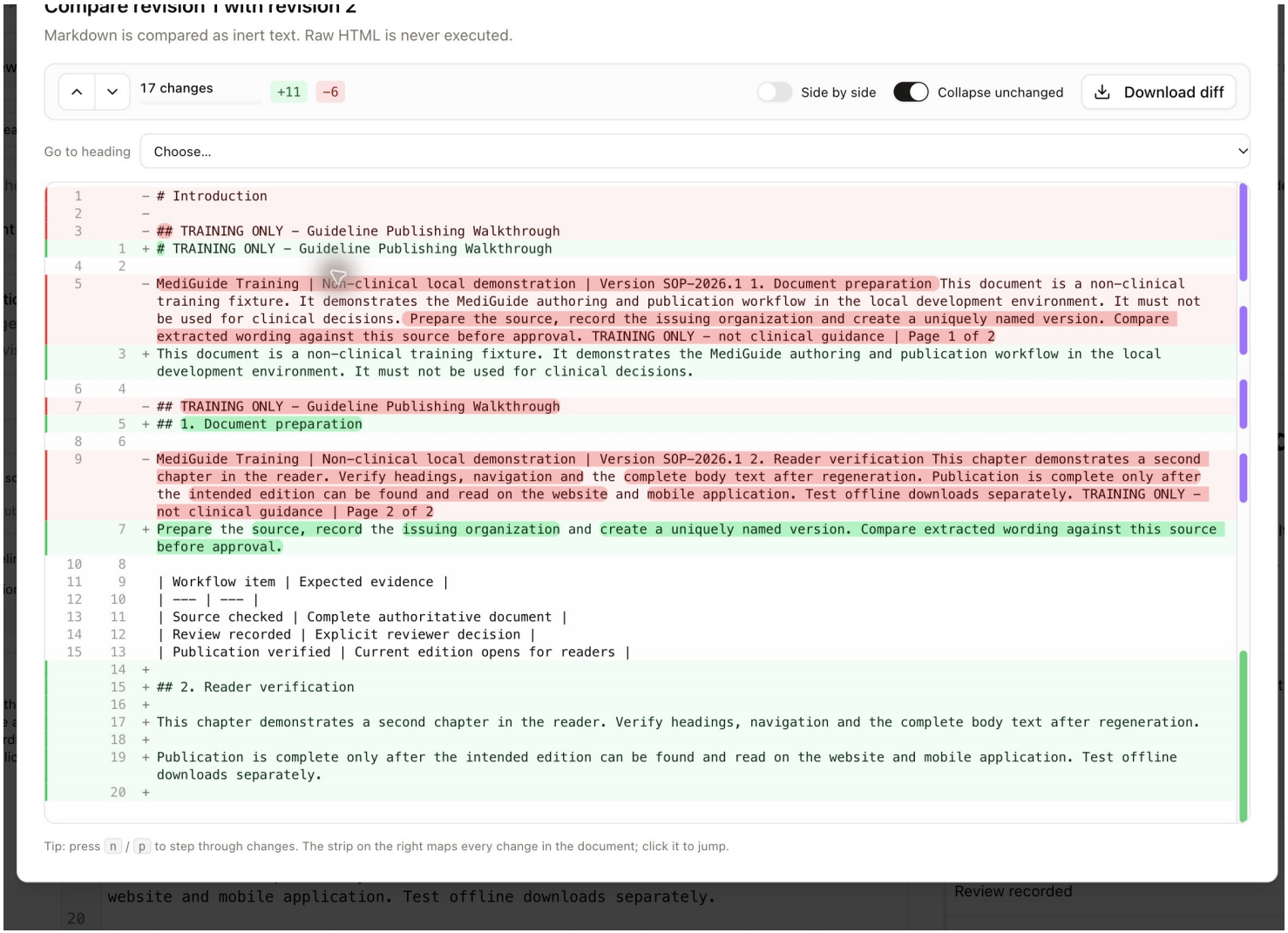
History > Markdown revision history



Rows identify revision numbers, source and time. Use Preview or Compare before deciding to restore an earlier source.

Compare source revisions

History > Compare



This actual diff compares extraction revision 1 with corrected revision 2. Red is removed source; green is added source. A diff does not establish clinical correctness.

13 Assign reviewers and record decisions

Review & activity coordinates work and retains comments, assignments and the audit trail.

1. Assign responsibility

Open Review & activity > Reviewer assignments. Select an active clinical reviewer, optionally set a due date, then Assign reviewer. If the list is empty, ask an administrator to check active status and guideline.review access.

2. Make requests traceable

Use comments to record source discrepancies, missing material and requested corrections. Refer to the version, revision, section or block and source page whenever available.

3. Resolve collaboration work

Resolve or reopen comments as the source issue is checked. Refresh or poll to see others' work; the editor does not provide guaranteed live cursors or realtime presence.

4. Keep review separate

Completing an assignment or resolving a comment does not mark a block reviewed. High-risk blocks still need the explicit decision by an authorized reviewer, followed by regeneration acceptance and publication validation.

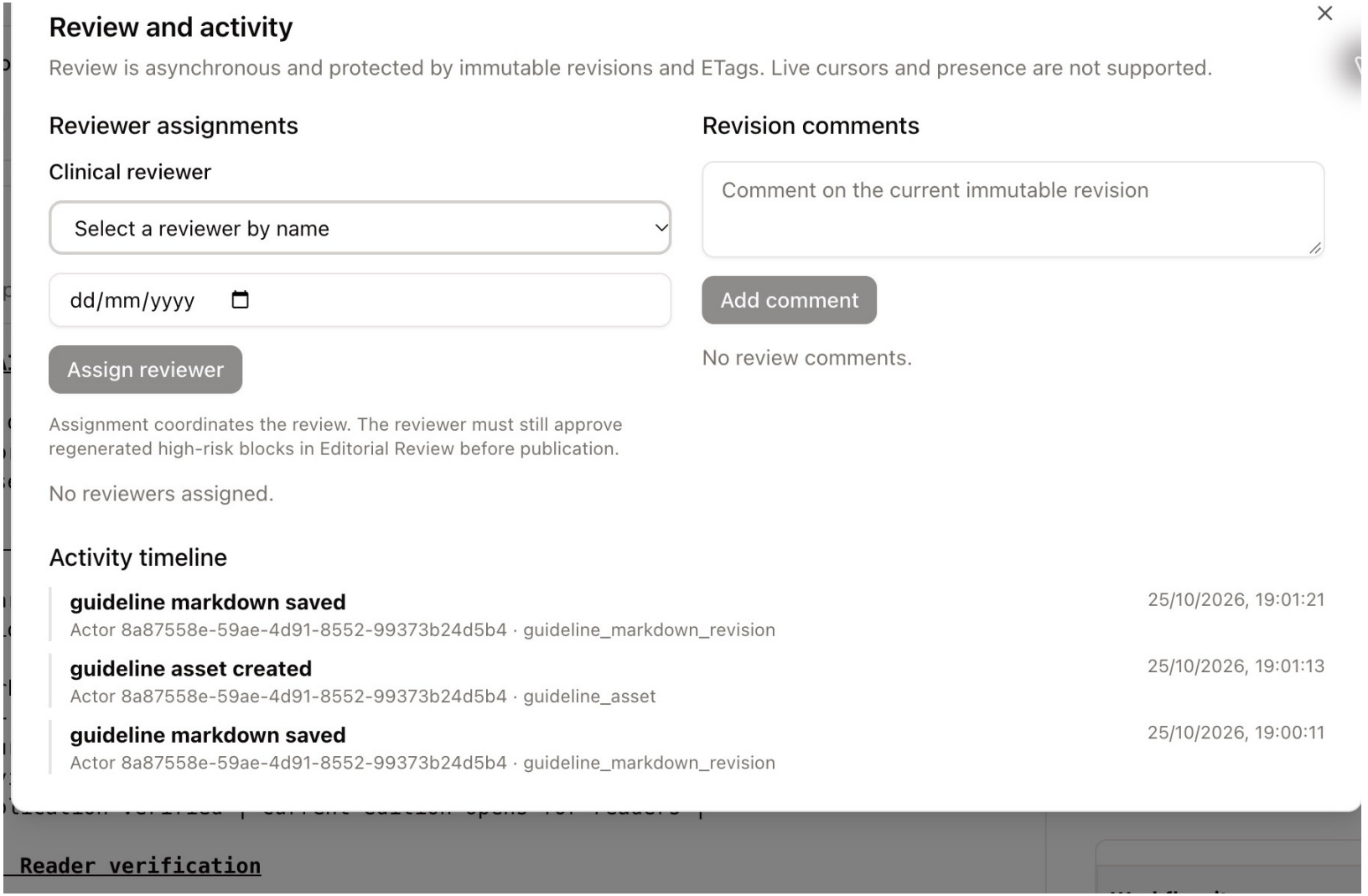
Continue when
Reviewer responsibility and unresolved questions are visible and attributable.

Watch point: Do not place patient identifiers, passwords or unrelated personal information in review comments.

Screen plates follow: Reviewer coordination and audit

Reviewer coordination and audit

Review & activity



Reviewer assignments, comments and Activity log live here. An assignment or resolved comment is separate from a block decision.

14 Regenerate the saved source

Regeneration rebuilds the reader projection and search material from one immutable Markdown revision.

1. Start from the saved revision

Confirm Save draft completed and authoritative Markdown validation has no blocking errors. Select Regenerate in the toolbar (described as Regenerate structured content in some guides).

2. Monitor the revision-bound job

Watch the stage and percentage until generation completes or requires review. Check that the job refers to the revision you intend to publish. Do not treat successful generation as clinical approval.

3. Handle newer edits

If Regeneration stopped: newer revision appears, use Reload current revision, check the saved source and start a new regeneration. Retrying the superseded job is intentionally disabled.

4. Handle a failure

Use Retry only when it is offered for the current eligible job and the cause is understood. Record a persistent error for operations. Review generated blocks again after regeneration; do not assume prior decisions were inherited.

Continue when
A generated projection exists for the current saved revision, ready for review.

Watch point: If source changes after review begins, save, regenerate and repeat the review/acceptance cycle for the new revision.

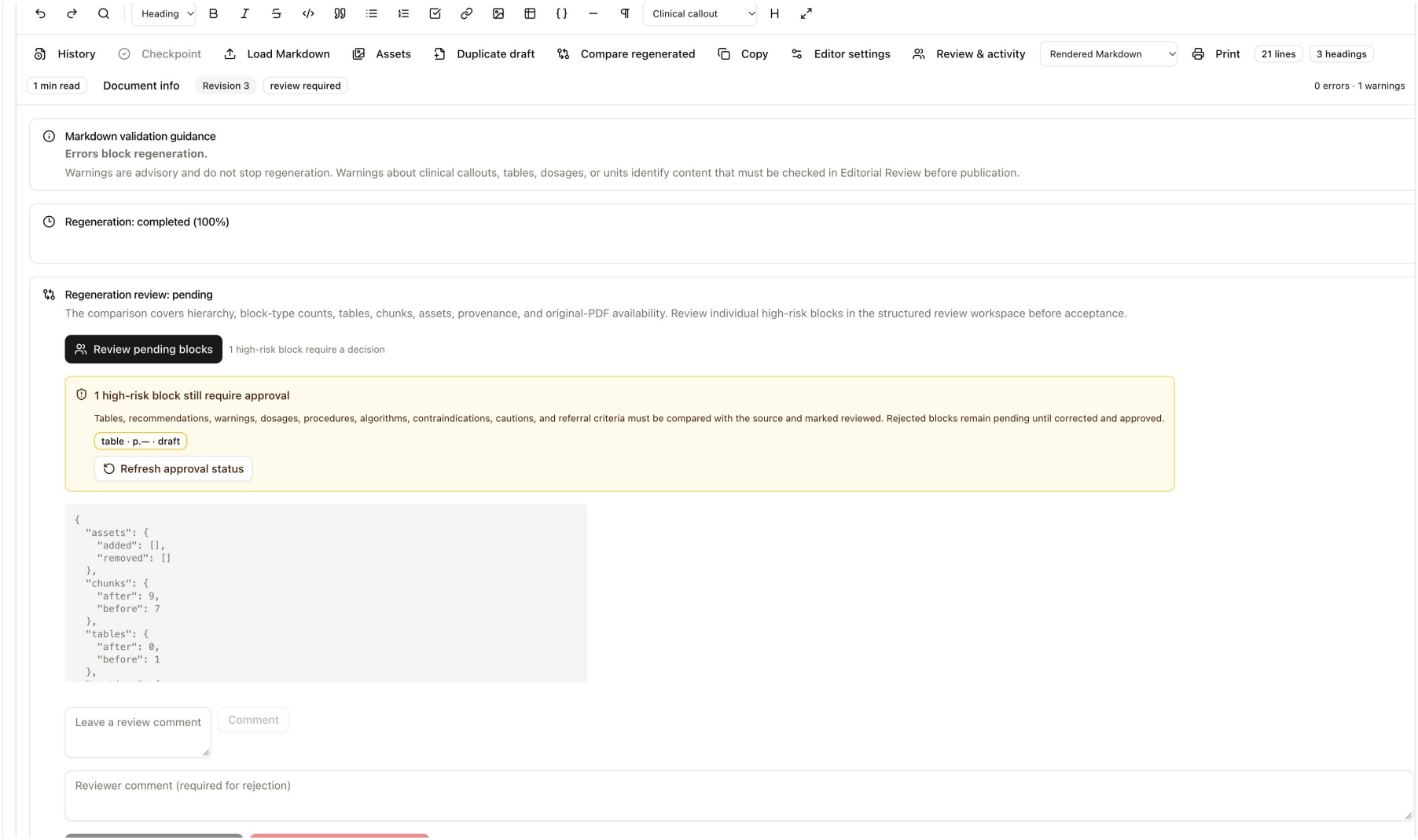
Screen plates follow: Generated, pending review

Read the comparison critically

The training comparison’s legacy table counter says after 0, while the actual structured Review and Completeness report contain one reviewed table. Counts alone are insufficient evidence of loss or completeness. Compare the real sections/blocks, report and source; investigate any unexplained mismatch before publishing.

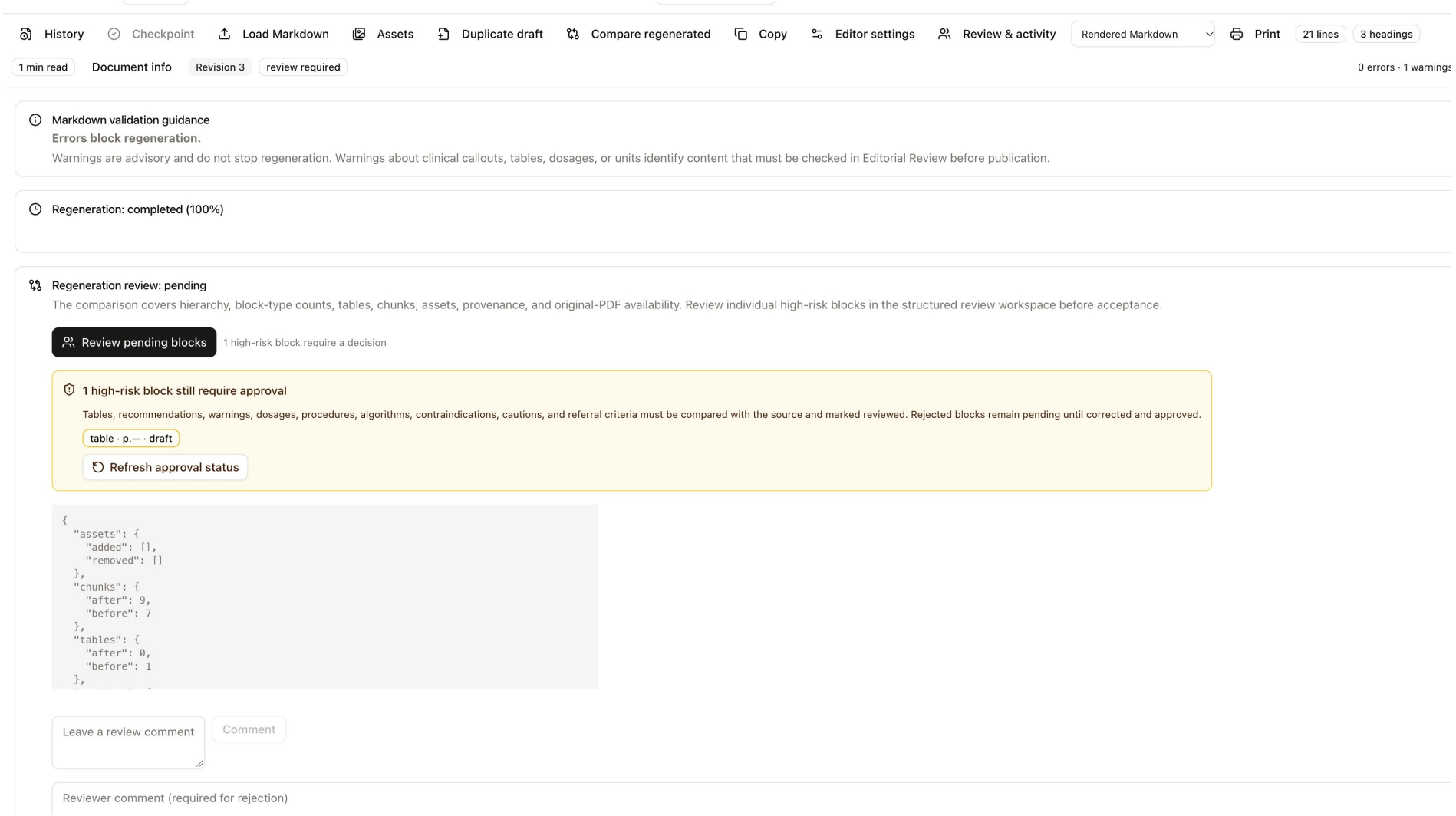
Generated, pending review

Regenerate > regeneration comparison



Generation completed for revision 3. Acceptance remains disabled until the required blocks are reviewed. Comparison counts need confirmation in the actual rendered output.

Generated, pending review — close-up



Generation completed for revision 3. Acceptance remains disabled until the required blocks are reviewed. Comparison counts need confirmation in the actual rendered output.

15 Navigate Editorial Review

Document > draft version > Editorial Review. The workspace opens at the first pending high-risk block.

1. Understand the workspace

The left column is Sections; the middle is Source PDF or Reader preview; the right is the block list and selected block. Narrow screens use Sections / Source / Review block tabs.

2. Choose the right filter

Pending review only targets outstanding individual decisions. All high-risk blocks includes approved safety-sensitive content. All blocks is essential for checking ordinary prose and surrounding context.

3. Compare before deciding

Select a section and block, confirm its source page/provenance and compare the rendered output with the original. Use Review next pending to move through the queue.

4. Use header controls

The progress bar summarizes individual review. The blockers button opens Publication checklist. Validate reruns publication checks. Progress and checklist counts can differ; the authoritative publication validator controls readiness.

Continue when
You can locate the original source, rendered block and review status for the same section.

Watch point: No pending high-risk blocks does not prove that ordinary paragraphs, lists or entire sections have been reviewed.

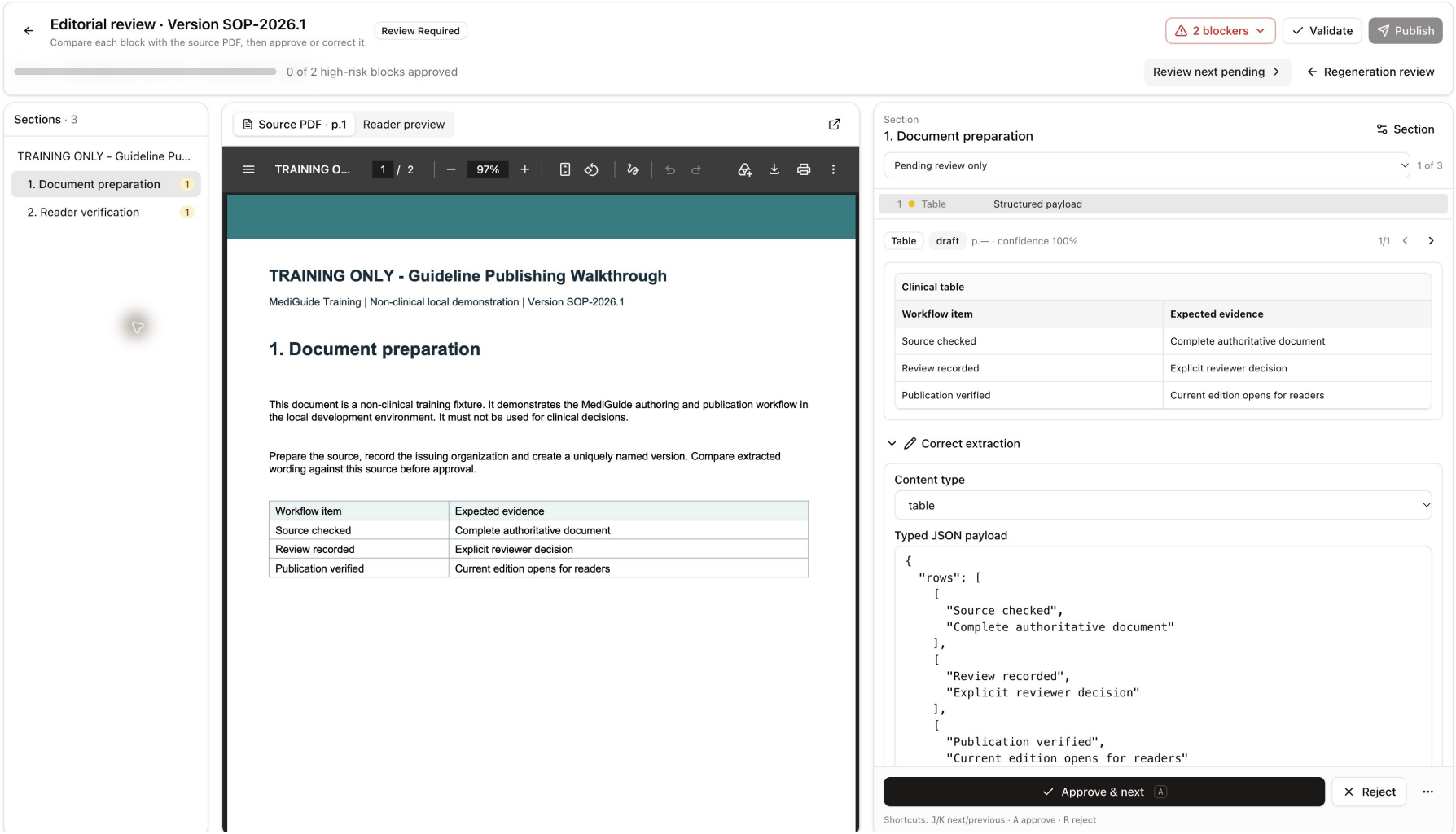
Screen plates follow: Source PDF beside the table; Publication blockers grouped by issue

If the tab strip is covered

In this capture session the sticky header overlapped the Review / Bulk approve low-risk / Completeness report strip. Use keyboard Tab or Shift+Tab until the selected tab receives focus, then Left/Right to choose the tab and Enter if needed. This navigates the existing UI without skipping any review gate.

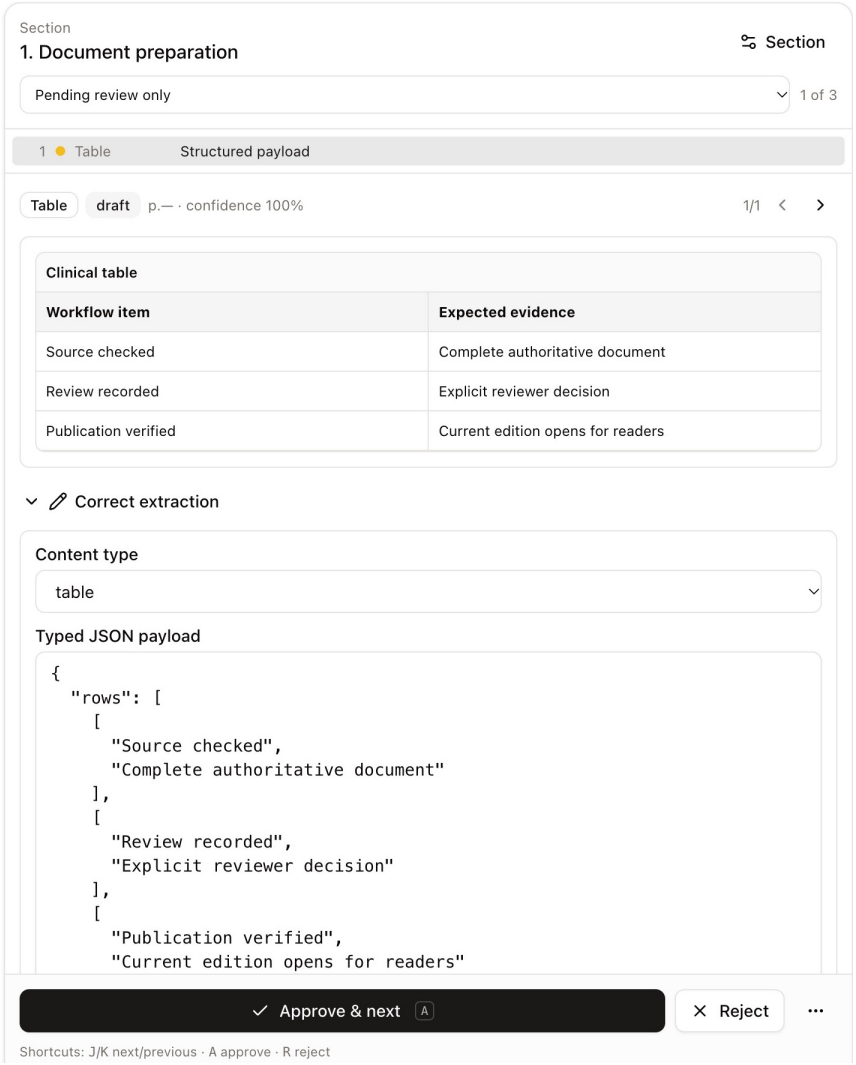
Source PDF beside the table

Editorial Review > Review



Sections are on the left, original PDF in the middle, rendered block and actions on the right. The table content can be compared cell by cell.

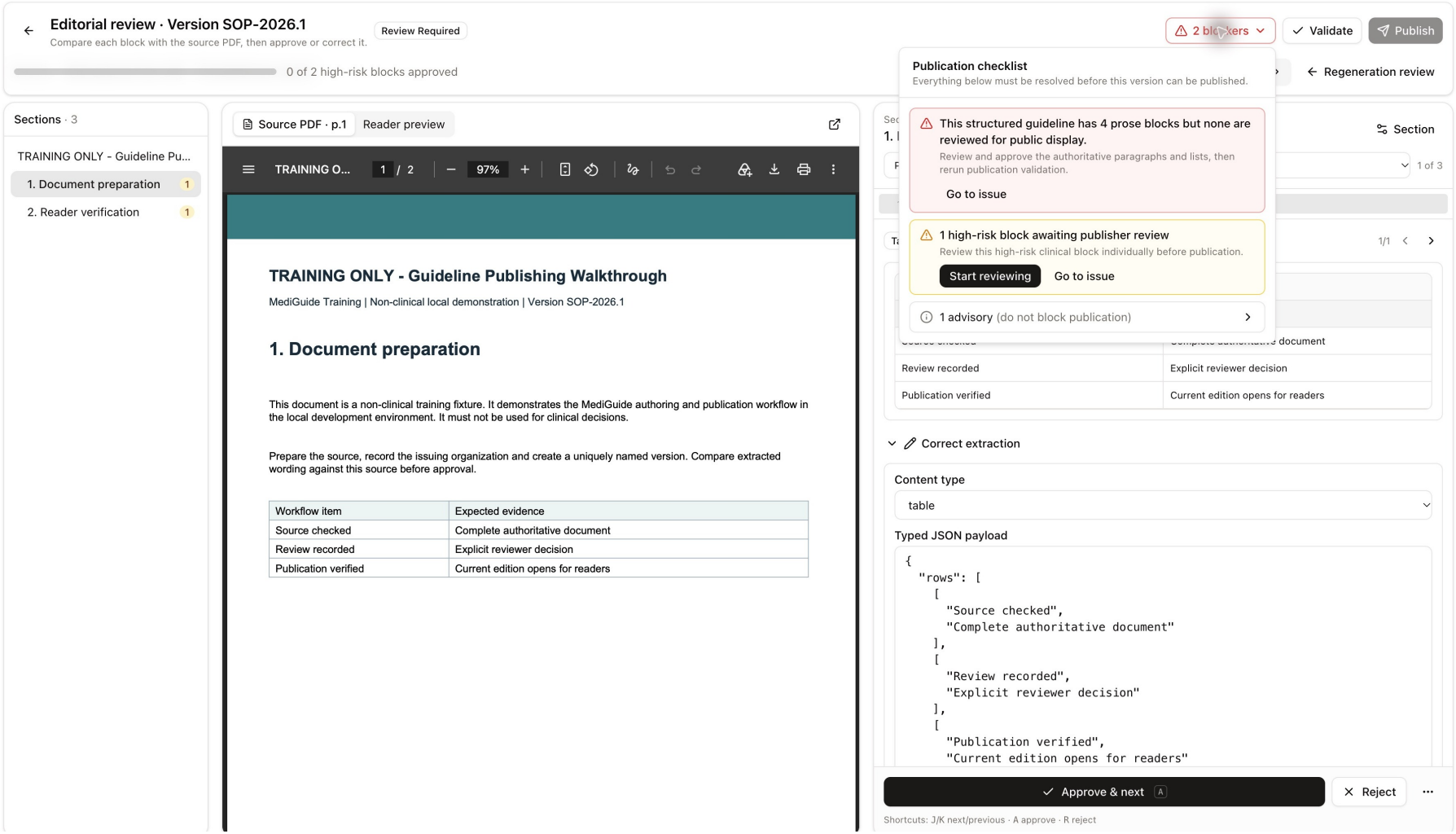
Source PDF beside the table — close-up



Sections are on the left, original PDF in the middle, rendered block and actions on the right. The table content can be compared cell by cell.

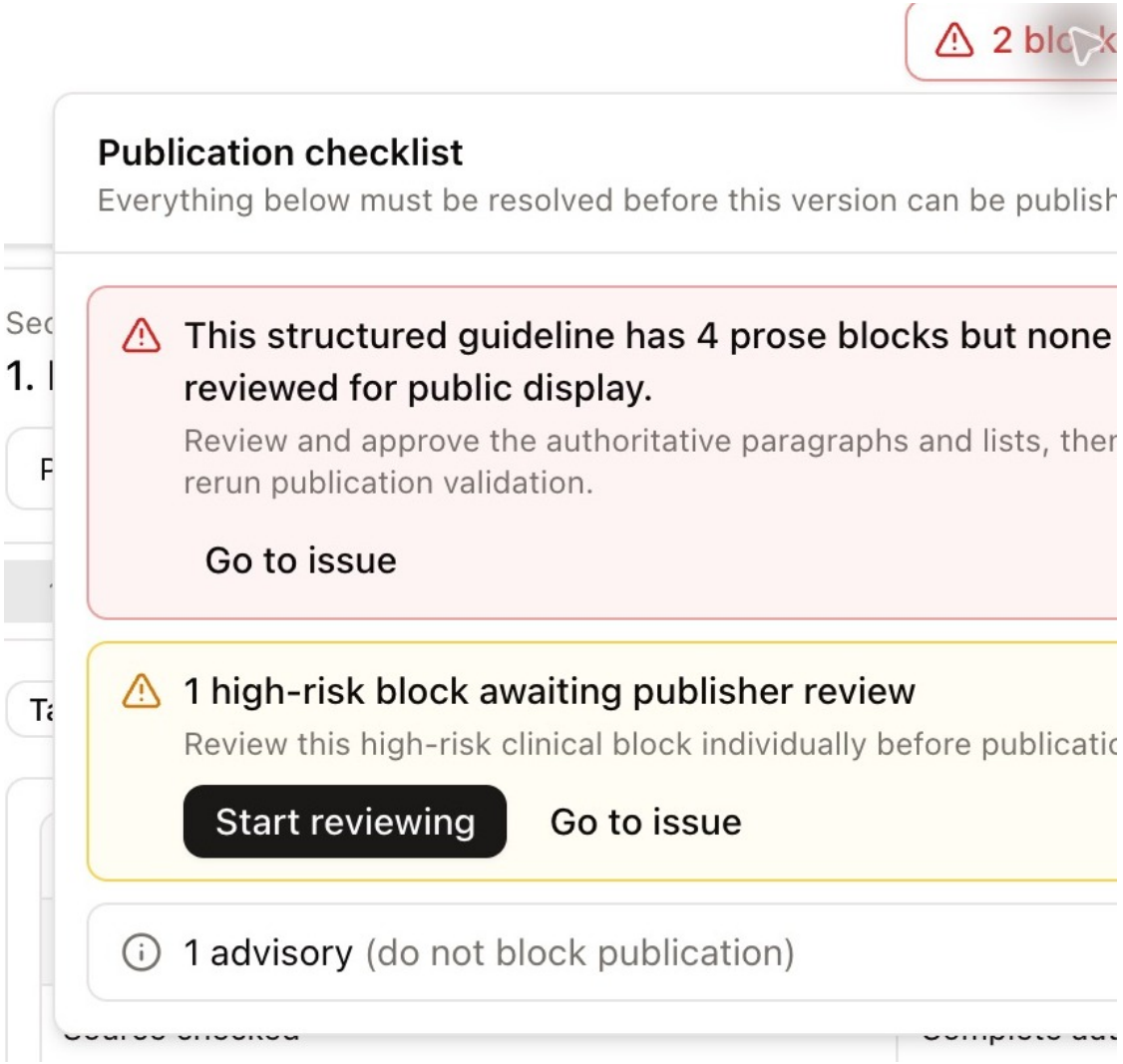
Publication blockers grouped by issue

Header > blockers > Publication checklist



This snapshot shows missing reviewed prose and one pending table. Use Go to issue or Start reviewing; do not remove content just to clear a blocker.

Publication blockers grouped by issue — close-up



This snapshot shows missing reviewed prose and one pending table. Use Go to issue or Start reviewing; do not remove content just to clear a blocker.

16 Review ordinary content explicitly

Approving tables alone can leave published chapters without their explanatory prose.

1. Inspect the full content

Use All blocks to check paragraphs, headings, lists and references against the source. Verify that content is complete and in order, including front matter and appendices.

2. Use the controlled low-risk queue

Open Bulk approve low-risk. Select only the eligible blocks you actually verified. The backend allowlist includes paragraphs, headings, ordered/unordered lists, references and page breaks.

3. Attest deliberately

Review the selected content and counts. The operation allows at most 500 eligible blocks per submission. Confirm the attestation that every selected block was checked against the authoritative source, then Confirm approval.

4. Verify the result

Confirm the selected blocks are reviewed. Repeat in sensible batches, then inspect the completeness report for reviewed prose and populated clinical leaf sections.

Continue when
Reviewed ordinary content accompanies reviewed clinical tables and callouts.

Watch point: Figures, high-risk blocks, unknown types and types outside the allowlist cannot use this bulk approval. Selection is not permission to skip reading.

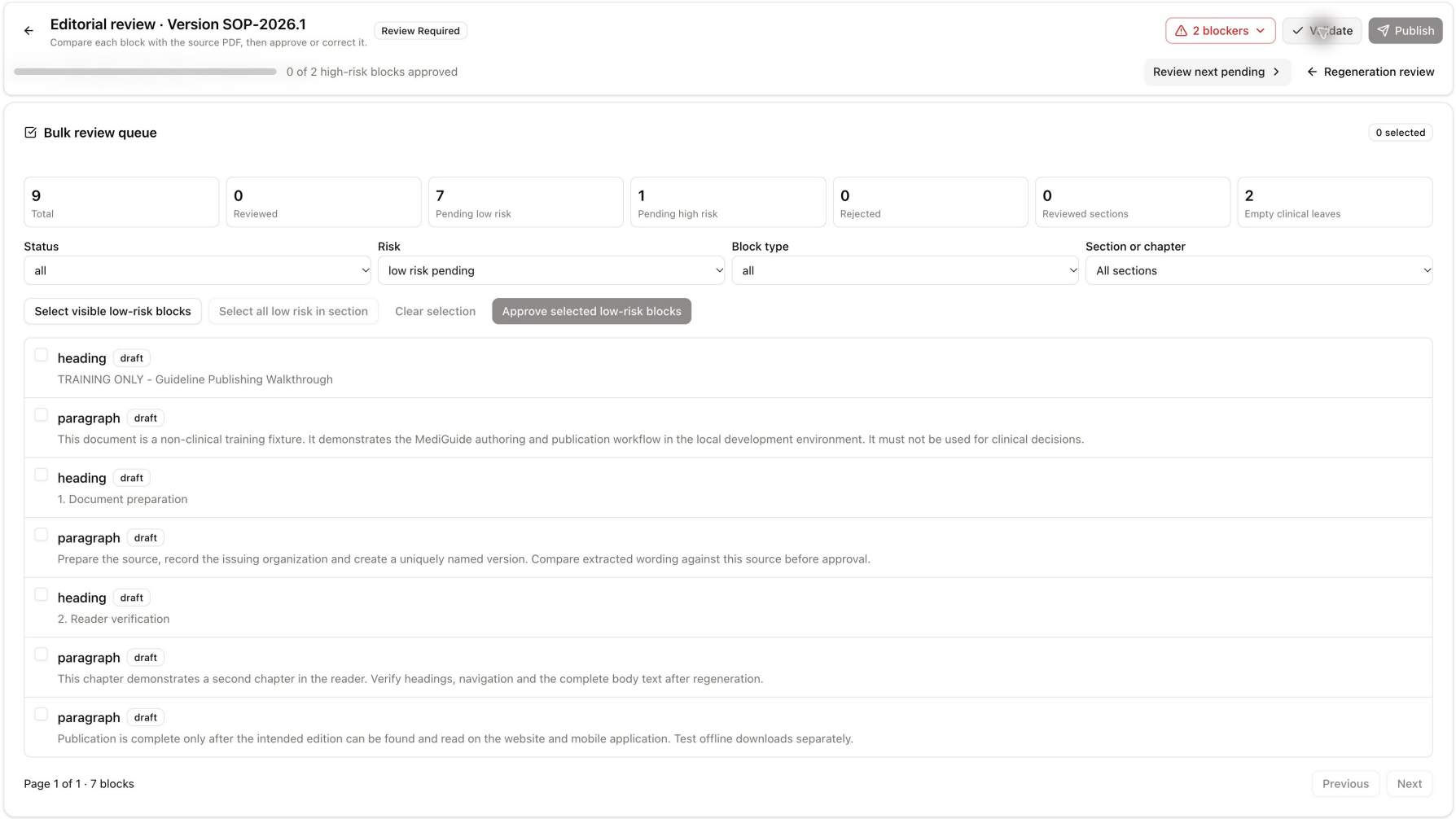
Screen plates follow: Controlled low-risk queue; Approval requires a source attestation

Reproduce this batch

Set Risk to low risk pending and inspect the other filters. Read the eligible content against the source. Select visible low-risk blocks only for that verified set. Inspect the selected count before Approve selected low-risk blocks. In the dialog check the type totals, attest source fidelity, and Confirm approval. Verify Reviewed increases and Pending low risk decreases.

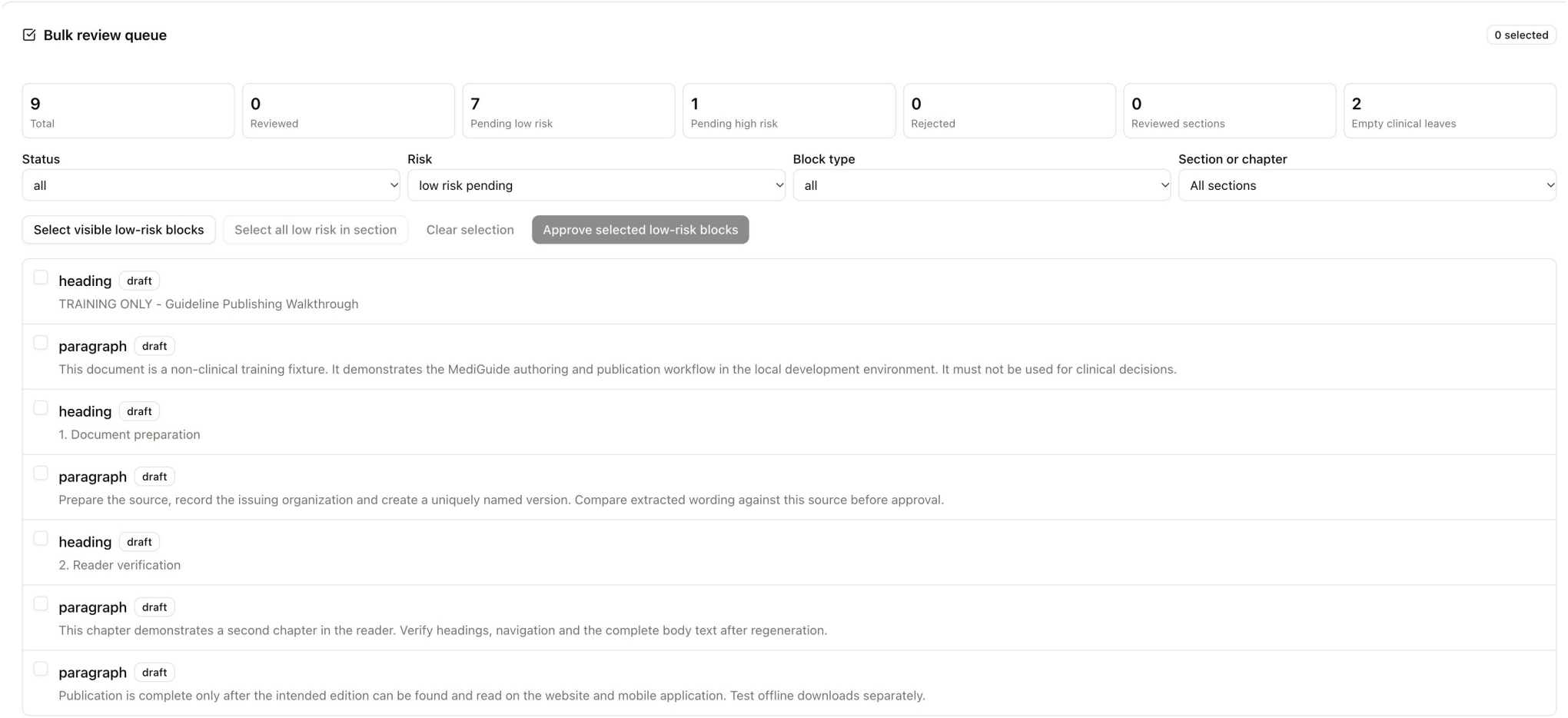
Controlled low-risk queue

Editorial Review > Bulk approve low-risk



Seven eligible blocks are pending: three headings and four paragraphs. Filters and selection controls allow a deliberate, source-checked batch.

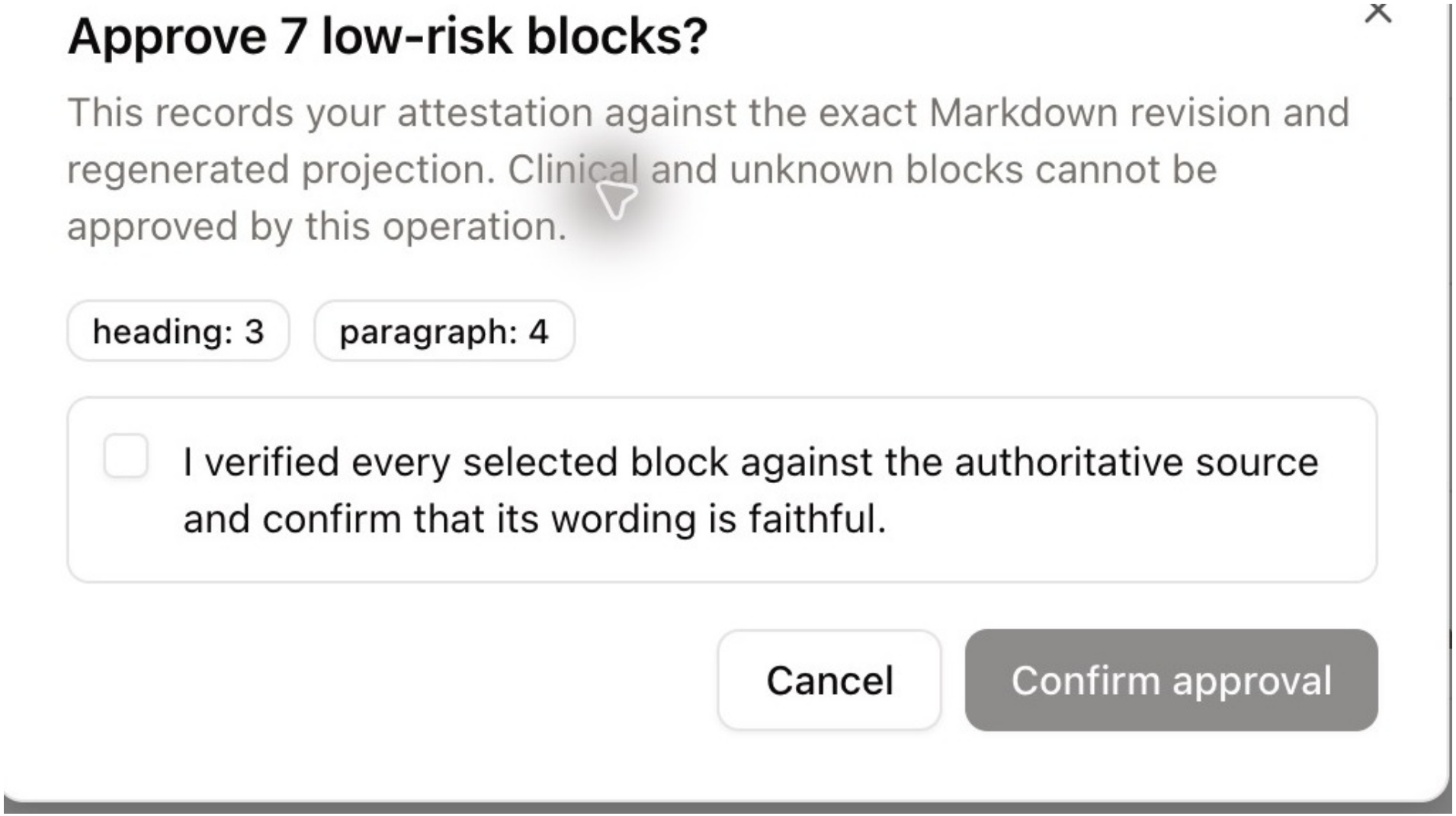
Controlled low-risk queue — close-up



Seven eligible blocks are pending: three headings and four paragraphs. Filters and selection controls allow a deliberate, source-checked batch.

Approval requires a source attestation

Approve selected low-risk blocks



The dialog states the selected type counts and exact-revision attestation. Check the box only after verifying every selected block.

17 Approve high-risk content individually

Each safety-sensitive decision must be deliberate, attributable and checked against the authoritative source.

1. Compare the complete clinical statement

Individually review tables, recommendations, warnings, cautions, contraindications, dosages, procedures, algorithms, algorithm references and referral criteria. Use the server policy and checklist for the actual required set.

2. Review figures and conservative types

Compare clinically sensitive assets and figure blocks, including axes, legends and labels. Key points, evidence, definitions, clinical notes and unknown blocks are not automatically bulk-eligible; check the available authorized review action.

3. Approve or reject

Select Approve & next (or Approve figure & next) only when faithful and correct. Reject when it must not publish; a rejected block remains outstanding until corrected and approved.

4. Work efficiently without bypassing review

Outside text fields/dialogs, J moves next, K previous, A approves and advances, R rejects. Confirm the reviewed badge and decision trail. Save or discard an unsaved correction before deciding.

Continue when
Every required high-risk block and relevant asset has an authorized approval.

Watch point: Do not bulk-approve clinical tables or accept the entire regeneration as a shortcut around individual review.

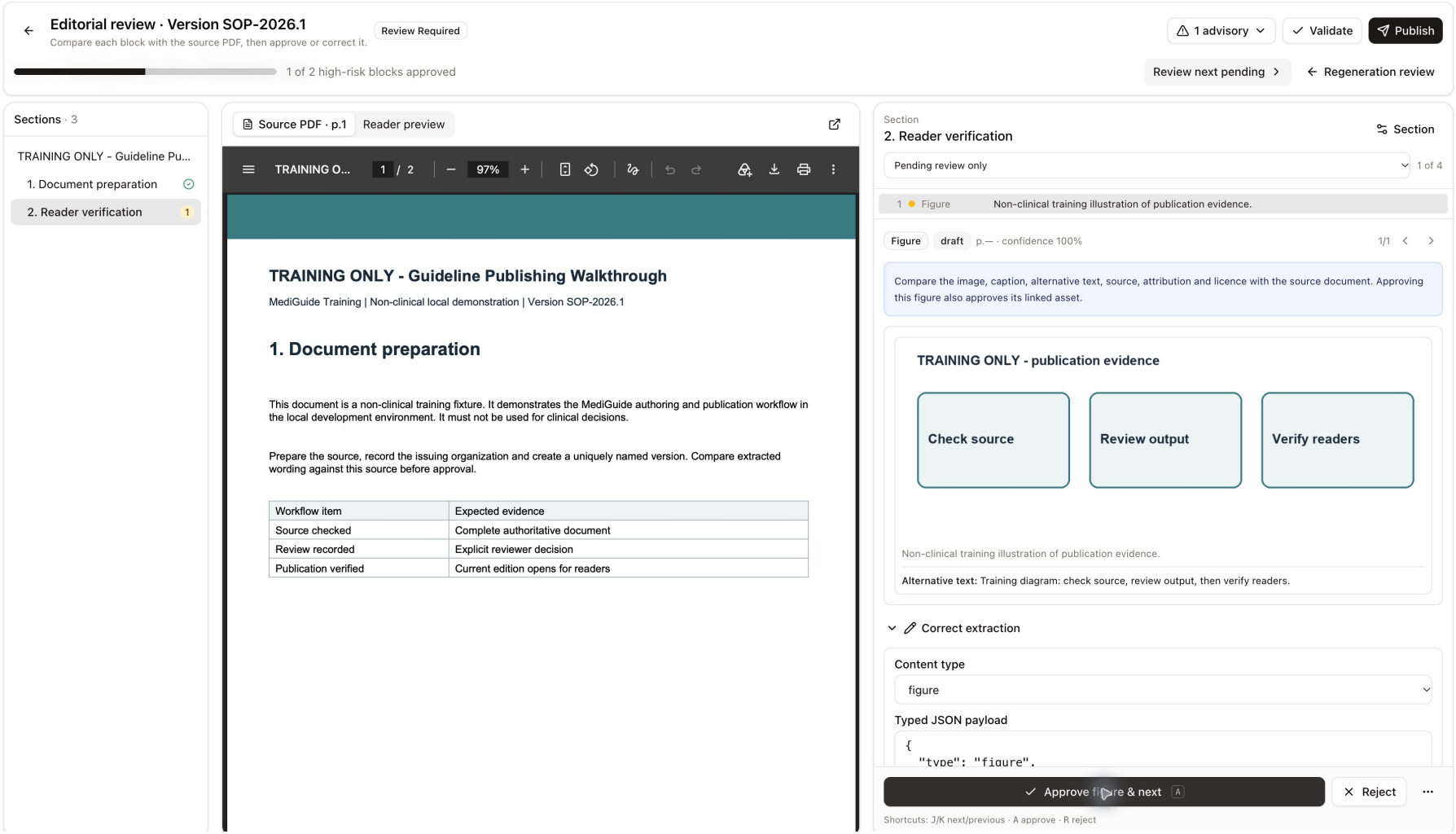
Screen plates follow: Individual figure review

Worked individual decisions

The training table has two columns and three data rows, all compared with PDF page 1. The training figure is a separate non-clinical editorial illustration added by the author; verify its original image and metadata, not a fictitious PDF page. The example’s figure has no PDF page provenance, shown as p.—.

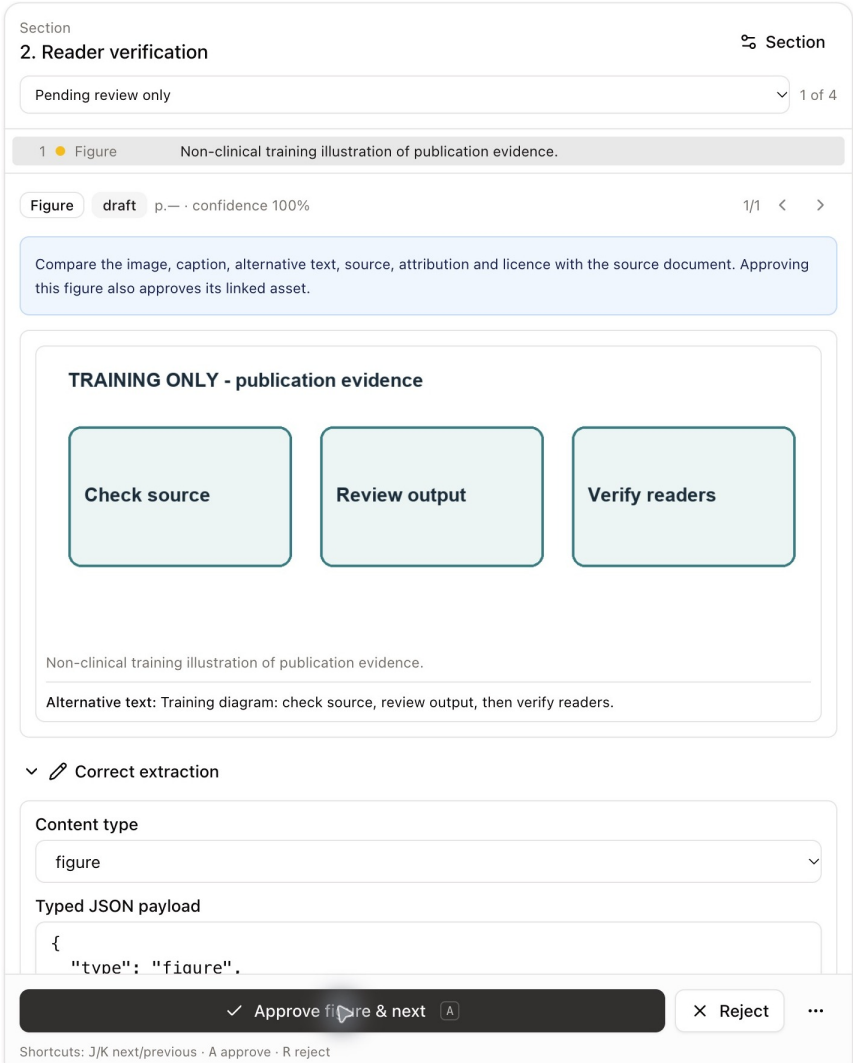
Individual figure review

Editorial Review > Review



Approving a figure also approves its linked asset. Compare the exact image, caption, alternative text and any source/license information.

Individual figure review — close-up



Approving a figure also approves its linked asset. Compare the exact image, caption, alternative text and any source/license information.

18 Correct extraction and section defects

Choose the correction method intentionally. Source fixes and structured corrections have different consequences.

1. Prefer a durable source correction

For missing wording, table cells, figure references or hierarchy problems, return to Edit Markdown. Correct against the source, save, regenerate and re-review the changed projection.

2. Use a structured correction when appropriate

In Editorial Review, expand Correct extraction. Authorized users can edit the typed content type/payload and Save correction. This is an advanced content operation; request help rather than guessing JSON fields.

3. Repair section navigation carefully

Use Section settings for title, level, slug, move or merge actions. Start new section here is available in the block menu. Check ordering, links and hierarchy afterward; deletion must not remove meaningful clinical content.

4. Re-review the result

A saved block correction resets its decision to draft. Approve the corrected block only after rechecking it. Structured-only changes may be replaced by a later regeneration from Markdown, so reconcile the source for future editions.

Continue when
The corrected source/projection remains faithful, structurally valid and reviewed.

Watch point: Never invent missing clinical values, source pages or attribution. Escalate missing source evidence.

19 Check the completeness report

Editorial Review > Completeness report. This is a read-only readiness and coverage report, not an approval button.

1. Inspect coverage

Check total/reviewed sections and clinical leaf sections, empty leaves, block counts by type/state, reviewed percentage and reviewed paragraph coverage. Look for chapters with headings but no reviewed body.

2. Compare with the current publication

Investigate unexpected reductions in chapters, sections, blocks, tables or high-risk blocks. A mostly empty candidate must not replace a complete published guideline just because some tables passed review.

3. Check supporting evidence

Inspect original-PDF or offline-fallback availability, source revision/job identity, regeneration acceptance, validation issues and RAG chunks/embeddings. Confirm that expected images and source context are represented.

4. Export and correct

Export JSON or CSV for audit when needed. Resolve missing content at the source or review the outstanding ordinary/high-risk blocks, then refresh the report. Exporting never approves or publishes anything.

Continue when
Coverage is understood, no unexplained content loss remains, and the report refers to the intended revision/job.

Watch point: A high reviewed percentage alone is insufficient if a missing chapter or one critical table is concealed by the total.

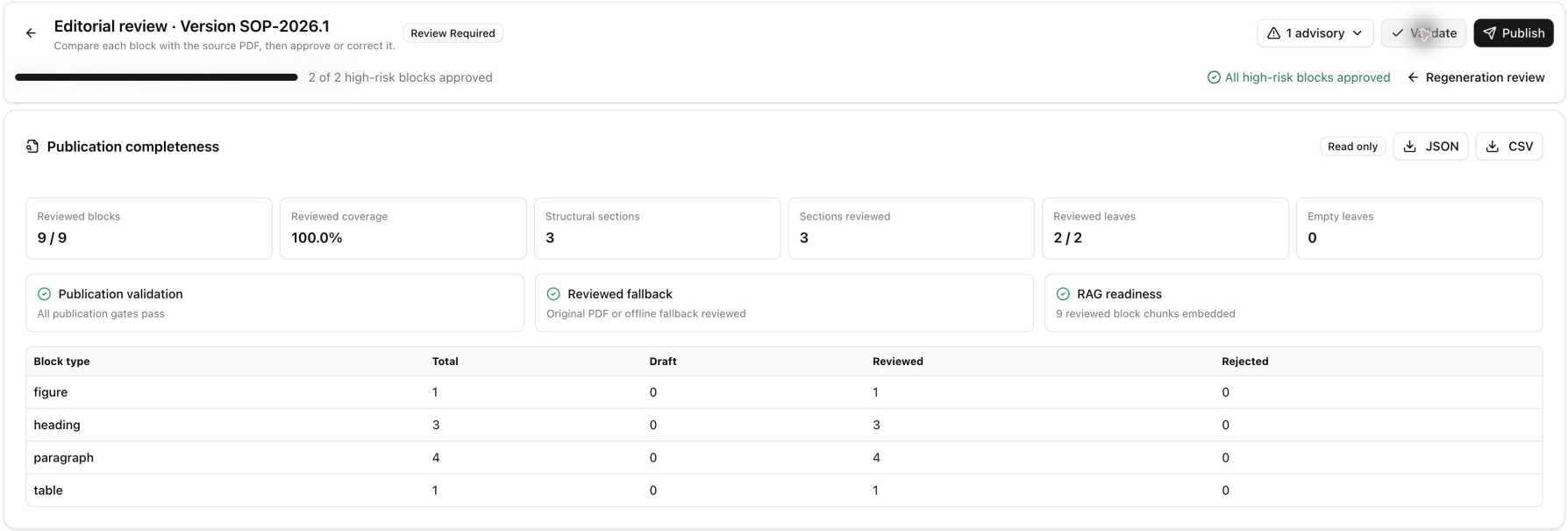
Screen plates follow: Completeness after review

Read the actual evidence

In this walkthrough, the report shows heading 3 reviewed, paragraph 4 reviewed, table 1 reviewed and figure 1 reviewed. Those counts fit this tiny two-chapter fixture only. For a real guideline reconcile the report with every source chapter and appendix.

Completeness after review

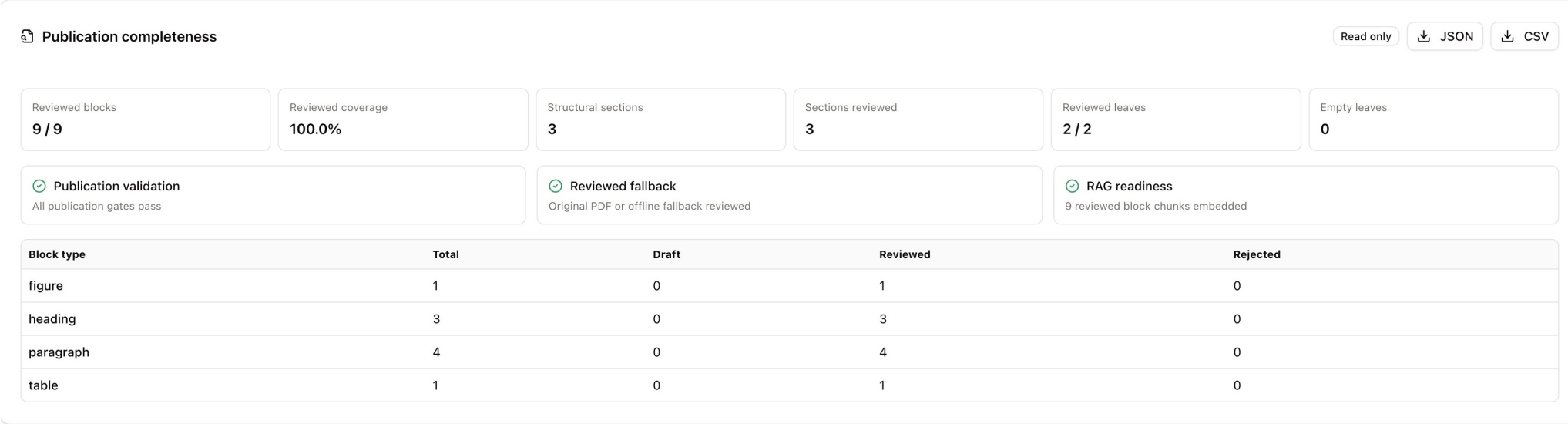
Editorial Review > Completeness report



Block type	Total	Draft	Reviewed	Rejected
figure	1	0	1	0
heading	3	0	3	0
paragraph	4	0	4	0
table	1	0	1	0

The training example has 9/9 reviewed blocks, 100% coverage, 2/2 reviewed leaves, zero empty leaves, and all publication gates passing.

Completeness after review — close-up



Block type	Total	Draft	Reviewed	Rejected
figure	1	0	1	0
heading	3	0	3	0
paragraph	4	0	4	0
table	1	0	1	0

The training example has 9/9 reviewed blocks, 100% coverage, 2/2 reviewed leaves, zero empty leaves, and all publication gates passing.

20 Accept the regenerated projection

Return to the Markdown editor after individual and ordinary-content review is complete.

1. Open the regeneration comparison

Use Regeneration review when returning from Editorial Review, or open Edit Markdown for the draft. Inspect hierarchy, block-type counts, tables, chunks, assets, provenance and original-PDF availability.

2. Refresh the approval status

Select Refresh approval status. If outstanding high-risk blocks remain, choose Review pending blocks, compare and approve the corrected content, then return and refresh again.

3. Accept only the faithful result

When required review is complete and the comparison is satisfactory, select Accept regenerated projection. If it is not faithful, provide a rejection comment and choose Reject and return to Markdown.

4. Protect the accepted identity

Acceptance applies to the exact generated revision/job. If the source is saved again or regenerated, the new result needs a new review and acceptance cycle. Do not publish an accepted older projection against newer source.

Continue when
Regeneration review reads accepted for the current saved and generated revision.

Watch point: The acceptance button is refused while required high-risk blocks remain draft or rejected. No force-accept path is part of this SOP.

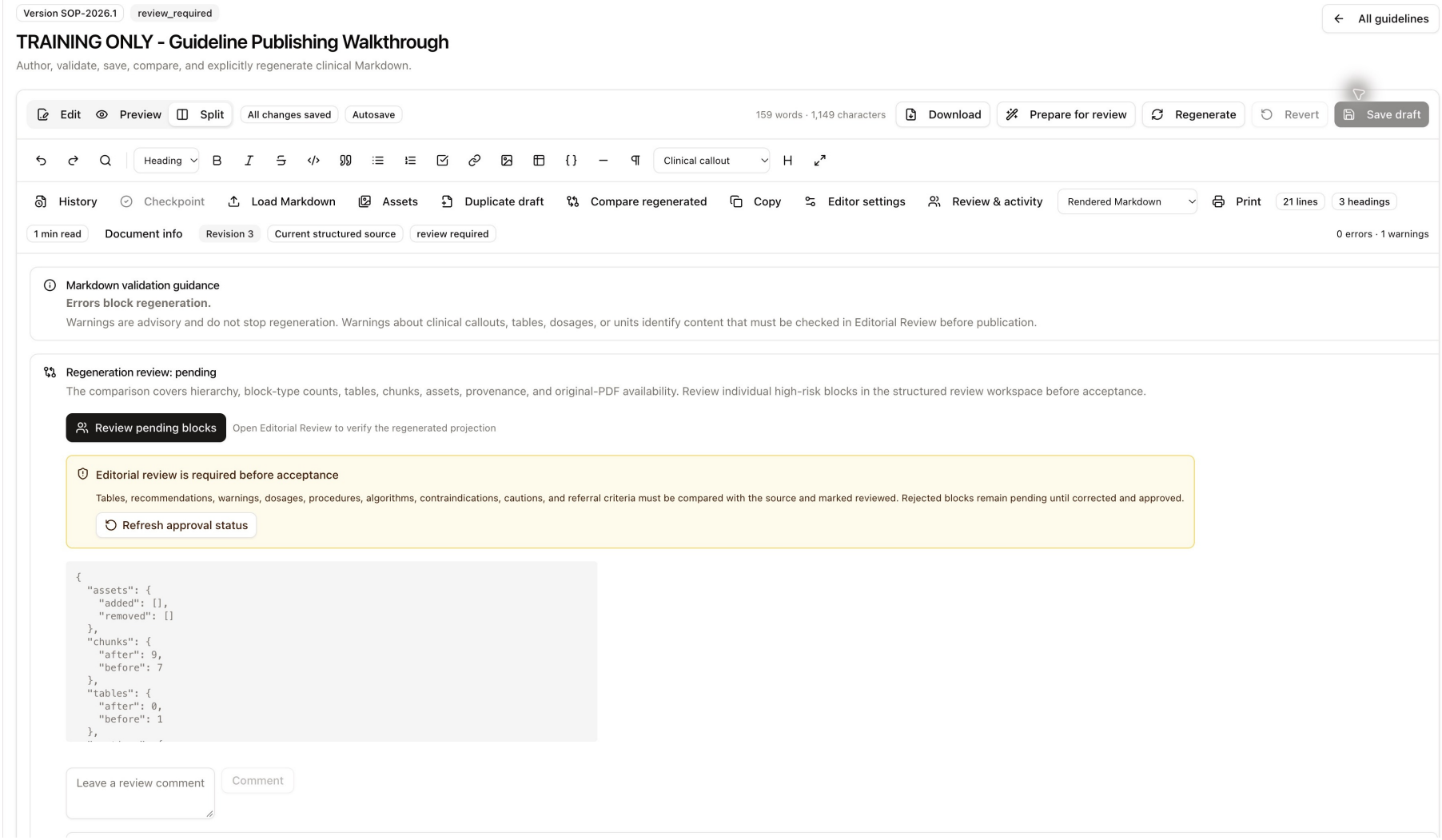
Screen plates follow: Acceptance becomes available; Regeneration accepted

Accepted does not mean published

The screenshot says Regeneration review: accepted and Current structured source approved. The version can still show review_required until publication. Use the authoritative checklist and published-version state rather than interpreting one badge in isolation.

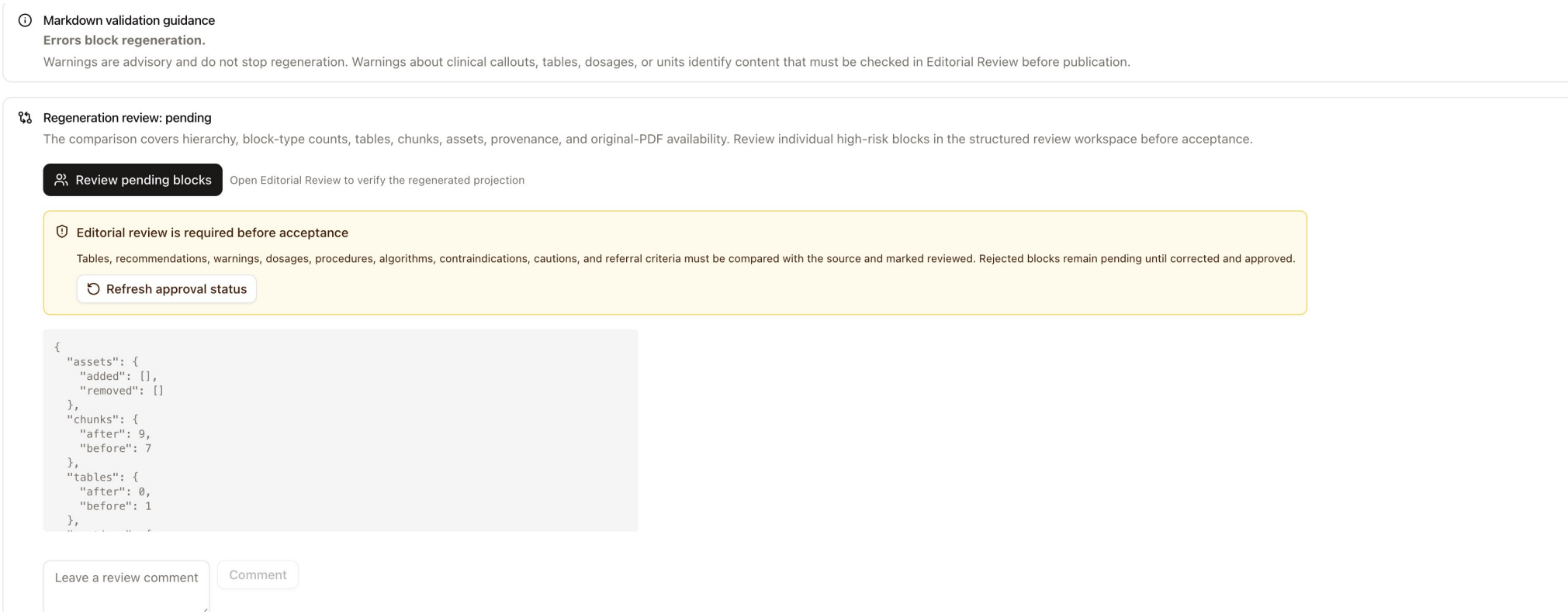
Acceptance becomes available

Edit Markdown > Regeneration review



After returning from review, Accept regenerated projection is enabled. Confirm that the comparison still describes the intended saved revision.

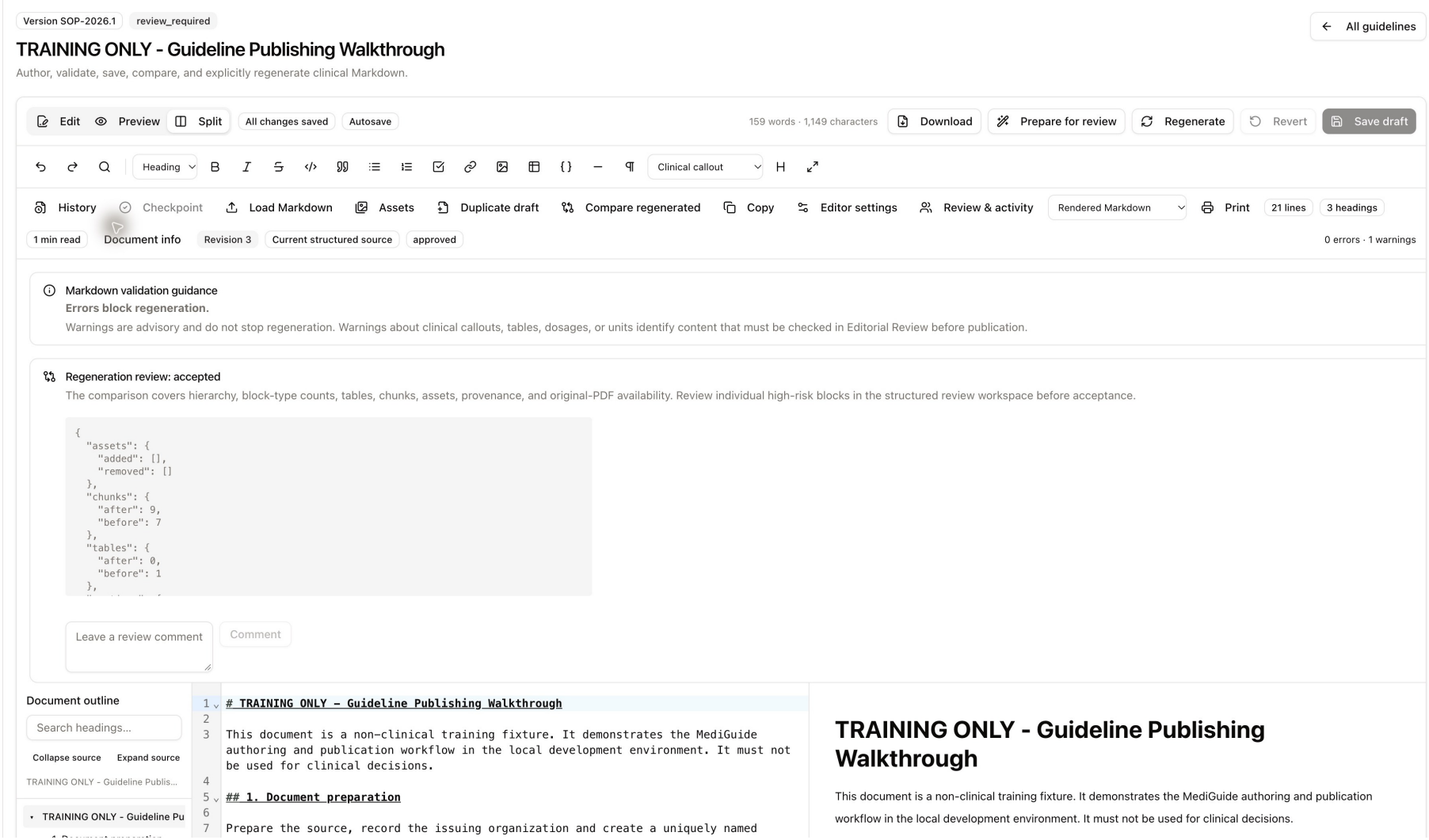
Acceptance becomes available — close-up



After returning from review, Accept regenerated projection is enabled. Confirm that the comparison still describes the intended saved revision.

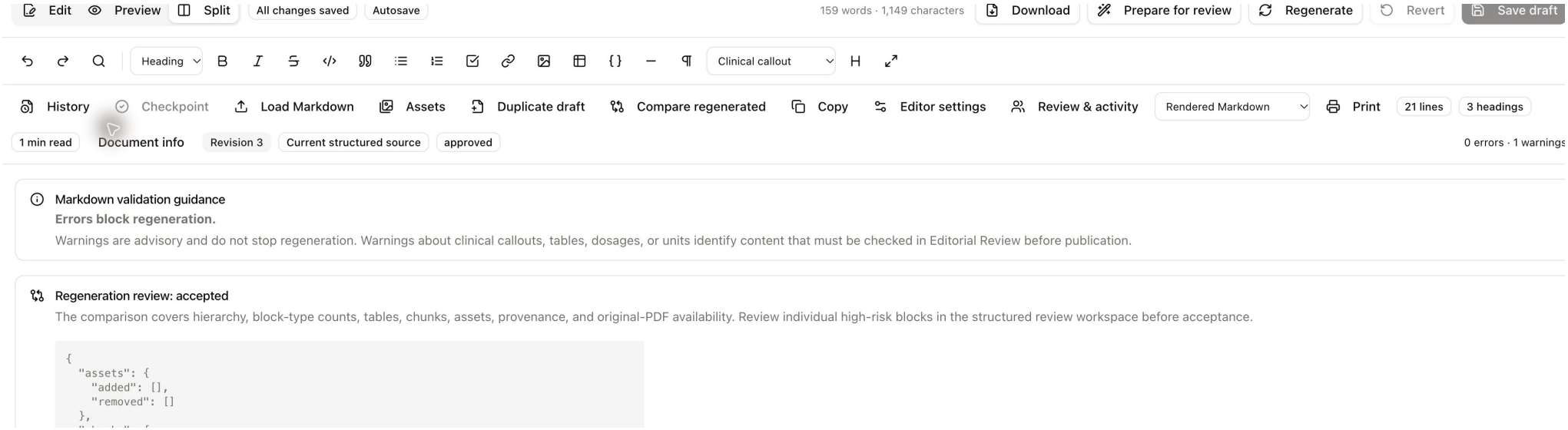
Regeneration accepted

Edit Markdown > Regeneration review



The visible state reads accepted and Current structured source approved. This is still a private draft until a publisher publishes it.

Regeneration accepted — close-up



The visible state reads accepted and Current structured source approved. This is still a private draft until a publisher publishes it.

21 Validate and publish the version

The publisher performs the final gate. Publication makes the approved version current for reader-facing services.

1. Run authoritative validation

In Editorial Review select Validate and open Publication checklist. Resolve real defects and the review queue. The document page's Publish action also validates before attempting publication.

2. Check every gate

Confirm source/generated/accepted revision identity, valid hierarchy and payloads, resolved/reviewed assets, approved required blocks, meaningful reviewed prose, adequate coverage, accepted regeneration and required source/offline fallbacks.

3. Publish deliberately

Select Publish only after no blockers remain. Editorial Review asks for confirmation that publication is permanent and audited. Confirm only for the intended document/version. The backend rechecks eligibility.

4. Confirm success and hand over

Check the success message and current published version on the document. The previous approved edition remains in history. Continue with website and mobile checks; guideline publication does not require a new application or website software release.

Continue when
The intended edition is the current published version, not merely a saved draft or accepted regeneration.

Watch point: If Publication needs review / Publication blocked appears, return to the named issue. Do not create a software release to bypass a content-publication blocker.

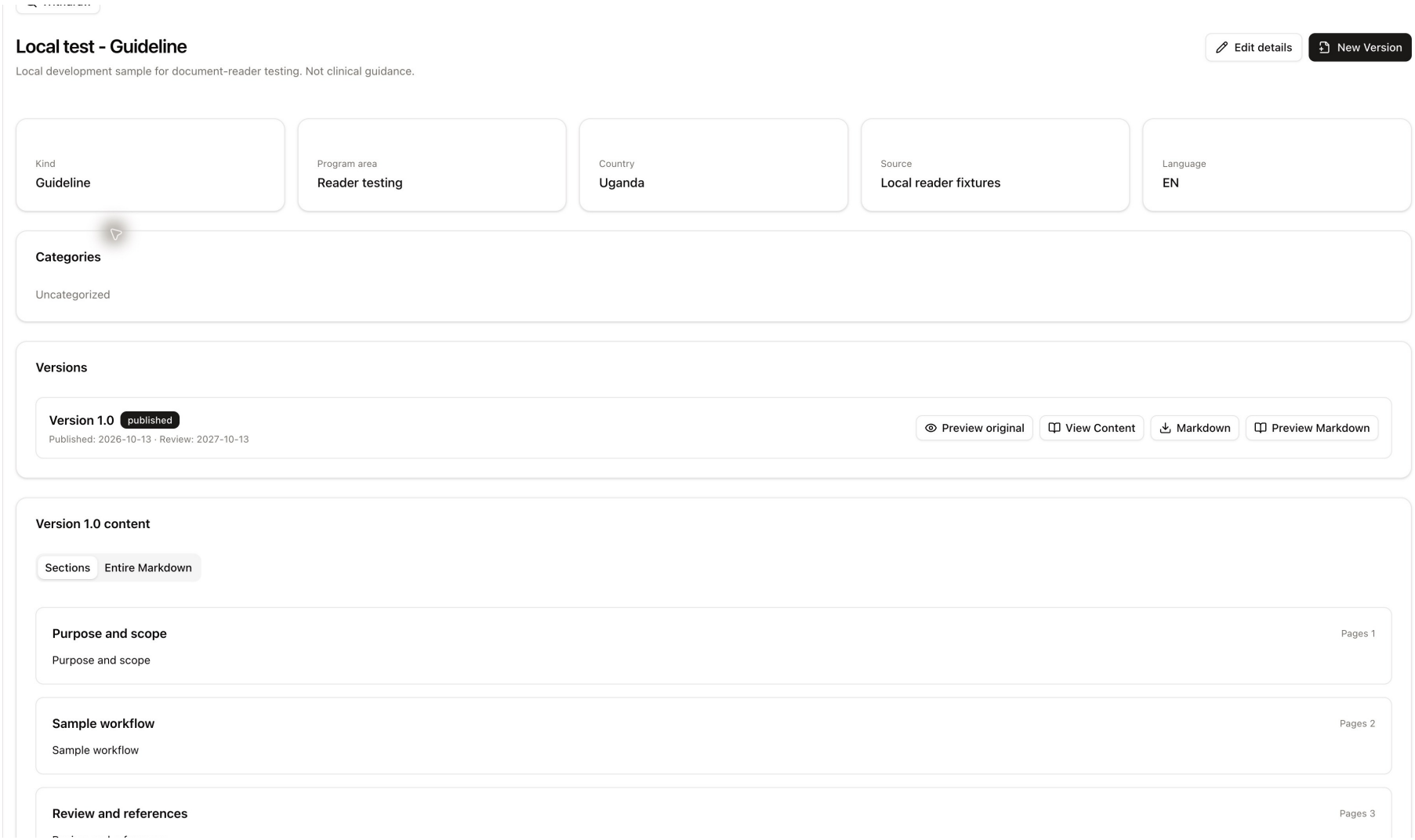
Screen plates follow: A genuinely published version

Exact confirmation wording

Editorial Review asks: “Publish this reviewed version? The action is permanent and audited.” The authorized publisher checks the document and version, then confirms. The training walkthrough was stopped at this confirmation; the published-state screenshot is an already published local fixture. No production publication was performed for this SOP.

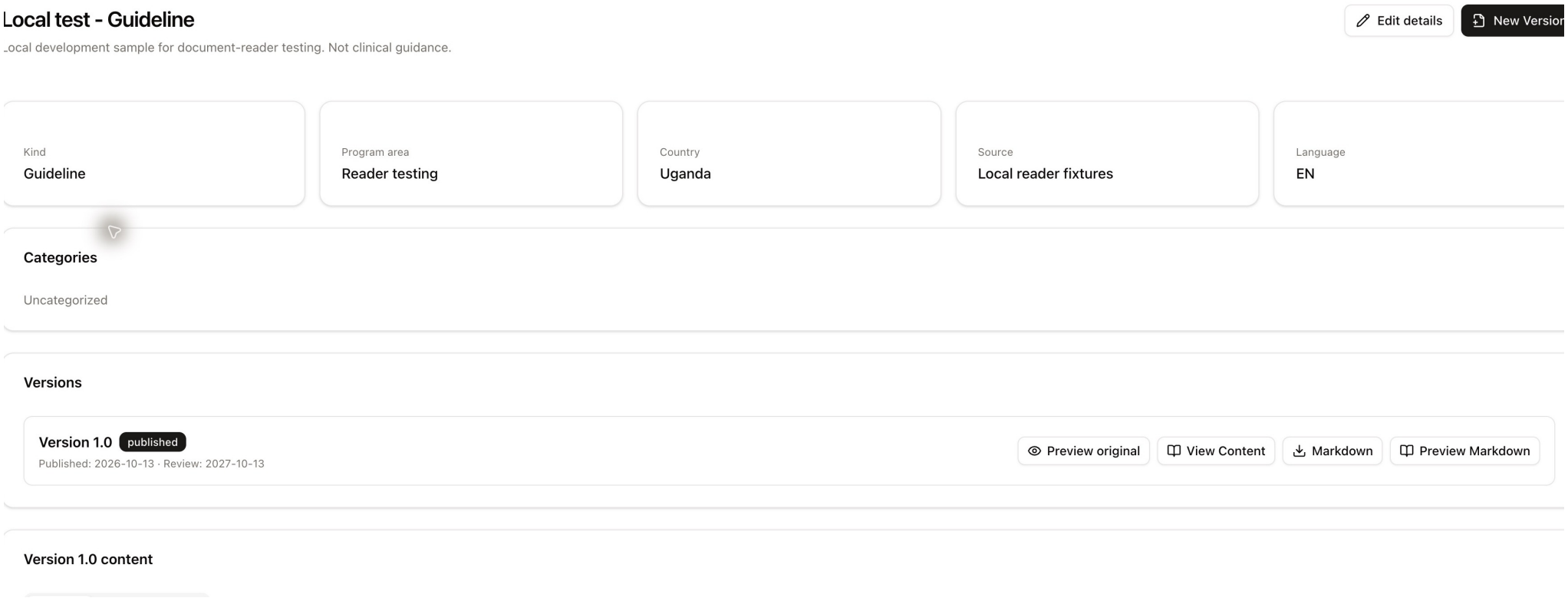
A genuinely published version

Document detail > Versions



This is an existing local reader fixture, Local test - Guideline, version 1.0 published. It is a separate record from the training walkthrough draft.

A genuinely published version — close-up



This is an existing local reader fixture, Local test - Guideline, version 1.0 published. It is a separate record from the training walkthrough draft.

22 Verify the public website

Use a public or signed-out view as well as the private dashboard preview.

1. Find the published document

Open the public guidelines website and search for the official title. Check document kind, classification and visible version metadata. Confirm that a replacement shows the intended current edition.

2. Check discovery and navigation

Open applicable category/disease listings and any configured hub placement. Test chapter navigation, nested sections, search terms, internal links and back navigation on desktop and narrow widths.

3. Compare representative content

Check the first, middle and last chapters; review at least one table, clinical callout, figure and footnote where present. Confirm no empty clinical chapter has replaced complete reviewed content.

4. Check downloads and citations

Open the original/download fallback where provided. Verify representative published search results and RAG citations resolve to the right document, current version and source context. Record missing results separately from reader defects.

Continue when
A reader without dashboard access can find and read the expected approved version.

Watch point: A private preview is not a public acceptance test. Publication and client caches/indexing may not become visible at exactly the same instant.

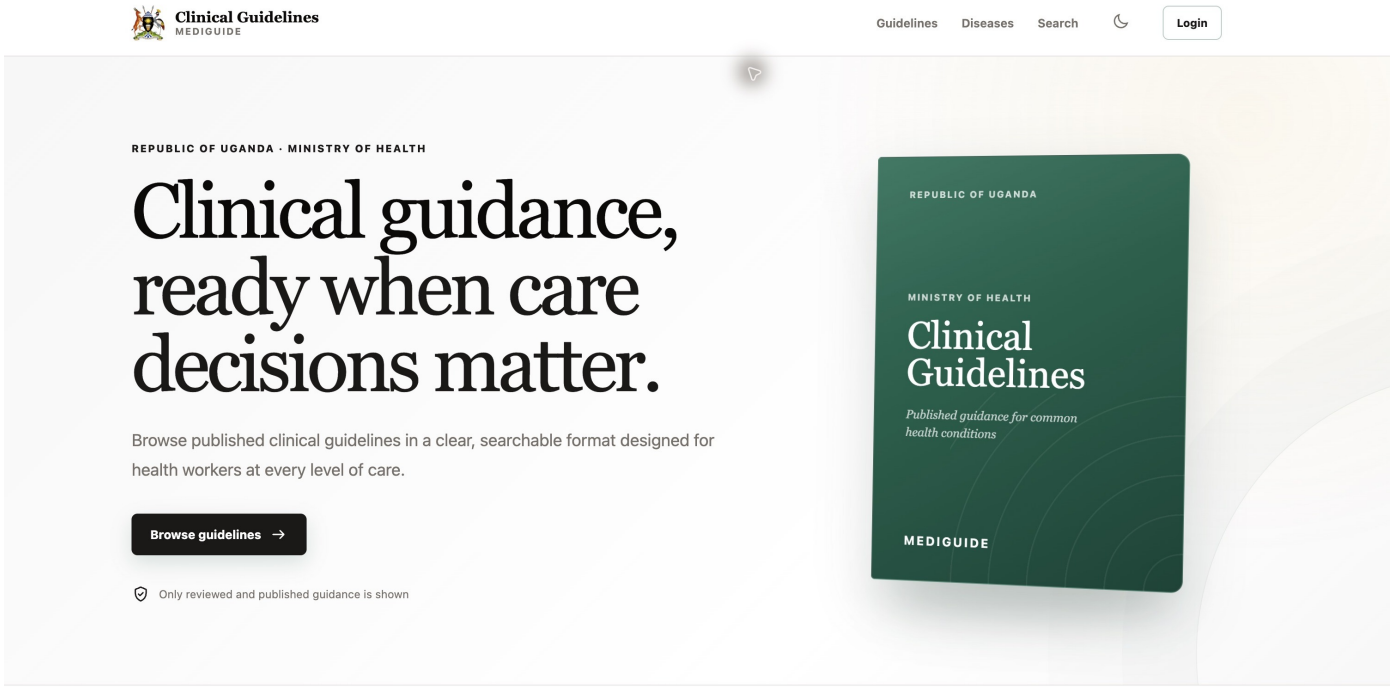
Screen plates follow: Public guidelines website; Published structured reader; Reader navigation reaches the table

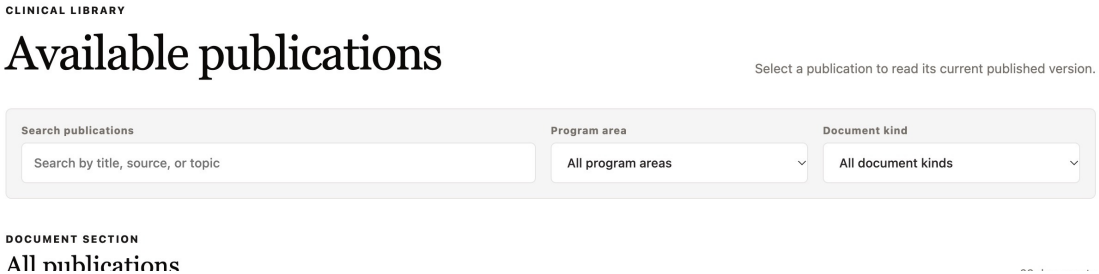
Exact website controls

Use Search publications, Program area and Document kind in the public library. Choose Read guideline on the intended card. In the reader use Search sections, chapter buttons, Change text size and source-page controls. Verify the edition displayed beside the source organization.

Public guidelines website

Public website > Guidelines

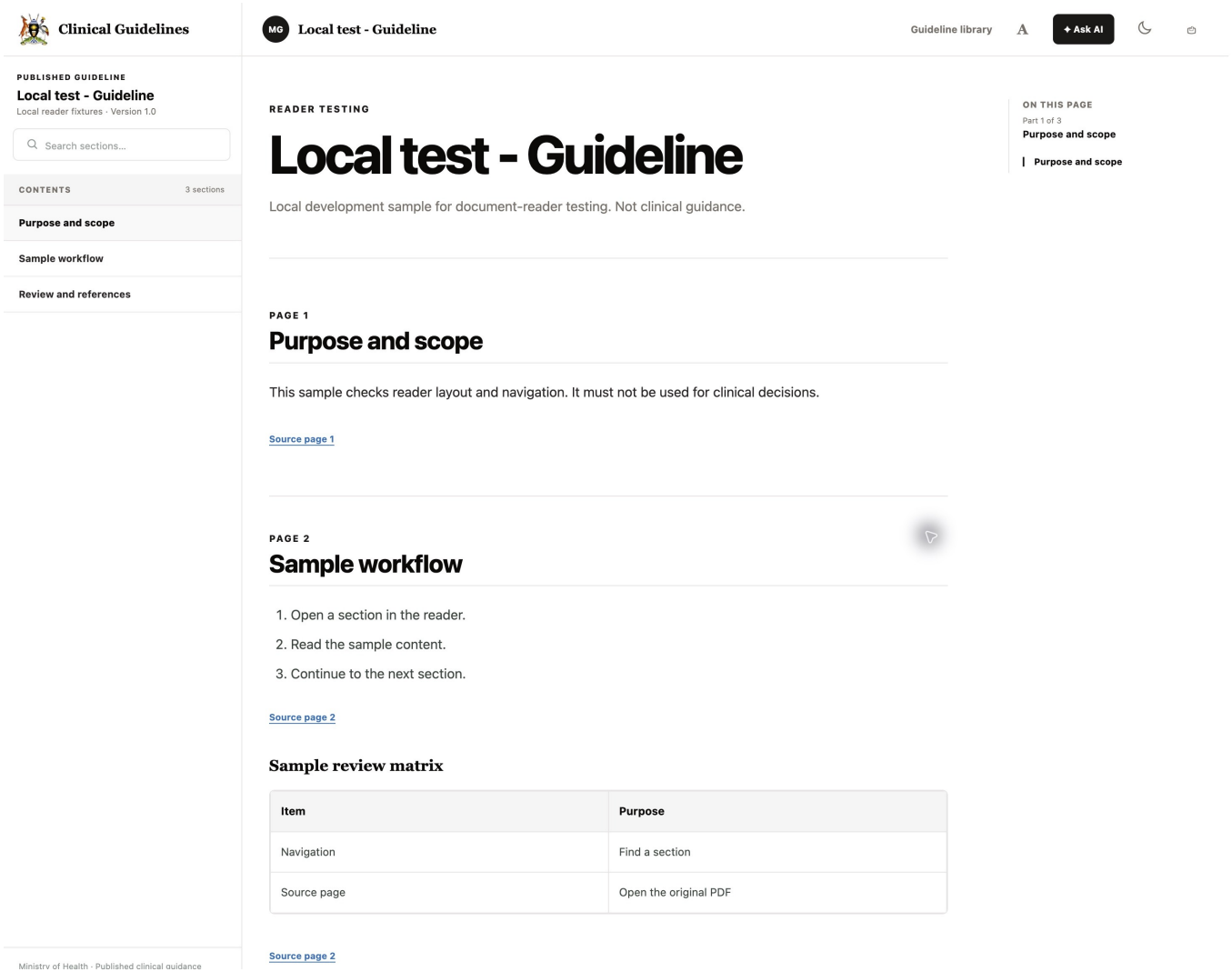




The library exposes published documents to readers. Search by official title and inspect the version rather than relying on a private dashboard preview.

Published structured reader

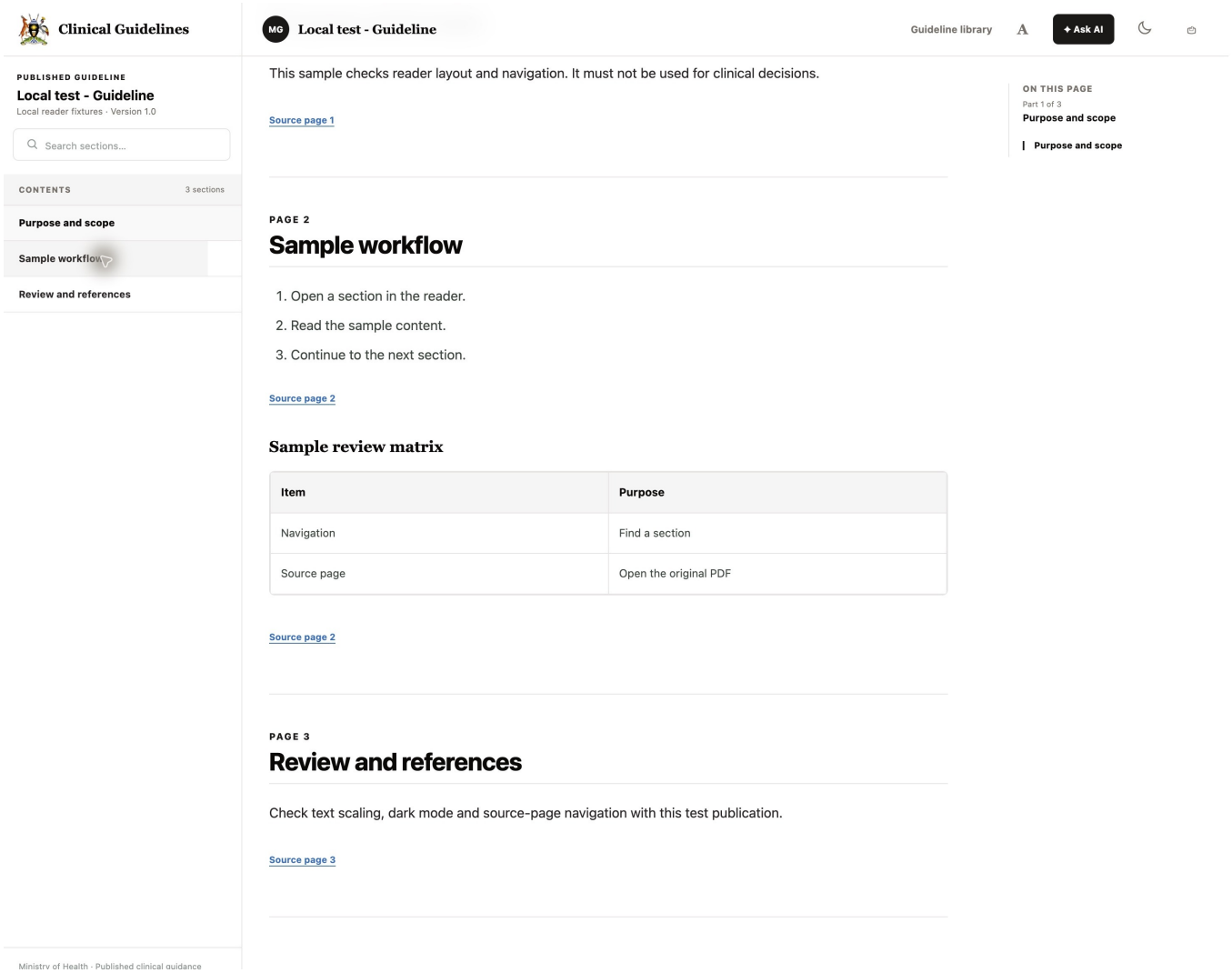
Public library > Read guideline



This existing published fixture exposes chapter navigation, source-page actions and a structured table. It is not the newly reviewed training draft.

Reader navigation reaches the table

Public reader > Sample workflow



The chapter action changes the URL anchor and scroll position. Verify ordered lists, table headers and cells, then the final chapter.

23 Verify the app and offline package

Use a supported mobile build with connectivity first, then test the downloaded content offline.

1. Refresh online discovery

Open the app online, refresh/synchronize available content and search for the title. Check relevant browse categories/diseases. If the new edition is absent, verify website/API publication before diagnosing the device.

2. Open the structured reader

Confirm the edition and chapter list. Test nested sections, readable tables, figures, links, font scaling and narrow-screen layout. Compare representative safety-sensitive content with the source and public website.

3. Download and reopen offline

Download or refresh the guideline package using the app's available download controls. After completion, disable connectivity and open the saved document. Test chapters, tables and images. Reconnect when the test is finished.

4. Confirm update behavior

Existing downloads may need a refresh after a new edition publishes. Verify that the refreshed package represents the intended current version. Record device, app build, edition and synchronization time in the handover record.

Continue when
The approved edition is discoverable online and its downloaded package works offline on a representative device.

Watch point: An old offline copy is not evidence that server publication failed. Offline or disconnected devices cannot receive immediate updates or withdrawal information.

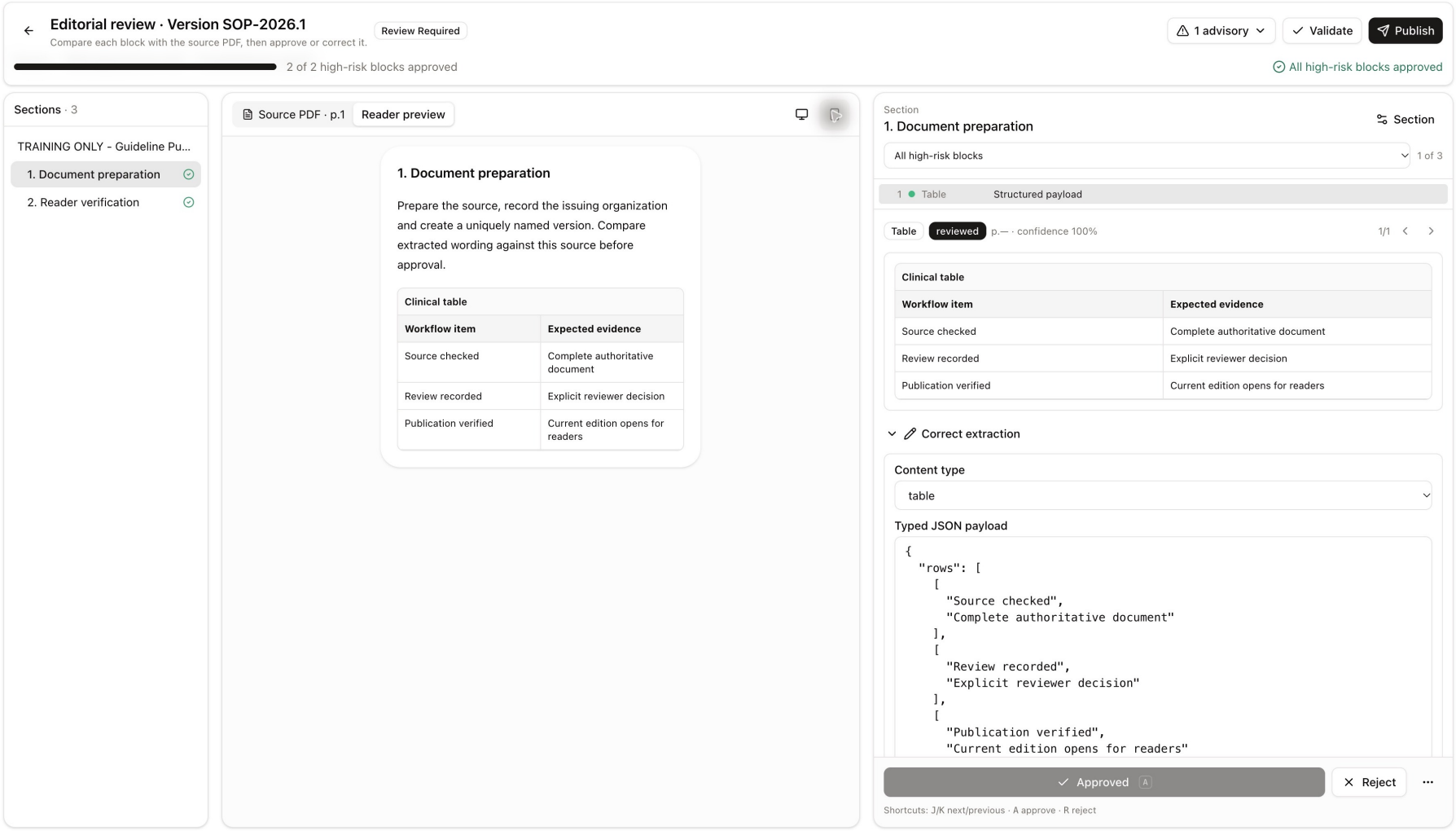
Screen plates follow: Mobile width table preview; Mobile width figure preview

Evidence boundary

The dashboard screenshots are exact mobile-width previews of the training draft. The additional Android screenshots are existing repository/store screenshots of another guideline. This SOP provides the on-device acceptance steps; it does not claim that the training draft was published or verified offline on a device.

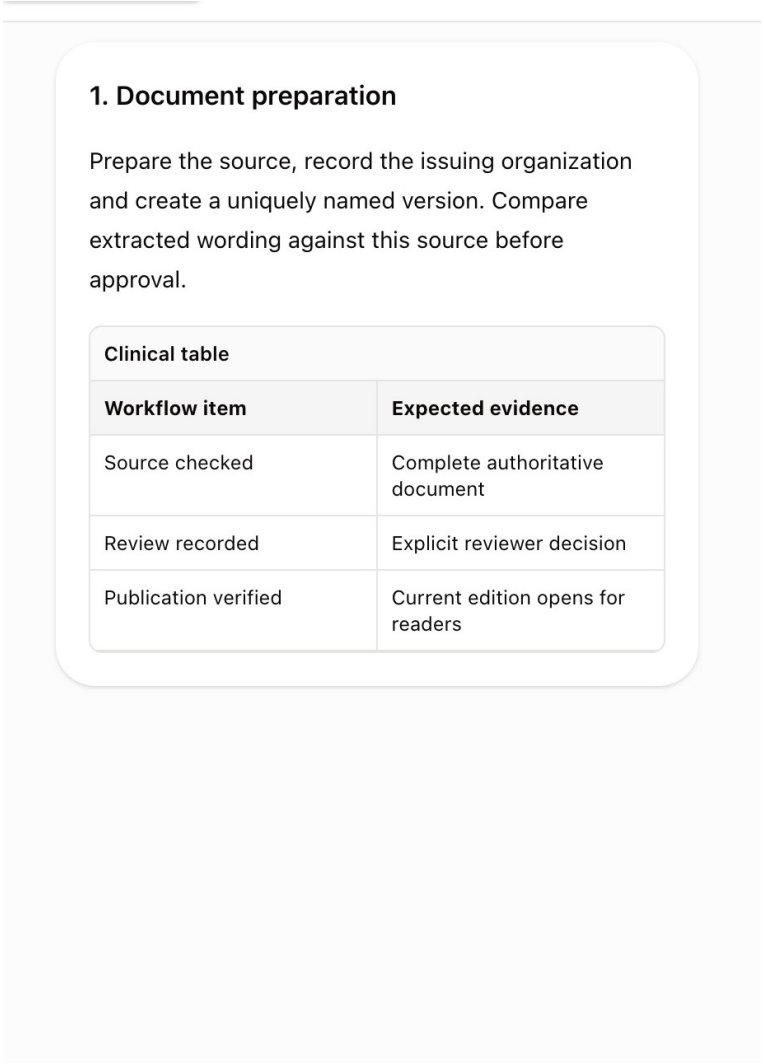
Mobile width table preview

Editorial Review > Reader preview > Mobile preview



This is the real dashboard preview at mobile width. It is useful for layout inspection; it is not an on-device app acceptance test.

Mobile width table preview — close-up



1. Document preparation

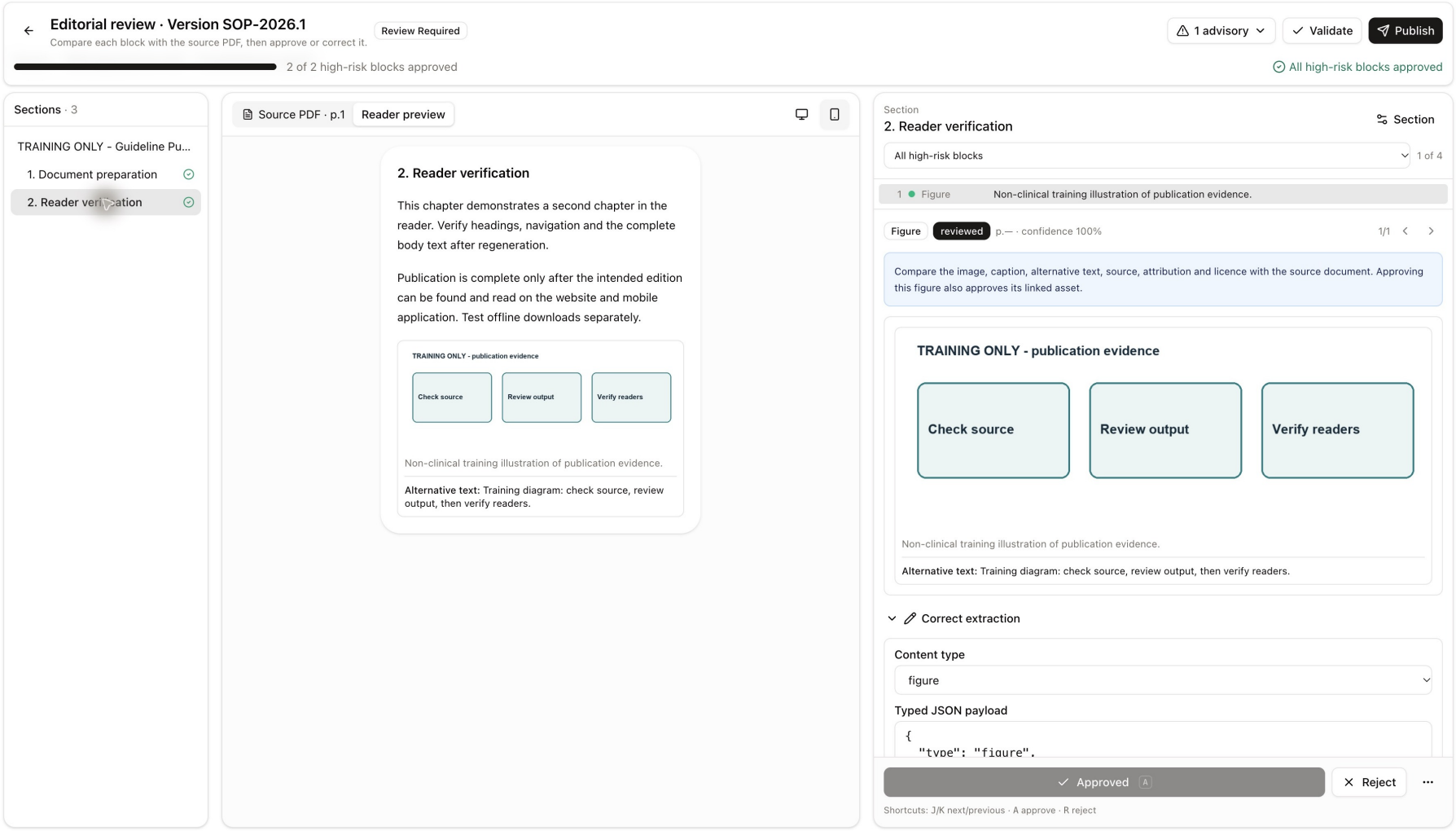
Prepare the source, record the issuing organization and create a uniquely named version. Compare extracted wording against this source before approval.

Clinical table	
Workflow item	**Expected evidence**
Source checked	Complete authoritative document
Review recorded	Explicit reviewer decision
Publication verified	Current edition opens for readers

This is the real dashboard preview at mobile width. It is useful for layout inspection; it is not an on-device app acceptance test.

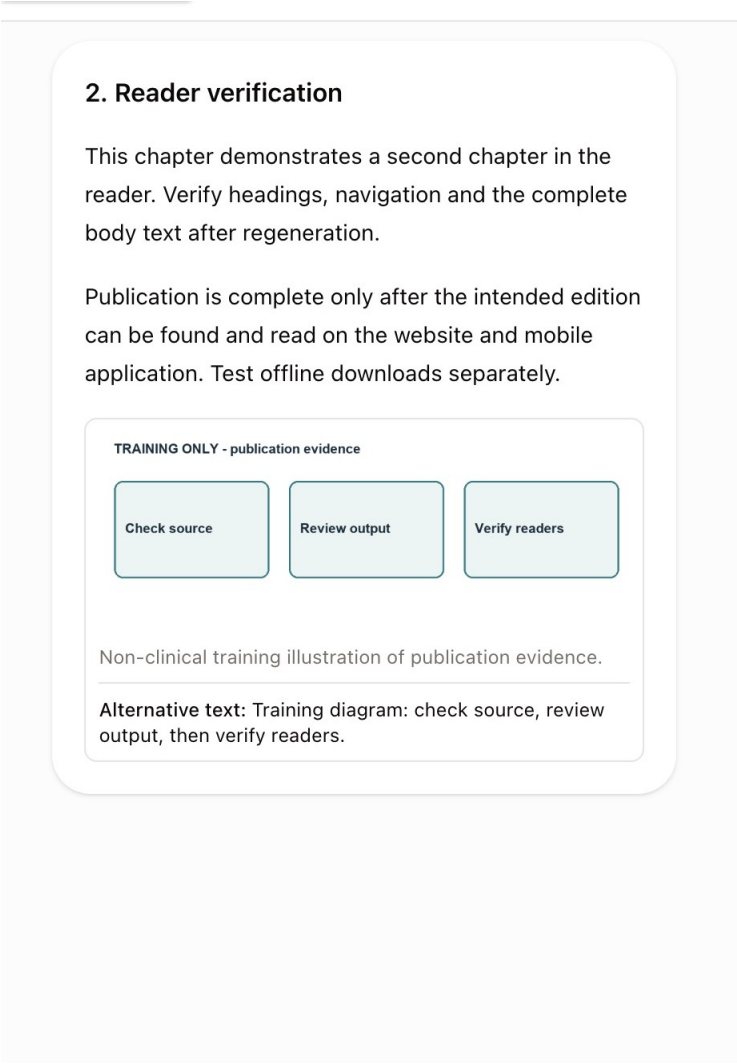
Mobile width figure preview

Reader preview > Mobile preview



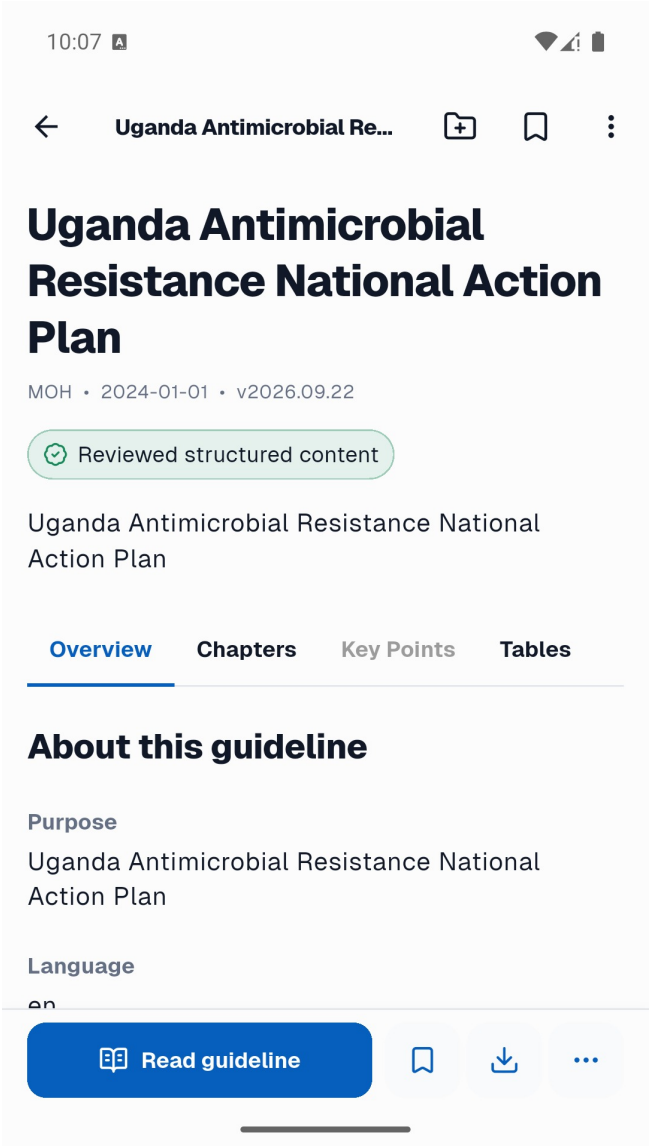
Check the entire image and caption at narrow width, then inspect equivalent content in the supported app build.

Mobile width figure preview — close-up



Check the entire image and caption at narrow width, then inspect equivalent content in the supported app build.

Android app — guideline overview



What to check on the device

Confirm the edition

Match the title, issuing body, version/date and reviewed structured content badge with the approved current publication.

Navigate the content

Open Chapters, use Search chapters and content, and select representative chapters. Use Read guideline and inspect the full content, including tables and figures.

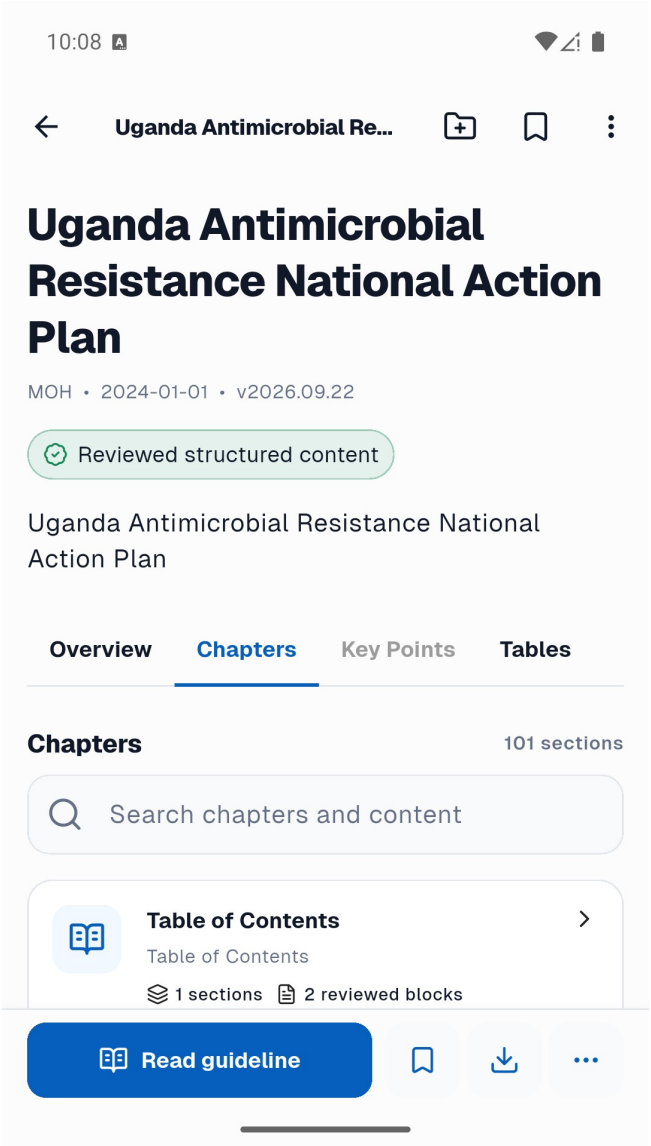
Verify downloads

The bottom download icon is visible in these app screens. Complete the download, then open the saved guideline with connectivity disabled. Check the edition and images, reconnect, and record the result.

Scope of this image

This is an existing Android store screenshot for the Antimicrobial Resistance plan, not a live test of the local training draft. These screenshots do not prove an offline package test passed.

Android app — chapters and download



What to check on the device

Confirm the edition

Match the title, issuing body, version/date and reviewed structured content badge with the approved current publication.

Navigate the content

Open Chapters, use Search chapters and content, and select representative chapters. Use Read guideline and inspect the full content, including tables and figures.

Verify downloads

The bottom download icon is visible in these app screens. Complete the download, then open the saved guideline with connectivity disabled. Check the edition and images, reconnect, and record the result.

Scope of this image

This is an existing Android store screenshot for the Antimicrobial Resistance plan, not a live test of the local training draft. These screenshots do not prove an offline package test passed.

24 Notify readers after verification

Notifications are an optional, separate approval workflow. They do not publish the guideline itself.

1. Complete reader checks first

Confirm that the intended version is published and works on the public website and app. Prepare a short factual change summary without overstating clinical changes.

2. Create a draft campaign

On the document, use Notify users when available. Choose All eligible users or the intended user/role audience, change summary, priority, optional schedule and Push / In-app channels.

3. Review before sending

Select Create draft campaign. The dialog explicitly creates a draft; it does not send directly. Open campaign to continue in Notification Operations for the normal review and approval process.

4. Verify delivery responsibly

Confirm the target document/version and audience, then use the approved notification workflow. Test the deep link and inspect delivery outcomes. Notification permission/preferences and provider delivery affect reach.

Continue when
If an announcement is needed, its approved campaign links to the verified published guideline.

Watch point: Do not describe a draft campaign as sent, or notification delivery as proof that content publication succeeded.

25 Update, withdraw or restore access

Published clinical content is immutable. Corrections use a new edition; urgent removal uses withdrawal.

1. Prepare a replacement edition

Open the published Markdown read-only view and choose the draft-from-published action. Enter a new unique version label. The immutable source is copied; regeneration does not run automatically. Correct, save, regenerate, review, accept, validate and publish.

2. Withdraw unsafe public content

An authorized publisher can select Withdraw and record a reason. This removes the document from website, app discovery, search and AI retrieval while retaining files and history. Follow local incident communication procedures.

3. Revise a withdrawn document

Choose Revise document or Continue draft. Use a new unique version label, check any omitted-content warnings and complete the full workflow. The document stays withdrawn while you work.

4. Republish unchanged approved content

Review and republish runs publication checks for the approved edition. Review the withdrawal reason/history and attest readiness. If clinical wording must change, publish a new reviewed draft instead.

Continue when
The public state matches the clinical decision, and the history explains the change.

Watch point: Downloaded copies are removed when devices next check online. Withdrawal cannot instantly reach a device that remains disconnected.

26 Special routes: forms and links

Use these paths only for document kinds intentionally configured to publish an original file or external URL.

1. Original-file route

Create document and version, then Upload file. Choose the correct PDF or Word (.docx), Upload source and wait for text indexing. Review form shows the file exactly as readers will receive it; use Preview original to check every relevant page.

2. Publish an original file

Confirm readability, edition, source, metadata and the indexing result. Publish from the document page when checks pass. No Markdown, regeneration or structured-block editor is used for this route; a corrected file belongs in a new draft version.

3. External-link route

Create document and version, choose Add link, enter the complete HTTPS URL and save. Review the actual destination, title, organization, availability and mobile behavior. No file extraction or Markdown workflow runs.

4. Verify the reader result

Publish after required checks, then test the public website and app opening behavior. For links, content and access may change at the external host; for original files, verify the actual PDF/Word layout and download behavior.

Continue when
Readers receive the intended reviewed original file or valid external destination.

Watch point: Do not promise offline availability or structured clinical page citations for an external link. Route-specific publication checks still apply.

27 Troubleshoot without losing work

Fix the cause at the correct stage. Keep evidence and avoid duplicate uploads or unreviewed shortcuts.

1. Upload or processing stalls

Distinguish transfer progress from the processing job. Resume with the same file when eligible. For repeated failures, capture job/stage/attempt/error and ask operations to check storage, workers and embedding services.

2. Regenerate is disabled or stale

Save unsaved changes and fix blocking Markdown errors. For a newer-revision warning, reload the current revision and regenerate it. Preserve local edits before resolving a server conflict.

3. Review or publication is blocked

A high-risk table warning usually means individual review is still needed. Resolve missing image references, invalid typed payloads, rejected blocks, missing reviewed prose, empty leaves and unexplained coverage regressions. Refresh approval status before acceptance.

4. Published content is absent

Verify the current published edition and withdrawal state, then test a signed-out website view and a refreshed online app. Check taxonomy/filter placement, synchronization, download age and search/index readiness. Escalate with document/version IDs and exact failing reader steps.

Continue when
The issue has a reproducible cause, an owner and a safe next action.

Watch point: Never remove clinically meaningful content, invent missing evidence, bulk-approve high-risk blocks or republish a template to make a validator pass.

28 Publication handover checklist

Use this checklist as the minimum operational record for an edition. Complete it before calling publication finished.

1. Source and authorship

Record document title/ID, kind, version/ID, issuing organization, source filename and edition, source revision/checkpoint, author, reviewer and publisher. Record the changes made and any unresolved source questions.

2. Review and readiness

Record the regenerated revision/job, completeness report export, ordinary-content review, individual high-risk/asset decisions, accepted regeneration and a passing publication validation. No unexplained content-loss or empty-clinical-section issue remains.

3. Publication and reader checks

Record publication time, current edition, public URL, web test result, mobile build/device, online discovery result, downloaded edition and offline check. Include representative table, figure and RAG citation checks where applicable.

4. Aftercare

Name the owner for defects, review-due date and withdrawal/escalation contact. Record the optional notification campaign separately. Keep audit exports in the organization's approved location; do not attach credentials or patient data.

Continue when
Publication is complete when the correct reviewed edition is current and reader-facing verification is recorded.

Watch point: If verification fails, keep the issue open and assess withdrawal or a corrective edition under local governance.

Final acceptance record

Document and source

Title/ID: _____ Version/ID: _____
Issuing body/source filename: _____
Saved revision and regeneration job: _____

Review evidence

Author: _____ Authorized reviewer: _____
Ordinary prose reviewed: ____ Required individual decisions complete: ____
Completeness export: _____ Acceptance time: _____

Publication evidence

Publisher: _____ Publication time: _____
Current edition confirmed: ____ Public URL: _____
First/middle/last chapters, table, figure and source download checked: ____

App and offline evidence

Device / OS / app build: _____
Online discovery and intended edition: ____ Download completed: ____
Offline chapters, tables and figures: ____ Reconnected: ____
Tester and time: _____

Handover

Open defects and owner: _____
Next review date: _____ Escalation contact: _____
Notification campaign, if any: _____

Sources, scope and maintenance

Existing SOPs were available

This guide expands repository runbooks: guideline-authoring-and-publication-workflow.md; guideline-editorial-review-workspace.md; guideline-editor-permissions.md; guideline-warning-resolution.md; guideline-validation-troubleshooting.md; content-hub-and-document-publishing-workflow.md. All are under docs/.

Implemented controls

Capture labels were checked in the actual running dashboard, plus dashboard/components/guidelines/ and dashboard/app/(dashboard)/guidelines/. Review policy is implemented in backend/internal/models/guideline_block_review_policy.go.

Image provenance

Native screenshots were captured from the real dashboard and public reader on 25 October 2026. Screen plates are unaltered captures apart from cropping and scaling; no dashboard controls were recreated or generated. Android plates are existing files in user_app/fastlane/metadata/android/en-US/images/phoneScreenshots/.

Known UI caveats captured during authoring

The review tab strip could be overlapped by the sticky header; keyboard navigation still opened the tabs. Legacy comparison table counts disagreed with the actual structured table/report; reconcile against rendered output and source. An advisory table warning can remain visible after the table is individually reviewed.

Maintenance and ownership

SOP owner: _____ Governance approver: _____
Approval date: _____ Next review: _____
Replace affected screen plates after workflow, policy, validator or reader changes. Record real web/app/offline acceptance evidence for each publication.